

PassReady Prep — NCE Question Bank

Generated: 2026-10-03 · Showing: In review · Total items: 200

How to review

✓ **KEY** marks the correct option. Every option shows why it is right or wrong; check both the key and each distractor.

Mark each item **Approve**, **Revise** (note what to change) or **Retire**. Approved items are published from Admin → NCE Questions.

▮ **Check** notes mark specific facts or citations the writer was less than certain of — verify those first.

Check: one clearly best answer · factual accuracy (theorists, dates, test properties, code sections) · distractors plausible but defensibly wrong · no answer given away by length or wording · references support the key.

NCE domains (NBCC 2023 content outline)

Professional Practice and Ethics 19 · Intake, Assessment, and Diagnosis 19 · Areas of Clinical Focus 47 · Treatment Planning 14 · Counseling Skills and Interventions 48 · Core Counseling Attributes 13 — of 160 scored items

In this copy

Professional Practice and Ethics **24** · Intake, Assessment, and Diagnosis **24** · Areas of Clinical Focus **59** · Treatment Planning **17** · Counseling Skills and Interventions **60** · Core Counseling Attributes **16** · **37** flagged ▮

Professional Practice and Ethics

24 items · NCE weight 12% (19 of 160 scored items)

1. nce-s-eth-001 · Professional Counseling Orientation and Ethical Practice · informed consent and its required elements · easy · In review

A newly licensed counselor is drafting an informed consent document for a private practice. Which element is a required part of informed consent under the ACA Code of Ethics?

A. A guarantee that the presenting problem will improve within a set number of sessions

Counselors explain potential benefits and risks; promising a specific outcome is misleading.

B. An explanation of the limits of confidentiality and how fees and billing are handled ✓ **KEY**

A.2.b lists confidentiality limits and fees and billing arrangements among required disclosures.

C. A summary of the counselor's personal religious and political values for the client

Counselors avoid imposing personal values (A.4.b); disclosing them is not a consent element.

D. A signed waiver releasing the counselor from liability for any outcome of treatment

Liability waivers are not part of informed consent and do not serve client welfare.

Rationale: Informed consent gives clients the information needed to choose whether to enter or continue counseling. ACA A.2.b names elements such as the purposes, goals, techniques, risks and benefits of services, the counselor's credentials, fees and billing, the limits of confidentiality, and the client's right to refuse services. Consent is an ongoing process, not a single signature.

References: ACA Code of Ethics — A.2.b Types of Information Needed

☐ Approve ☐ Revise ☐ Retire Notes:

A 12-year-old is brought to counseling by his custodial grandmother, who holds legal guardianship. The boy says he does not want to be there. How should the counselor BEST approach consent?

- A. Treat the grandmother's signature as sufficient and skip any discussion with the boy
Legal consent from a guardian does not remove the duty to seek the minor's assent.
- B. Decline to begin services until the boy is old enough to provide legal consent alone
Minors routinely receive counseling with guardian consent; refusing service is not required.
- C. Let the boy decide alone whether to continue, since his preference overrides consent
A minor generally cannot give legal consent; the guardian's role cannot simply be set aside.
- D. Obtain the guardian's consent and seek the boy's assent in language he can understand ✓ KEY**
A.2.d directs counselors to seek the assent of clients who cannot give consent and to include guardians.

Rationale: When a client cannot legally consent, counselors obtain consent from the parent or guardian and still seek the client's assent, explaining services in developmentally appropriate terms. This balances the guardian's legal rights with the minor's autonomy and welfare. Age-of-consent rules for minors vary by state, so counselors also check state law.

References: ACA Code of Ethics — A.2.d Inability to Give Consent

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is starting a grief group for adults. During the first session, what should the counselor tell members about confidentiality?

- A. That members are asked to keep what is shared private, but this cannot be guaranteed ✓ KEY**
B.4.a asks counselors to explain the importance and parameters of confidentiality in group work.
- B. That group confidentiality is protected by the same law that binds the counselor
Group members are generally not bound by the counselor's legal duties; this overstates protection.
- C. That confidentiality applies only to the counselor, so members may share freely
Members should be asked to respect each other's privacy; this undermines group safety.
- D. That any member who discloses outside the group will face legal action by the counselor
This is an inaccurate threat; the counselor cannot enforce group confidentiality this way.

Rationale: In group work the counselor can promise to protect confidentiality but cannot guarantee that other members will. ACA B.4.a requires counselors to clearly explain the importance and parameters of confidentiality for the specific group. Setting this expectation at the outset supports informed consent and group trust.

References: ACA Code of Ethics — B.4.a Group Work

☐ Approve ☐ Revise ☐ Retire Notes:

A client discloses a new diagnosis of a contagious, life-threatening disease and says she has not told her long-term partner, with whom she continues to have unprotected sex. State law permits disclosure in this situation. What should the counselor do FIRST?

- A. Contact the partner right away and inform him of the diagnosis without telling the client
B.2.c calls for assessing the client's intent to inform the partner before any disclosure.
- B. Keep the information confidential, since health conditions are outside the limits
B.2.c recognizes that disclosure to an identifiable third party at high risk may be justified.
- C. Assess whether the client intends to inform her partner or to change the risky behavior ✓ KEY
B.2.c directs counselors to assess client intent before disclosing to third parties at risk.
- D. Refer the client to a medical provider and end counseling until she discloses to him
Ending services as leverage is coercive and may amount to abandonment (A.12).

Rationale: ACA B.2.c states that when clients disclose a contagious, life-threatening disease, counselors may be justified in disclosing to identifiable third parties at serious and foreseeable risk if laws permit. Before doing so, counselors assess the client's intent to inform those parties or to stop harmful behavior. State law varies, so counselors confirm what their jurisdiction permits or requires.

References: ACA Code of Ethics — B.2.c Contagious, Life-Threatening Diseases

☐ Approve ☐ Revise ☐ Retire Notes:

During a session, a client names a former coworker, states he has bought a handgun, and describes a plan to shoot her at her home this weekend. Which action is MOST consistent with the counselor's ethical duties?

- A. Keep the threat confidential and explore the client's anger at the next session
A specific, imminent threat to an identifiable person is a recognized limit of confidentiality.
- B. Ask the client to sign a no-harm contract and continue with the planned session work
No-harm contracts do not reduce risk and do not meet the duty to protect an identified victim.
- C. Take protective steps such as warning the victim or police, as state law directs ✓ KEY
B.2.a permits disclosure to protect identified others from serious and foreseeable harm.
- D. End the session early and refer the client to a psychiatrist for medication review
Referral alone does nothing to protect the identified person from imminent harm.

Rationale: Following Tarasoff v. Regents of the University of California (1976), counselors may have a duty to protect identifiable victims of serious threats. ACA B.2.a states that confidentiality does not apply when disclosure is required to protect identified others from serious and foreseeable harm. Whether the duty is mandatory or permissive, and what steps satisfy it, varies by state law.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements · State law — Duty-to-warn/protect statutes (mandatory vs permissive, Tarasoff)

☐ Approve ☐ Revise ☐ Retire Notes:

A client frustrated about a custody dispute says, "Sometimes I could just kill my ex." He names no plan, has no weapons, has no history of violence, and laughs it off. What is the counselor's BEST response?

A. Explore the statement further to assess intent, plan, means, and history of violence ✓ KEY

An ambiguous statement calls for violence risk assessment before any decision to disclose.

B. Notify the ex-spouse and local police immediately that a threat has been made

Breaching confidentiality without assessing seriousness or foreseeability is premature.

C. Ignore the remark, since venting about an ex-spouse is common in custody conflicts

Dismissing a statement of violence without assessment risks missing real danger.

D. Terminate services because the client has made a threat against another person

Termination does not address risk and may leave the client and others less safe.

Rationale: The duty to protect is triggered by serious and foreseeable harm, not by every angry remark. The counselor should assess intent, plan, means, target, and history, consult as needed, and document the reasoning. If assessment reveals a serious threat, protective action under B.2.a and state law follows.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

7. nce-s-eth-007 · Professional Counseling Orientation and Ethical Practice · mandated reporting of abuse and neglect · medium · In review

A school-based counselor notices bruises in the shape of a hand on a 7-year-old's arm. The child says, "Dad gets mad," and then stops talking. The counselor is unsure whether abuse occurred. What should the counselor do?

A. Gather more evidence by questioning the parents before deciding whether to report

Investigating is the role of child protective services and may place the child at risk.

B. Wait to see whether new injuries appear before making a report to the authorities

Delay is not permitted once reasonable suspicion exists; certainty is not required.

C. Ask the child's teacher to make the report so the counseling relationship is protected

In most states the duty to report is personal and cannot be handed off to a colleague.

D. Make a report to child protective services based on reasonable suspicion of abuse ✓ KEY

Mandated reporters report reasonable suspicion; proof or certainty is not required.

Rationale: Counselors are mandated reporters of suspected child abuse and neglect in every U.S. state. The threshold is reasonable suspicion or cause to believe, not proof, and the counselor should not investigate. Specific timelines, reporting agencies, and procedures vary by state law, and many school policies add internal notification steps.

References: State law — Child abuse reporting statutes: reasonable suspicion standard · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

► **Check:** Rationale says counselors are mandated reporters 'in every U.S. state'; definitions vary by state.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a HIPAA-covered agency keeps process notes about session dynamics and her own impressions in a file separate from the medical record. Under the HIPAA Privacy Rule, how are these notes treated?

- A. They are part of the designated record set and may be released for any payment purpose
Separately kept psychotherapy notes are excluded from routine payment disclosures.
- B. They are psychotherapy notes, and most disclosures require a specific authorization ✓ KEY
HIPAA defines psychotherapy notes and requires specific authorization for most uses.
- C. They lose all protection once a client signs a general release of medical information
A general release does not suffice; psychotherapy notes need their own authorization.
- D. They must be given to the client on request under the HIPAA right of access to records
Psychotherapy notes are excluded from the HIPAA right of access, though state law may differ.

Rationale: HIPAA gives psychotherapy notes, meaning a counselor's notes analyzing session content that are kept separate from the medical record, special protection. Most uses and disclosures require a specific authorization, and they are excluded from the client's general right of access. Items such as diagnosis, medications, session times, and treatment plans are not psychotherapy notes.

References: HIPAA — Privacy Rule: psychotherapy notes definition and authorization (45 CFR 164.501, 164.508)

☐ Approve ☐ Revise ☐ Retire Notes:

A competent adult client asks to see a copy of her counseling records. What does the ACA Code of Ethics direct the counselor to do?

- A. Provide reasonable access, limiting it only with compelling evidence of harm to her ✓ KEY
B.6.e directs reasonable access, limited only when access would cause the client harm.
- B. Refuse the request, since counseling records belong only to the counselor and agency
Clients have a right of reasonable access to their own records under B.6.e.
- C. Release the records only after she obtains a court order or a lawyer makes a request
No court order is needed for a competent client to access her own records.
- D. Give her a summary only, because clients may not see the counselor's written records
The Code does not limit clients to summaries; it supports access and help interpreting records.

Rationale: ACA B.6.e states that counselors provide reasonable access to records and copies when requested by competent clients. Access is limited only when there is compelling evidence that it would cause harm to the client, and counselors document the request and their reasoning. Counselors also offer assistance in interpreting records (B.6.f).

References: ACA Code of Ethics — B.6.e Client Access

☐ Approve ☐ Revise ☐ Retire Notes:

Two years after ending counseling with a client, a counselor runs into the former client at a community event, and the former client asks the counselor out on a date. Under the ACA Code of Ethics, what applies?

A. A romantic relationship is allowed now, since more than one year has passed since the end

The ACA standard is five years, not one year.

B. A romantic relationship is allowed if a colleague agrees there is no risk of exploitation

Consultation does not override the five-year prohibition.

C. A romantic relationship is prohibited for five years after the last professional contact ✓ **KEY**

A.5.c prohibits such relationships for five years following the last professional contact.

D. A romantic relationship is permitted as long as the former client initiated the request

Who initiates does not change the prohibition; responsibility rests with the counselor.

Rationale: ACA A.5.c prohibits sexual or romantic relationships with former clients, their romantic partners, or family members for five years after the last professional contact, in person or electronic. Even after five years, counselors must show forethought and document that the relationship is not exploitative. The responsibility rests with the counselor regardless of who initiates.

References: ACA Code of Ethics — A.5.c Sexual and/or Romantic Relationships With Former Clients

☐ Approve ☐ Revise ☐ Retire Notes:

11. nce-s-eth-011 · Counseling and Helping Relationships · multiple relationships and boundary crossings · medium · In review

A counselor in a small rural town is invited by a long-term client to attend the client's high school graduation, which the client has worked toward despite major barriers. The counselor believes attending could benefit the client. What should the counselor do?

A. Decline, because any contact outside the office is an ethical violation for counselors

The Code permits boundary extensions that benefit clients when handled with safeguards.

B. Weigh benefits and risks, discuss it with the client, and document the rationale ✓ **KEY**

A.6.b asks counselors to document rationale, benefit, and anticipated consequences.

C. Attend without discussion, since a graduation is a public event open to everyone

A boundary extension should be discussed and documented, not done casually.

D. Attend only if the client agrees to end counseling before the graduation ceremony

Terminating services to permit contact is unnecessary and harms the client.

Rationale: ACA A.6.b recognizes that extending boundaries, such as attending a formal ceremony, can benefit clients. Before doing so, counselors document the rationale, potential benefit, and anticipated consequences, and take precautions such as informed consent, consultation, and supervision. Rural settings make such decisions more common, which increases the need for careful reasoning.

References: ACA Code of Ethics — A.6.b Extending Counseling Boundaries

☐ Approve ☐ Revise ☐ Retire Notes:

A licensed counselor completes a one-day introductory workshop on a specialized trauma therapy and wants to start using it with several clients who have complex trauma. Which course of action BEST meets the counselor's ethical obligations?

- A. Use the method now, since the counselor's license covers all trauma treatments
A license defines legal scope, not competence in every specialized method.
- B. Use the method now but tell clients it is experimental and get written consent
Consent does not replace the training needed to use a new specialty competently.
- C. Use the method now, since a workshop certificate shows enough competence to practice
A single introductory workshop rarely provides the training and supervision C.2.b requires.
- D. Pursue further training and supervised practice before using it, and refer if needed ✓ KEY
C.2.b requires education, training, and supervised experience before practicing a new specialty.

Rationale: ACA C.2.a limits practice to the boundaries of competence based on education, training, supervised experience, and credentials. When entering a new specialty area, C.2.b requires appropriate education, training, and supervised experience, and steps to protect clients from harm while skills develop. Referral is appropriate when the counselor cannot yet provide competent care.

References: ACA Code of Ethics — C.2.b New Specialty Areas of Practice · ACA Code of Ethics — C.2.a Boundaries of Competence

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants guidance on keeping separate professional and personal social media profiles. Which section of the 2014 ACA Code of Ethics MOST directly addresses this issue?

- A. Section B, Confidentiality and Privacy
Section B covers privacy broadly but social media is addressed in Section H.
- B. Section D, Relationships With Others
Section D addresses relationships with colleagues and other professionals.
- C. Section C, Professional Responsibility
Section C covers competence, advertising, and credentials, not social media profiles.
- D. Section H, Distance Counseling ✓ KEY
Section H covers distance counseling, technology, and social media, including H.6.

Rationale: The 2014 ACA Code has nine sections, A through I. Section H, Distance Counseling, Technology, and Social Media, was new in 2014; H.6.a directs counselors who keep both a professional and a personal presence on social media to create separate profiles. Knowing the section layout helps counselors find guidance quickly.

References: ACA Code of Ethics — Section H; H.6.a Virtual Professional Presence

☐ Approve ☐ Revise ☐ Retire Notes:

A licensed counselor who also holds the National Certified Counselor (NCC) credential is accused of an ethics violation. Which statement about the NBCC Code of Ethics is accurate?

A. It replaces the state licensing board, so state law no longer applies to the counselor

State licensure law applies independently of national certification.

B. It binds NCCs, and NBCC can act on the certification apart from any state board action ✓ **KEY**

NBCC enforces its Code for its certificants; boards enforce licensure separately.

C. It applies only to counselor educators who teach in CACREP-accredited graduate programs

The NBCC Code applies to NCCs and applicants, not only to educators.

D. It is advisory only, so NBCC cannot suspend or revoke a credential for a violation

NBCC can impose sanctions on certification, including revocation.

Rationale: The NBCC Code of Ethics applies to National Certified Counselors and applicants for NBCC credentials. NBCC can investigate complaints and sanction or revoke certification, which is separate from any action a state licensing board takes on a license. A counselor may therefore answer to more than one body for the same conduct.

References: NBCC Code of Ethics — Scope and application to NCCs and applicants; enforcement of certification

☐ Approve ☐ Revise ☐ Retire Notes:

While evaluating a chosen course of action, a counselor asks, "Would I be comfortable if this decision were reported in the local newspaper?" Which test from Stadler's model is the counselor applying?

A. The test of justice

Justice asks whether the counselor would treat others in the same situation the same way.

B. The test of universality

Universality asks whether the counselor would recommend the action to others in the same situation.

C. The test of publicity ✓ **KEY**

Publicity asks whether the counselor would be comfortable having the action made public.

D. The test of fidelity

Fidelity is a moral principle about keeping commitments, not one of Stadler's tests.

Rationale: Many ethical decision-making models, including the Forester-Miller and Davis practitioner's guide, include Stadler's three tests for evaluating a selected action. Justice asks whether others would be treated the same, publicity asks whether the counselor would accept public reporting, and universality asks whether the counselor would recommend it to colleagues. Passing all three supports the soundness of the decision.

References: Corey (Theory & Practice) — Ethical issues: steps in ethical decision making

► **Check:** Stadler's tests are cited to Corey's ethical decision-making chapter in general, not a section.

☐ Approve ☐ Revise ☐ Retire Notes:

A clinical supervisor learns that a post-master's supervisee has not told clients that she is under supervision or that their cases are discussed with the supervisor. What is the supervisor's MOST appropriate response?

A. Require the supervisee to inform clients of her status and how supervision affects privacy ✓ KEY

F.1.c requires that clients know supervisee qualifications and supervision's effect on confidentiality.

B. Allow it, since supervision is internal and does not change confidentiality for clients

Case discussion with a supervisor is a disclosure clients must be told about.

C. Report the supervisee to the licensing board before discussing the problem with her

Corrective supervision is the first step; a board report is premature here.

D. End the supervision contract at once and tell the supervisee to find a new supervisor

Abrupt termination leaves clients unprotected and skips remediation.

Rationale: A supervisor's primary obligation is to monitor the welfare of the clients served by the supervisee (F.1.a). Supervisors ensure that clients are aware of the supervisee's qualifications and of how the supervision process affects the limits of confidentiality (F.1.c). Supervisors share responsibility for client care and should correct the problem promptly through supervision.

References: ACA Code of Ethics — F.1.c Informed Consent and Client Rights · ACA Code of Ethics — F.1.a Client Welfare

☐ Approve ☐ Revise ☐ Retire Notes:

Which statement BEST describes the difference between state licensure and national certification for counselors?

A. Licensure is legal permission to practice in a state; certification is a voluntary credential ✓ KEY

Licensure is state regulation; certifications such as the NCC are voluntary and national.

B. Certification is required by law in every state; licensure is optional for private practice

This reverses the two; licensure is the legal requirement for independent practice.

C. Licensure and certification are the same credential, issued by different agencies by region

They are distinct credentials with different legal weight and issuing bodies.

D. Certification allows practice in all states; licensure applies only to school counselors

National certification does not authorize practice in any state.

Rationale: Licensure is a state legal credential that regulates the practice and title of professional counseling within that jurisdiction. National certification, such as the NBCC's National Certified Counselor credential, is voluntary and shows that a counselor met national standards but does not by itself permit practice. Interstate practice is addressed through state law and mechanisms such as the Counseling Compact.

References: Gladding (Counseling: A Comprehensive Profession) — Credentialing: licensure, certification, and accreditation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor learns that a client who uses a wheelchair is being denied a reasonable accommodation by her landlord. The counselor wants to contact the housing authority about this client's case. What must the counselor do FIRST?

A. Contact the housing authority right away, since advocacy is a core professional duty

Advocating for an identifiable client without consent breaches confidentiality.

B. Avoid advocacy entirely, because it falls outside the counselor's professional role

A.7.a affirms advocacy to address barriers to client growth and development.

C. Write a public letter about the landlord to raise awareness of housing discrimination

Publicizing the case exposes the client and still lacks her consent.

D. Obtain the client's consent before advocating on her behalf as an identifiable client ✓ KEY

A.7.b requires client consent before advocating on behalf of an identifiable client.

Rationale: ACA A.7.a supports advocacy at individual, group, institutional, and societal levels to address barriers that inhibit client growth. A.7.b requires counselors to obtain client consent before engaging in advocacy on behalf of an identifiable client. Seeking consent also respects the client's autonomy and may support her own self-advocacy.

References: ACA Code of Ethics — A.7.b Confidentiality and Advocacy

☐ Approve ☐ Revise ☐ Retire Notes:

19. nce-s-eth-019 · Professional Counseling Orientation and Ethical Practice · telehealth and technology ethics · medium · In review

A counselor licensed in one state is asked to provide video counseling to a client who lives in another state. Which consideration is MOST important before beginning?

A. Only the laws of the counselor's home state apply, since that is where the license is

H.1.b notes that the client's state laws may also apply.

B. The counselor may be subject to the laws of both the counselor's and client's locations ✓ KEY

H.1.b directs counselors to know the laws of both locations in distance counseling.

C. Licensure rules do not apply to video sessions, since no in-person contact takes place

Distance counseling is still the practice of counseling and is regulated.

D. Any license is valid nationwide for telehealth as long as the platform is encrypted

Encryption addresses security, not legal authority to practice across states.

Rationale: ACA H.1.b states that counselors providing distance services understand they may be subject to the laws and regulations of both their practicing location and the client's place of residence. In practice, many states regulate based on where the client is located, and rules vary by state. Interstate options such as the Counseling Compact apply only among member states.

References: ACA Code of Ethics — H.1.b Laws and Statutes · Counseling Compact — Privilege to practice in member states

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is starting video sessions with a new client who has a history of suicidal ideation. Which step is MOST important to complete during the first telehealth session?

- A. Ask the client to keep the camera off so that the session feels less intrusive
Turning off video reduces the counselor's ability to assess and does not address safety.
- B. Record the session so it can be reviewed later if any emergency happens
Recording requires consent and does nothing to prepare for an emergency.
- C. Confirm the client's location and a local emergency plan and contact person ✓ KEY
Knowing the client's location and local resources is essential for responding to a crisis.
- D. Use any free video app the client prefers so that the client feels comfortable
Platforms must protect confidentiality; preference alone is not the standard (H.2.d).

Rationale: Distance counseling requires planning for emergencies the counselor cannot handle in person. Counselors confirm the client's physical location, identify local emergency services, and agree on a plan as part of telehealth informed consent (H.2.a). This is especially important for clients with elevated risk.

References: ACA Code of Ethics — H.2.a Informed Consent and Disclosure

► **Check:** Confirm ACA H.2.a explicitly lists emergency procedures among telehealth informed-consent items.

☐ Approve ☐ Revise ☐ Retire Notes:

21. nce-s-eth-021 · Social and Cultural Diversity · termination and referral · medium · In review

A counselor with strong religious beliefs is assigned a client who wants help with stress in her same-sex marriage. The counselor feels uncomfortable and wants to refer the client elsewhere. What does the ACA Code of Ethics state?

- A. Referral is appropriate whenever a client's issue conflicts with the counselor's faith
A.11.b prohibits referral based solely on the counselor's personally held values.
- B. Referral is required, since counselors must avoid any client whose values differ
The Code calls for respecting diversity and avoiding value imposition, not avoiding clients.
- C. Referral based only on the counselor's personal values is not permitted by the Code ✓ KEY
A.11.b prohibits values-based referral; counselors seek training to work with diverse clients.
- D. Referral is acceptable if the counselor explains the religious reason to the client
Explaining the reason does not make a values-based referral ethical and may harm the client.

Rationale: ACA A.11.b states that counselors refrain from referring prospective and current clients based solely on the counselor's personally held values, attitudes, beliefs, and behaviors. A.4.b directs counselors to avoid imposing their own values. Counselors seek training and supervision when values affect their work, and referral is reserved for genuine gaps in competence.

References: ACA Code of Ethics — A.11.b Values Within Termination and Referral · ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices that a colleague has been arriving late, smelling of alcohol, and falling asleep in team meetings. The colleague carries a full caseload. What should the counselor do FIRST, if it can be done safely?

A. Speak privately with the colleague about the concerns and encourage seeking help ✓ KEY

C.2.g and I.2.a support helping colleagues recognize impairment and informal resolution first.

B. File a complaint with the state licensing board without speaking to the colleague

Formal reports follow when informal steps fail or clients face imminent harm.

C. Tell the colleague's clients about the concerns so that they can make choices

This breaches the colleague's privacy and is not the counselor's role.

D. Say nothing, since a colleague's personal life is outside the counselor's concern

The Code expects counselors to act when colleague impairment may harm clients.

Rationale: ACA C.2.g asks counselors to monitor themselves for impairment and to assist colleagues or supervisors in recognizing their own impairment, providing consultation and help when warranted. I.2.a directs counselors to first attempt informal resolution with a colleague when feasible. If the problem persists or clients are at substantial risk, counselors take further action, such as reporting to supervisors or the licensing board.

References: ACA Code of Ethics — C.2.g Impairment · ACA Code of Ethics — I.2.a Informal Resolution

☐ Approve ☐ Revise ☐ Retire Notes:

23. nce-s-eth-023 · Professional Counseling Orientation and Ethical Practice · professional organizations and their roles · easy · In review

A student comparing master's programs in clinical mental health counseling asks which organization accredits counseling graduate programs. Which organization is it?

A. NBCC

The National Board for Certified Counselors (NBCC) grants national certification (such as the NCC) and administers the NCE.

B. CACREP ✓ KEY

The Council for Accreditation of Counseling and Related Educational Programs (CACREP) accredits master's and doctoral counseling programs.

C. ACA

The American Counseling Association (ACA) is a professional membership association that publishes the Code of Ethics.

D. AMHCA

The American Mental Health Counselors Association (AMHCA) is a professional association for clinical mental health counselors.

Rationale: CACREP, the Council for Accreditation of Counseling and Related Educational Programs, accredits counseling graduate programs. NBCC certifies individual counselors and administers the NCE, while ACA and its divisions and affiliates, such as AMHCA and ASCA, are membership associations. State licensing boards regulate practice within each state.

References: CACREP 2024 Standards — Accreditation of counseling master's and doctoral programs · Gladding (Counseling: A Comprehensive Profession) — Professional organizations and accreditation

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor researcher wants to study help-seeking and plans to briefly mislead participants about the study's true aim, because full disclosure would bias their answers. Under the ACA Code of Ethics, when is this acceptable?

A. Whenever participants have signed a general consent form for the overall study

General consent does not by itself justify deception.

B. Whenever the researcher believes the study results will be published widely

Publication plans do not justify deceiving participants.

C. Whenever participants are students, since course credit offsets any harm done

Students need added protection, and credit does not offset deception.

D. Only if no alternative is feasible, the value justifies it, and debriefing follows ✓ **KEY**

G.2 permits deception only when alternatives are not feasible, value justifies it, and debriefing occurs.

Rationale: The ACA Code allows deception in research only when alternative procedures are not feasible and the prospective value of the research justifies it. Deception that is likely to cause physical or emotional harm is not permitted, and researchers explain the deception during debriefing as soon as feasible. Institutional review boards also review such designs.

References: ACA Code of Ethics — G.2 Rights of Research Participants (deception and debriefing)

► **Check:** Cited as ACA G.2 generally; confirm subsection letters for deception and debriefing.

☐ Approve ☐ Revise ☐ Retire Notes: _____

Intake, Assessment, and Diagnosis

24 items · NCE weight 12% (19 of 160 scored items)

25. nce-s-ass-001 · Counseling and Helping Relationships · intake interview structure · easy · In review

A counselor at a community agency begins a first session with a 34-year-old client who has been referred for "stress." Before asking about presenting concerns or history, what should the counselor address?

A. A full family genogram covering at least three generations

A genogram can be useful later, but it is not the opening task and does not secure informed consent.

B. Informed consent, including the limits of confidentiality ✓ **KEY**

Clients need to know their rights, the nature of services, and the limits of confidentiality before disclosing sensitive information.

C. A provisional DSM-5-TR diagnosis for the referral form

A diagnosis requires assessment data that have not yet been gathered; it cannot come first.

D. Long-term treatment goals the client hopes to reach

Goal setting follows assessment of the presenting problem; it is premature at the very start.

Rationale: Intake interviews typically open with informed consent and an explanation of confidentiality and its limits, then move to presenting problem, history, mental status, risk, and goals. Discussing limits first lets the client make an informed choice about what to disclose.

References: ACA Code of Ethics — A.2.a Informed Consent; B.1.d Explanation of Limitations

☐ Approve ☐ Revise ☐ Retire Notes:

26. nce-s-ass-002 · Counseling and Helping Relationships · mental status examination · hard · In review

When asked why he came in, a 61-year-old client describes his commute, the parking garage, a conversation with a neighbor, and the weather in great detail before finally explaining that he has been sleeping poorly. How should the counselor record this in the mental status examination?

A. Tangentiality, noted under thought process

Tangential speech drifts away and does not return to the original question; this client does return to it.

B. Flight of ideas, noted under thought content

Flight of ideas involves rapid, accelerated shifts, usually with pressured speech, and is a thought process finding, not content.

C. Circumstantiality, noted under thought process ✓ **KEY**

Excessive, irrelevant detail that eventually reaches the point is circumstantiality, a disturbance of thought process.

D. Perseveration, noted under perceptual disturbance

Perseveration is repetition of the same response, and it is not a perceptual finding.

Rationale: The MSE separates thought process (how ideas connect) from thought content (what the person thinks about). Circumstantial speech is roundabout but goal-directed in the end; tangential speech never gets back to the goal. Both are recorded under thought process.

References: Hays (Assessment) — Mental status examination: thought process

☐ Approve ☐ Revise ☐ Retire Notes:

During intake, a 19-year-old college student says, "Sometimes I wish I could just go to sleep and not wake up." What should the counselor do FIRST?

A. Ask directly about thoughts of suicide, plans, intent, and means ✓ KEY

Passive death wishes call for direct, specific questions about ideation, plan, intent, means, and past behavior to estimate risk.

B. Avoid the subject so as not to put the idea of suicide in her mind

Asking about suicide does not increase risk; avoiding it leaves risk unassessed.

C. Arrange an involuntary hospitalization before the session ends

Level of care is decided after assessment; a passive wish alone does not establish need for involuntary admission.

D. Refer her to a psychiatrist and end the intake for the day

Referral without assessment abandons the risk question and may leave the client without support.

Rationale: A wish to be dead is the lowest rung of suicidal ideation on structured tools such as the C-SSRS, but it requires follow-up. The counselor asks plainly about active thoughts, method, intent, plan, preparatory behavior, and prior attempts, then decides on next steps based on that picture.

References: C-SSRS — Ideation severity: wish to be dead through active ideation with plan and intent

☐ Approve ☐ Revise ☐ Retire Notes:

A 47-year-old veteran reports recurring suicidal thoughts without a current plan or intent. He mentions that he keeps a loaded handgun in his nightstand. Which step is MOST important to include in his safety plan?

A. A written promise that he will not harm himself before next week

No-harm contracts lack evidence of protective value and do not address lethal means.

B. A list of reasons for living to read when he feels hopeless

Reasons for living can help, but they do not reduce access to a highly lethal method.

C. A plan to increase his exercise and sleep routine this month

Wellness activities are reasonable adjuncts but leave the most urgent risk factor untouched.

D. Lethal means counseling to store the gun outside the home ✓ KEY

Firearm access sharply raises lethality; collaborative means restriction is a core safety-plan step.

Rationale: Firearms are the most lethal common method of suicide, and reducing access during high-risk periods saves lives. The Stanley-Brown Safety Planning Intervention includes making the environment safe, such as having a trusted person hold a firearm or storing it locked away from home.

References: Stanley-Brown SPI — Step 6: Making the environment safe (lethal means)

► **Check:** Confirm the Stanley-Brown step number cited for means restriction ('Step 6').

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor trained years ago asks a colleague where to record the Axis V Global Assessment of Functioning (GAF) score when using DSM-5-TR. What is the BEST answer?

A. On Axis V, as in earlier versions of the manual

DSM-5-TR has no Axis V; the multiaxial system is gone.

B. On Axis IV, together with psychosocial stressors

There are no axes in DSM-5-TR, so Axis IV does not exist either.

C. Nowhere; DSM-5-TR uses nonaxial documentation ✓ **KEY**

DSM-5 dropped the multiaxial system and the GAF; diagnoses are listed together without axes.

D. In a required appendix scored from zero to 100

No GAF appendix exists; the GAF was removed from the manual.

Rationale: DSM-5 and DSM-5-TR replaced the five-axis system with nonaxial documentation. Psychosocial and contextual factors are noted with codes from "Other Conditions That May Be a Focus of Clinical Attention," and the GAF was dropped; the WHODAS 2.0 is offered as a measure of disability.

References: DSM-5-TR — Section I: Use of the Manual (nonaxial documentation)

☐ Approve ☐ Revise ☐ Retire Notes:

A 28-year-old client currently meets criteria for a major depressive episode. Two years ago she had a 5-day period of elevated mood, decreased need for sleep, and rapid speech that friends noticed, with no marked impairment, hospitalization, or psychotic features. She has had no other mood episodes. Which diagnosis BEST fits?

A. Major depressive disorder, recurrent

A past hypomanic episode rules out major depressive disorder.

B. Bipolar II disorder ✓ **KEY**

At least one hypomanic episode plus at least one major depressive episode, with no manic episode, defines bipolar II.

C. Bipolar I disorder

Bipolar I requires a manic episode, which lasts at least one week (or any duration if hospitalized) and causes marked impairment, hospitalization, or psychosis.

D. Cyclothymic disorder

Cyclothymia requires two years of subthreshold symptoms and is excluded once a major depressive episode occurs.

Rationale: Hypomania lasts at least four consecutive days, is observable by others, and does not cause marked impairment or require hospitalization. Mania lasts at least a week (or any duration if hospitalized) and is severe enough to cause marked impairment, require hospitalization, or include psychotic features. A hypomanic episode plus a major depressive episode equals bipolar II.

References: DSM-5-TR — Bipolar II Disorder criteria; Hypomanic Episode Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A test developer gives the same anxiety inventory to the same group of adults twice, two weeks apart, and correlates the two sets of scores. Which type of reliability is being estimated?

A. Test-retest reliability ✓ **KEY**

Correlating scores from the same test given twice to the same people estimates stability over time.

B. Alternate-form reliability

Alternate-form reliability uses two different but equivalent versions of the test.

C. Split-half reliability

Split-half reliability comes from one administration divided into two halves.

D. Interrater reliability

Interrater reliability is agreement between two or more scorers or observers.

Rationale: Test-retest reliability (the coefficient of stability) reflects how consistent scores are across time. The interval matters: too short invites practice and memory effects, too long lets real change lower the coefficient.

References: Hays (Assessment) — Reliability: test-retest (coefficient of stability)

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor has a single administration of a 40-item vocabulary test in which every item is scored simply right or wrong. Which statistic is BEST suited to estimate the test's internal consistency?

A. A test-retest correlation

Test-retest requires two administrations and measures stability, not internal consistency.

B. Cohen's kappa coefficient

Kappa indexes agreement between raters, corrected for chance.

C. An alternate-form coefficient

Alternate-form reliability needs a second, equivalent version of the test.

D. Kuder-Richardson Formula 20 ✓ **KEY**

KR-20 estimates internal consistency from one administration of dichotomously scored items.

Rationale: Internal consistency estimates come from one administration. KR-20 is used with dichotomous (right/wrong) items; coefficient (Cronbach's) alpha generalizes it to items with more than two score points. Split-half reliability, corrected with the Spearman-Brown formula, is another single-administration option.

References: Erford (Research and Evaluation in Counseling) — Internal consistency: KR-20 and coefficient alpha

☐ Approve ☐ Revise ☐ Retire Notes:

Developers of a new mechanical aptitude test give it to newly hired technicians and correlate those scores with supervisor performance ratings collected 18 months later. What type of validity evidence does this provide?

A. Concurrent validity

Concurrent validity uses a criterion measured at about the same time as the test.

B. Predictive validity ✓ KEY

The criterion is measured well after testing, so this is criterion-related predictive validity.

C. Content validity

Content validity concerns how well items sample the domain, judged by experts, not a criterion correlation.

D. Discriminant validity

Discriminant validity is construct evidence showing low correlation with unrelated constructs.

Rationale: Criterion-related validity has two forms that differ by timing: concurrent (criterion gathered at the same time) and predictive (criterion gathered later). Correlating test scores with later job ratings shows how well the test forecasts future performance.

References: Hays (Assessment) — Validity: criterion-related (concurrent and predictive)

☐ Approve

☐ Revise

☐ Retire

Notes:

A new depression scale correlates .78 with an established depression inventory and .06 with a measure of musical aptitude. The low correlation with musical aptitude is BEST described as evidence of which type of validity?

A. Convergent validity

Convergent evidence is the high correlation with the established depression inventory, not the low one.

B. Predictive validity

Predictive validity concerns forecasting a later criterion, which is not described here.

C. Discriminant validity ✓ KEY

A low correlation with a theoretically unrelated construct supports discriminant (divergent) validity.

D. Face validity

Face validity is whether a test looks like it measures its target; it is not shown by correlations.

Rationale: Construct validity is supported by convergent evidence (strong relations with measures of the same construct) and discriminant evidence (weak relations with measures of different constructs). The multitrait-multimethod matrix examines both together.

References: Erford (Research and Evaluation in Counseling) — Construct validity: convergent and discriminant evidence

☐ Approve

☐ Revise

☐ Retire

Notes:

A district reading test reports, for each fourth grader, whether the student has mastered at least 80% of the grade-level reading objectives, regardless of how classmates performed. What kind of test is this?

A. A norm-referenced test

Norm-referenced tests compare a score with a norm group, which this report does not do.

B. An ipsative measure

Ipsative measures compare traits within one person, not mastery of a standard.

C. A projective technique

Projective techniques use ambiguous stimuli to explore personality.

D. A criterion-referenced test ✓ KEY

Comparing performance with a fixed standard of mastery is criterion-referenced.

Rationale: Criterion-referenced tests interpret a score against a set standard or body of content (what the person can do). Norm-referenced tests interpret a score against the performance of a comparison group (how the person ranks).

References: Hays (Assessment) — Norm-referenced vs criterion-referenced interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

Scores on a test are normally distributed. Approximately what percentage of test takers score between one standard deviation below the mean and two standard deviations above the mean?

A. About 82% ✓ KEY

About 34% fall between -1 SD and the mean, and about 47.5% between the mean and +2 SD, for roughly 82%.

B. About 68%

68% is the share within plus or minus one SD of the mean.

C. About 95%

95% is the share within plus or minus two SD of the mean.

D. About 48%

About 47.5% lies between the mean and +2 SD alone; the band below the mean is left out.

Rationale: On the normal curve about 68% of scores fall within 1 SD of the mean, about 95% within 2 SD, and about 99.7% within 3 SD. Halving those figures gives the share on one side: about 34% from the mean to 1 SD and about 47.5% from the mean to 2 SD.

References: Erford (Research and Evaluation in Counseling) — The normal curve: 68-95-99.7 rule

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor compares four results from different normally distributed tests. Which result reflects the HIGHEST standing relative to its norm group?

A. A z score of +1.2

A z of +1.2 is below the T score of 65, which equals $z = +1.5$.

B. A deviation IQ of 118

With mean 100 and SD 15, an IQ of 118 equals $z = +1.2$.

C. A T score of 65 ✓ KEY

With mean 50 and SD 10, a T of 65 is 1.5 SD above the mean, the highest of the four.

D. A percentile rank of 88

The 88th percentile corresponds to a z of about +1.17.

Rationale: Converting each score to z allows comparison: $z = (\text{score} - \text{mean}) / \text{SD}$. T scores have a mean of 50 and SD of 10; deviation IQs have a mean of 100 and SD of 15; the 84th percentile is about $z = +1.0$ and the 88th about $z = +1.17$.

References: Hays (Assessment) — Standard scores: z, T and deviation IQ conversions

☐ Approve ☐ Revise ☐ Retire Notes:

On a normally distributed test, one student's percentile rank rose from 50 to 60 and another student's rose from 89 to 99. Which statement about their raw-score gains is accurate?

A. Both students gained about the same number of raw-score points

Percentile ranks are not equal-interval units, so equal percentile gains do not mean equal raw gains.

B. The second student needed a much larger raw-score gain ✓ KEY

Scores thin out in the upper tail, so moving from the 89th to 99th percentile takes far more raw-score distance.

C. The first student needed a much larger raw-score gain

Scores cluster near the mean, so a 10-rank gain there takes a small raw-score change.

D. The gains cannot be compared without first converting to stanines

Stanines are not needed; converting percentiles to z scores shows the difference directly.

Rationale: Percentile ranks are ordinal. Because most scores pile up near the mean, a small raw change in the middle moves the percentile a lot, while the same percentile change in the tail requires a large raw change. Here 50 to 60 is about 0.25 SD; 89 to 99 is about 1.1 SD.

References: Hays (Assessment) — Percentile ranks: ordinal scale and interpretation limits

☐ Approve ☐ Revise ☐ Retire Notes:

A client obtains a score of 92 on a test with a standard error of measurement (SEM) of 4. Which range is the approximate 95% confidence band for the client's true score?

A. 88 to 96

Plus or minus 1 SEM gives a band of about 68% confidence.

B. 90 to 94

Plus or minus half an SEM is far narrower than any standard band.

C. 80 to 104

Plus or minus 3 SEM gives about 99.7% confidence, wider than asked.

D. 84 to 100 ✓ KEY

Plus or minus about 2 SEM (1.96×4 , roughly 8 points) gives the 95% band.

Rationale: The SEM is the standard deviation of a person's hypothetical scores around the true score. Bands of about 1, 2, and 3 SEM around the obtained score give roughly 68%, 95%, and 99.7% confidence that the true score lies inside.

References: Hays (Assessment) — Standard error of measurement and confidence intervals

☐ Approve ☐ Revise ☐ Retire Notes:

A test has a standard deviation of 15 and a reliability coefficient of .91. What is its standard error of measurement?

A. 4.5 points ✓ KEY

$SEM = 15 \times \text{the square root of } (1 - .91) = 15 \times 0.3 = 4.5$.

B. 1.35 points

This multiplies the SD by .09 and omits the square root.

C. 13.65 points

This multiplies the SD by the reliability coefficient itself.

D. 0.3 points

This is only the square root of $(1 - .91)$, not multiplied by the SD.

Rationale: $SEM = SD \times \text{the square root of } (1 - r)$. Higher reliability means a smaller SEM and narrower confidence bands; a perfectly reliable test would have an SEM of zero.

References: Erford (Research and Evaluation in Counseling) — Standard error of measurement formula

☐ Approve ☐ Revise ☐ Retire Notes:

A school-based counselor is part of a team evaluating the intellectual functioning of a 10-year-old. Which instrument is designed for this child's age?

A. WAIS-IV

The WAIS-IV is normed for adults and older adolescents (16 and up).

B. WPPSI-IV

The WPPSI-IV is for preschool and early primary children, below age 8.

C. WISC-V ✓ KEY

The WISC-V covers ages 6 through 16, including a 10-year-old.

D. MMPI-A

The MMPI-A is an adolescent personality inventory, not an intelligence test.

Rationale: The Wechsler scales are split by age: WPPSI-IV for young children (about 2½ to 7½), WISC-V for ages 6 to 16, and WAIS-IV for ages 16 to 90. Each yields a Full Scale IQ with a mean of 100 and SD of 15.

References: Hays (Assessment) — Intelligence testing: Wechsler scales by age range

► **Check:** Check the Wechsler age ranges used in the rationale (WPPSI-IV, WISC-V, WAIS-IV).

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants a personality inventory built on the five-factor model (neuroticism, extraversion, openness, agreeableness, and conscientiousness). Which instrument fits this purpose?

A. Rorschach Inkblot Test

The Rorschach is a projective technique, not a five-factor inventory.

B. NEO Personality Inventory-3 ✓ KEY

The NEO-PI-3 measures the five broad domains of the five-factor model and their facets.

C. Myers-Briggs Type Indicator

The MBTI sorts people into Jungian types rather than measuring the five factors.

D. MMPI-3

The MMPI-3 focuses on psychopathology and validity scales, not the five-factor model.

Rationale: The NEO-PI-3 (Costa and McCrae) assesses neuroticism, extraversion, openness, agreeableness, and conscientiousness, each with six facets. The MMPI family is used mainly to assess psychopathology and includes validity scales for over- and underreporting.

References: Hays (Assessment) — Personality inventories: NEO-PI-3 and the five-factor model

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor shows an adolescent a series of ambiguous drawings of people in everyday situations and asks him to tell a story about each one, with a beginning, middle, and end. Which instrument is being used?

A. Rorschach Inkblot Test

The Rorschach uses inkblots, not pictures of people, and asks what the person sees.

B. House-Tree-Person

In House-Tree-Person the client makes the drawings rather than responding to them.

C. Rotter Incomplete Sentences

The Rotter uses sentence stems for the client to finish, not pictures.

D. Thematic Apperception Test ✓ KEY

The TAT presents ambiguous picture cards and asks for a story about each.

Rationale: The Thematic Apperception Test, developed by Morgan and Murray, is a projective technique in which stories told about ambiguous scenes are thought to reveal needs, motives, and conflicts. Projective tests rely on ambiguous stimuli and generally have weaker psychometric support than objective inventories.

References: Hays (Assessment) — Projective techniques: Thematic Apperception Test

☐ Approve ☐ Revise ☐ Retire Notes:

44. nce-s-ass-020 · Assessment and Testing · screening instruments (PHQ-9, GAD-7, AUDIT, CAGE) · medium · In review

On the PHQ-9, a client's total score is 6, in the mild range, but she marks "several days" on the last item, which asks about thoughts that she would be better off dead or of hurting herself. What should the counselor do next?

A. Follow up at once with a direct suicide risk assessment ✓ KEY

Any endorsement of the self-harm item calls for direct risk assessment whatever the total score.

B. Note mild depression and recheck the PHQ-9 next month

Waiting ignores a positive response on the item that flags possible suicidal thinking.

C. Give the GAD-7 to see whether anxiety explains the item

An anxiety screen does not assess or rule out suicide risk.

D. Treat the item as invalid because the total score is low

A low total does not cancel a positive item 9; risk can exist with few other symptoms.

Rationale: The PHQ-9 has nine items scored 0 to 3 (total 0 to 27). Item 9 screens for passive or active thoughts of death or self-harm; any positive answer should be followed by a clinician's direct suicide risk assessment, since a screening total alone does not measure risk.

References: Hays (Assessment) — Screening instruments: Patient Health Questionnaire-9 (PHQ-9)

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old client says that when he goes a day without drinking he gets shaky, sweaty, and cannot sleep. Using the ASAM Criteria, under which dimension should the counselor rate these symptoms?

A. Readiness to change

This dimension rates motivation and engagement, not physical withdrawal.

B. Recovery environment

This dimension rates the client's living situation and supports.

C. Intoxication and withdrawal ✓ KEY

Tremor, sweating, and insomnia on stopping alcohol are withdrawal signs, rated in Dimension 1.

D. Relapse potential

This dimension rates risk of continued use, not current withdrawal.

Rationale: The ASAM Criteria rate six dimensions to match level of care. Dimension 1 covers intoxication and withdrawal; in the widely taught third edition the others address biomedical conditions, emotional/behavioral/cognitive conditions, readiness to change, relapse or continued-use potential, and the recovery environment. Alcohol withdrawal can be dangerous and may call for medically managed care.

References: ASAM Criteria — Dimension 1: intoxication and withdrawal

► **Check:** Distractors/rationale use ASAM 3rd-edition dimension names; the 4th edition (2023) renamed dimensions 3-6.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is asked to assess the cognitive abilities of a 13-year-old who arrived from Guatemala six months ago and is still learning English. Which approach is MOST appropriate?

A. Give a verbal English IQ test and report the score as usual

The score would mostly reflect English proficiency, so reporting it without caveats is invalid.

B. Use a nonverbal or native-language measure, interpreted with care ✓ KEY

Reducing language load and noting norm limits gives a fairer, more valid picture of ability.

C. Delay all testing until he has lived in the country for five years

A rigid wait is arbitrary and may delay needed services; fairer methods exist now.

D. Add 15 points to his score to adjust for the language barrier

Arbitrary score adjustment has no psychometric basis and distorts results.

Rationale: Counselors select instruments whose norms and language suit the client and interpret results in light of culture, language, and acculturation. When a test was not normed on the client's population, the report should say so. Nonverbal measures or measures in the client's dominant language reduce bias from limited English.

References: ACA Code of Ethics — E.8 Multicultural Issues/Diversity in Assessment · Sue & Sue (Counseling the Culturally Diverse) — Culturally biased assessment and test interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

During a biopsychosocial intake, a 55-year-old client with no prior mental health history describes new anxiety, a racing heart, weight loss despite a good appetite, and feeling hot all the time. What is the counselor's BEST next step?

A. Diagnose generalized anxiety disorder and begin relaxation training

GAD requires that symptoms not be due to a medical condition, which has not been ruled out.

B. Explore childhood experiences that may underlie the new anxiety

This skips the possibility that a medical problem is driving the symptoms.

C. Reassure the client that midlife anxiety is common and expected

Reassurance without evaluation may miss a treatable physical cause.

D. Recommend a medical evaluation to rule out a physical cause ✓ KEY

Late onset with weight loss and heat intolerance suggests a possible medical cause, such as thyroid disease.

Rationale: The biological part of a biopsychosocial history covers health, medications, and physical symptoms. New-onset anxiety in midlife with physical signs such as weight loss and heat intolerance can reflect conditions like hyperthyroidism, so a medical evaluation comes before a mental disorder diagnosis.

References: DSM-5-TR — Anxiety Disorder Due to Another Medical Condition; GAD Criterion E

☐ Approve

☐ Revise

☐ Retire

Notes:

A client being assessed for alcohol use says he "only has a couple of beers on weekends." With his written consent, the counselor also interviews his spouse. What is the PRIMARY value of this collateral information?

A. It can confirm or add to self-report that may be minimized ✓ KEY

Collateral sources help check self-report, which can be limited by minimization or poor insight.

B. It removes the need for a structured diagnostic interview

Collateral data supplement, not replace, a thorough clinical interview.

C. It lets the spouse set the goals of the client's treatment

Treatment goals are set collaboratively with the client, who directs his own care.

D. It supplies the evidence that state law requires to diagnose

Diagnosis does not legally require collateral reports; they are clinically useful, not mandatory.

Rationale: Collateral information from family, prior records, or other providers broadens the assessment, especially when the client may underreport, lack insight, or have memory problems. It is gathered with the client's consent and weighed alongside the client's own account.

References: Hays (Assessment) — Collateral sources of assessment information

☐ Approve

☐ Revise

☐ Retire

Notes:

Areas of Clinical Focus

59 items · NCE weight 29% (47 of 160 scored items)

49. nce-s-clf-001 · Assessment and Testing · depressive disorders · easy · In review

Under DSM-5-TR, what is the minimum duration for the symptoms of a major depressive episode?

A. One week

One week is the minimum duration for a manic episode, not a major depressive episode.

B. Two weeks ✓ KEY

A major depressive episode requires five or more symptoms present during the same 2-week period.

C. One month

One month is not the major depressive episode threshold; it is used for other diagnoses, such as the active phase of schizophrenia.

D. Six months

Six months is the duration for generalized anxiety disorder, not a major depressive episode.

Rationale: DSM-5-TR requires at least five of nine symptoms, including depressed mood or loss of interest or pleasure, present nearly every day during the same 2-week period and representing a change from prior functioning. Knowing duration thresholds helps counselors tell a depressive episode apart from shorter, situational low mood.

References: DSM-5-TR — Major Depressive Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

50. nce-s-clf-002 · Assessment and Testing · depressive disorders · medium · In review

A 41-year-old client reports feeling down more days than not for the past 3 years, with low energy, poor appetite, and hopelessness. She cannot recall a stretch of more than a few weeks without these symptoms and has never had a manic or hypomanic episode. Full criteria for a major depressive episode have not been met. Which diagnosis BEST fits?

A. Cyclothymic disorder

Cyclothymic disorder requires numerous periods of hypomanic symptoms alongside depressive symptoms; she reports no elevated mood.

B. Adjustment disorder with depressed mood

Adjustment disorder follows an identifiable stressor and does not persist more than 6 months after the stressor ends; this is a chronic 3-year pattern.

C. Disruptive mood dysregulation disorder

DMDD is a childhood diagnosis (first given between ages 6 and 18, onset before 10) marked by severe temper outbursts.

D. Persistent depressive disorder ✓ KEY

Depressed mood more days than not for at least 2 years in an adult, with two or more associated symptoms and no symptom-free period longer than 2 months, fits persistent depressive disorder.

Rationale: Persistent depressive disorder requires depressed mood for most of the day, more days than not, for at least 2 years in adults (1 year in children and adolescents), plus at least two associated symptoms such as appetite change, low energy, or hopelessness. During that period the person has not been without symptoms for more than 2 months at a time, and there has never been a manic or hypomanic episode.

References: DSM-5-TR — Persistent Depressive Disorder, Criteria A–C

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old has shown irritable mood on most days for the past 14 months, along with low self-esteem and poor concentration. He has not gone more than a few weeks without these symptoms, has no history of severe temper outbursts, and has never met criteria for a manic or hypomanic episode. Which statement is MOST accurate?

A. Criteria for persistent depressive disorder can be met at this point ✓ KEY

For children and adolescents, mood may be irritable and the duration requirement is 1 year, so 14 months is sufficient.

B. Persistent depressive disorder requires 2 full years in any age group

The 2-year requirement applies to adults; DSM-5-TR shortens it to 1 year for children and adolescents.

C. Irritable rather than sad mood rules out a depressive disorder diagnosis

DSM-5-TR allows irritable mood to satisfy the mood criterion for depressive disorders in children and adolescents.

D. Disruptive mood dysregulation disorder is the required diagnosis here

DMDD requires severe, recurrent temper outbursts with onset before age 10, which this teen does not show.

Rationale: DSM-5-TR modifies persistent depressive disorder for youth in two ways: mood can be irritable rather than depressed, and the minimum duration is 1 year instead of 2. Counselors working with adolescents should recognize that chronic irritability can be a depressive presentation rather than simply a behavior problem.

References: DSM-5-TR — Persistent Depressive Disorder, Criterion A (children and adolescents)

☐ Approve ☐ Revise ☐ Retire Notes:

Unless hospitalization is required, what is the minimum duration of a manic episode in DSM-5-TR?

A. Four consecutive days

Four consecutive days is the minimum for a hypomanic episode, not a manic episode.

B. Two weeks

Two weeks is the duration requirement for a major depressive episode.

C. One week ✓ KEY

A manic episode lasts at least 1 week and is present most of the day, nearly every day, or any duration if hospitalization is needed.

D. One month

One month is not a mood episode threshold; it is too long for mania.

Rationale: A manic episode is a distinct period of abnormally and persistently elevated, expansive, or irritable mood plus increased activity or energy lasting at least 1 week (any duration if hospitalization is necessary). A hypomanic episode needs only 4 consecutive days and does not cause marked impairment or require hospitalization.

References: DSM-5-TR — Bipolar I Disorder, Manic Episode Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old client has had two major depressive episodes. Last spring she had 5 days of unusually elevated mood, less need for sleep, and increased productivity that friends noticed but that did not impair her work or require hospitalization. She has never had psychotic symptoms or a longer elevated period. Which diagnosis is MOST appropriate?

A. Bipolar II disorder ✓ KEY

At least one hypomanic episode (4+ days, no marked impairment) plus at least one major depressive episode, with no history of mania, defines bipolar II disorder.

B. Bipolar I disorder

Bipolar I requires at least one full manic episode; her elevated period was hypomanic.

C. Cyclothymic disorder

Cyclothymic disorder requires symptoms that never meet full criteria for a major depressive episode; she has had two.

D. Major depressive disorder

A history of a hypomanic episode excludes a diagnosis of major depressive disorder.

Rationale: Bipolar II disorder requires at least one hypomanic episode and at least one major depressive episode, with no lifetime manic episode. Hypomania lasts at least 4 consecutive days, is observable by others, and is not severe enough to cause marked impairment or hospitalization. Screening for past hypomania is essential before treating depression as unipolar.

References: DSM-5-TR — Bipolar II Disorder, Criteria A–B; Hypomanic Episode

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old with no prior psychiatric history develops grandiosity, pressured speech, almost no sleep, and reckless spending. On the third day he is psychiatrically hospitalized because his behavior has become dangerous. Substances and medical causes are ruled out. Which statement is MOST accurate?

A. The episode is hypomanic because it lasted fewer than 4 days

Hypomania itself requires at least 4 days and, by definition, does not require hospitalization.

B. No bipolar diagnosis applies until symptoms reach 1 full week

The 1-week requirement is waived when hospitalization is necessary.

C. Bipolar I also requires a prior major depressive episode on record

A single manic episode is sufficient for bipolar I; a depressive episode is common but not required.

D. The episode is manic since hospitalization waives the duration ✓ KEY

DSM-5-TR counts a manic episode of any duration if hospitalization is necessary, so bipolar I disorder can be diagnosed.

Rationale: The 1-week duration for a manic episode applies unless hospitalization is necessary, in which case any duration qualifies. Bipolar I disorder requires only one lifetime manic episode; major depressive episodes are common but are not required for the diagnosis.

References: DSM-5-TR — Bipolar I Disorder, Manic Episode Criterion A and Criterion for Bipolar I

☐ Approve ☐ Revise ☐ Retire Notes:

For a diagnosis of generalized anxiety disorder, excessive anxiety and worry must occur more days than not for at least how long?

A. 1 month

One month is the post-attack worry period for panic disorder, not the GAD duration.

B. 6 months ✓ KEY

GAD requires excessive anxiety and worry more days than not for at least 6 months.

C. 3 months

Three months is not the GAD threshold.

D. 2 years

Two years is the adult duration for persistent depressive disorder.

Rationale: Generalized anxiety disorder involves excessive, hard-to-control worry about a number of events or activities, occurring more days than not for at least 6 months, with three or more associated symptoms in adults (only one in children), such as restlessness, fatigue, or sleep disturbance.

References: DSM-5-TR — Generalized Anxiety Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old client has had several sudden, unexpected surges of intense fear with a pounding heart, shortness of breath, and dizziness. For the past 2 months he has worried constantly about having another attack and has stopped exercising because a racing heart might trigger one. Medical causes have been ruled out. Which diagnosis BEST fits?

A. Panic disorder ✓ KEY

Recurrent unexpected panic attacks followed by at least 1 month of persistent worry or maladaptive behavior change define panic disorder.

B. Agoraphobia

Agoraphobia requires fear or avoidance in at least two situations such as crowds or public transit; his avoidance centers on bodily sensations.

C. Illness anxiety disorder

Illness anxiety centers on a preoccupation with having a serious illness, not on recurrent unexpected panic attacks.

D. Specific phobia, situational

Specific phobia involves fear cued by a particular object or situation, whereas these attacks are unexpected.

Rationale: Panic disorder requires recurrent unexpected panic attacks, with at least one attack followed by 1 month or more of persistent concern about further attacks or their consequences, or a significant maladaptive change in behavior such as avoiding exercise. Interoceptive avoidance like this is a common target of cognitive behavioral treatment.

References: DSM-5-TR — Panic Disorder, Criteria A–B · Barlow PCT — Interoceptive avoidance and exposure

☐ Approve ☐ Revise ☐ Retire Notes:

Ten days after a serious highway collision, a 24-year-old reports intrusive memories, nightmares, feeling as if the world is unreal, avoiding driving, and trouble sleeping and concentrating. The symptoms began right after the crash and are impairing her work. Which diagnosis is MOST appropriate at this time?

A. Posttraumatic stress disorder

PTSD requires symptoms lasting more than 1 month; only 10 days have passed.

B. Adjustment disorder with anxiety

When symptoms meet criteria for a trauma-specific disorder, adjustment disorder is not the best fit.

C. Acute stress disorder ✓ KEY

Acute stress disorder applies when trauma-related symptoms last from 3 days to 1 month after exposure.

D. Generalized anxiety disorder

GAD involves diffuse worry for at least 6 months and is not tied to a traumatic event.

Rationale: Acute stress disorder is diagnosed when nine or more symptoms across intrusion, negative mood, dissociation, avoidance, and arousal last from 3 days to 1 month after trauma exposure. If symptoms persist beyond 1 month and meet PTSD criteria, the diagnosis is changed to PTSD.

References: DSM-5-TR — Acute Stress Disorder, Criteria B–C

☐ Approve ☐ Revise ☐ Retire Notes:

A firefighter developed some sleep problems and irritability soon after a fatal fire. Eight months later, after more symptoms emerged, she meets full criteria for PTSD for the first time. She does not report depersonalization or derealization. Which specifier applies?

A. With dissociative symptoms

The dissociative specifier requires persistent depersonalization or derealization, which she denies.

B. With panic attacks

The with-panic-attacks specifier requires panic attacks alongside the disorder, and none are described here.

C. With acute onset

DSM-5-TR PTSD has no acute-onset specifier; the DSM-IV acute and chronic specifiers were removed.

D. With delayed expression ✓ KEY

This specifier is used when full criteria are not met until at least 6 months after the event, even if some symptoms began earlier.

Rationale: In DSM-5-TR, PTSD "with delayed expression" is specified when full diagnostic criteria are not met until at least 6 months after the event, although the onset and expression of some symptoms may be immediate. The other PTSD specifier is "with dissociative symptoms" (depersonalization or derealization).

References: DSM-5-TR — Posttraumatic Stress Disorder, specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A client with contamination obsessions washes his hands for hours daily. He says his family will probably become seriously ill if he stops washing, although he admits he could be wrong. Which DSM-5-TR insight specifier is MOST accurate?

A. With good or fair insight

Good or fair insight means the person recognizes the beliefs are definitely or probably not true.

B. With poor insight ✓ KEY

Poor insight applies when the person thinks the OCD beliefs are probably true.

C. With absent insight

Absent insight/delusional beliefs applies when the person is completely convinced the beliefs are true; he admits doubt.

D. With tic-related features

The tic-related specifier concerns a current or past tic disorder, not the degree of insight.

Rationale: DSM-5-TR rates insight in OCD on three levels: good or fair (beliefs definitely or probably not true), poor (beliefs probably true), and absent insight/delusional beliefs (completely convinced the beliefs are true). A separate "tic-related" specifier notes a current or past tic disorder.

References: DSM-5-TR — Obsessive-Compulsive Disorder, insight specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

60. nce-s-clf-012 · Counseling and Helping Relationships · obsessive-compulsive and related disorders · easy · In review

Which psychological intervention has the strongest evidence as a first-line treatment for obsessive-compulsive disorder?

A. Insight-oriented psychodynamic therapy

Psychodynamic therapy lacks the evidence base that exposure-based CBT has for OCD.

B. Thought stopping with reassurance

Reassurance acts as a compulsion and maintains OCD; thought stopping tends to backfire.

C. Exposure and response prevention ✓ KEY

ERP, a form of CBT, exposes clients to feared cues while they refrain from compulsions and is the first-line psychological treatment.

D. Nondirective supportive counseling

Supportive counseling alone does not target the obsession–compulsion cycle.

Rationale: Exposure and response prevention has clients face feared thoughts, images, or situations while resisting compulsions, allowing anxiety to decline without the ritual and weakening the negative reinforcement that maintains OCD. Reassurance-giving by the counselor or family is itself a compulsion to be reduced.

References: NICE guidelines — CG31 Obsessive-compulsive disorder and body dysmorphic disorder: CBT including ERP

☐ Approve ☐ Revise ☐ Retire Notes:

Over the past 12 months, a client has met five of the eleven DSM-5-TR criteria for alcohol use disorder. Which severity specifier applies?

A. Mild

Mild applies when two or three criteria are met.

B. Severe

Severe applies when six or more criteria are met.

C. Moderate ✓ KEY

Moderate applies when four or five criteria are met.

D. Subthreshold

Two criteria within 12 months are enough for a diagnosis, so five is well above threshold.

Rationale: DSM-5-TR substance use disorders require at least two of eleven criteria within a 12-month period. Severity is specified as mild (2–3 criteria), moderate (4–5), or severe (6 or more). Severity guides level-of-care decisions alongside a dimensional assessment such as the ASAM Criteria.

References: DSM-5-TR — Alcohol Use Disorder, severity specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old takes a prescribed opioid exactly as directed for chronic back pain under a physician's care. She reports needing a higher dose than at first and feeling withdrawal symptoms if a dose is late. No other use disorder criteria are present. How should the counselor understand these symptoms?

A. They are not counted toward an opioid use disorder ✓ KEY

Tolerance and withdrawal are not counted when the opioid is taken solely under appropriate medical supervision.

B. They meet criteria for mild opioid use disorder

Two counted criteria would be mild, but these two are excluded in supervised medical use.

C. They confirm a severe opioid use disorder is present

Physiological dependence alone does not establish any use disorder, let alone a severe one.

D. They point to opioid use disorder in remission

Remission requires a prior full use disorder, which has not been established.

Rationale: DSM-5-TR notes that tolerance and withdrawal are expected physiological adaptations when medications such as opioids, sedatives, or stimulants are taken as prescribed under medical supervision, and they are not counted toward a use disorder in that situation. A diagnosis requires other evidence of problematic use, such as craving or loss of control.

References: DSM-5-TR — Opioid Use Disorder, Criteria 10–11 note on medical supervision

► **Check:** The cited 'Criteria 10-11 note' detail is the writer's wording of where the DSM-5-TR note sits.

☐ Approve ☐ Revise ☐ Retire Notes:

Under DSM-5-TR, several inattentive or hyperactive-impulsive symptoms of ADHD must have been present before what age?

A. Age 7

Onset before age 7 was the DSM-IV standard; DSM-5 raised it to 12.

B. Age 12 ✓ KEY

DSM-5-TR requires several symptoms to have been present before age 12.

C. Age 16

Age 16 is not a criterion; age 17 marks where the adult symptom count of five applies.

D. Age 18

Age 18 is not the ADHD onset threshold.

Rationale: ADHD requires several inattentive or hyperactive-impulsive symptoms before age 12, present in two or more settings, with clear interference in functioning. Children need six symptoms in a domain; adolescents 17 and older and adults need five. Adult assessment therefore includes a developmental history.

References: DSM-5-TR — Attention-Deficit/Hyperactivity Disorder, Criteria A and C

☐ Approve ☐ Revise ☐ Retire Notes:

A 7-year-old struggles to take turns in conversation, misses jokes and hints, and speaks to teachers the same way he speaks to peers. A counselor is weighing autism spectrum disorder against social (pragmatic) communication disorder. Which finding would MOST support autism spectrum disorder?

A. Late onset of first spoken words

Language delay can occur in many conditions and is not a defining criterion that separates the two.

B. Trouble using language socially

Difficulty with social use of language is central to both disorders, so it does not distinguish them.

C. Below-average intellectual ability

Intellectual impairment is a possible specifier for ASD but is neither required nor distinguishing.

D. Restricted, repetitive behaviors ✓ KEY

ASD requires restricted, repetitive patterns of behavior, interests, or activities; their presence rules out SCD.

Rationale: Both ASD and social (pragmatic) communication disorder involve deficits in social communication. ASD also requires at least two kinds of restricted, repetitive patterns of behavior, interests, or activities (such as rigid routines, intense interests, or sensory differences). SCD is diagnosed only when such patterns are absent, past or present.

References: DSM-5-TR — Social (Pragmatic) Communication Disorder, Criterion D; Autism Spectrum Disorder, Criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old college student has had auditory hallucinations, persecutory delusions, and disorganized speech for about 3 months, including any early warning signs. There have been no mood episodes, and substance and medical causes are ruled out. Which diagnosis is MOST appropriate?

A. Schizophreniform disorder ✓ **KEY**

Schizophreniform disorder has schizophrenia-like symptoms lasting at least 1 month but less than 6 months.

B. Schizophrenia

Schizophrenia requires continuous signs of disturbance for at least 6 months.

C. Brief psychotic disorder

Brief psychotic disorder lasts at least 1 day but less than 1 month, with full return to functioning.

D. Delusional disorder

Delusional disorder involves delusions without prominent hallucinations or disorganized speech.

Rationale: The schizophrenia spectrum is partly sorted by duration: brief psychotic disorder lasts 1 day to less than 1 month; schizophreniform disorder lasts 1 month to less than 6 months; schizophrenia requires at least 6 months of disturbance, including at least 1 month of active-phase symptoms.

References: DSM-5-TR — Schizophreniform Disorder, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 26-year-old has had 5 weeks of prominent hallucinations and delusions. His family reports that for about 6 months before that, he withdrew from friends, showed flat affect, and voiced odd beliefs. No mood episodes have occurred, and substance and medical causes are excluded. Which diagnosis BEST fits?

A. Schizophreniform disorder

The prodromal period counts toward duration, so the total exceeds the less-than-6-months limit for schizophreniform disorder.

B. Schizophrenia ✓ **KEY**

More than 1 month of active symptoms plus a prodromal period gives over 6 months of continuous disturbance.

C. Brief psychotic disorder

Brief psychotic disorder lasts less than 1 month in total; the active phase alone is longer.

D. Schizoaffective disorder

Schizoaffective disorder requires a major mood episode concurrent with psychotic symptoms; none occurred.

Rationale: Schizophrenia requires continuous signs of disturbance for at least 6 months, which must include at least 1 month of active-phase symptoms and may include prodromal or residual periods with negative symptoms or attenuated positive symptoms. Collateral history from family is often essential for establishing the prodromal phase.

References: DSM-5-TR — Schizophrenia, Criterion C

☐ Approve ☐ Revise ☐ Retire Notes: _____

Which personality disorder belongs to DSM-5-TR Cluster C (the anxious or fearful cluster)?

A. Schizotypal personality disorder

Schizotypal is in Cluster A, the odd or eccentric cluster.

B. Histrionic personality disorder

Histrionic is in Cluster B, the dramatic, emotional, or erratic cluster.

C. Avoidant personality disorder ✓ KEY

Avoidant personality disorder is in Cluster C with dependent and obsessive-compulsive personality disorders.

D. Paranoid personality disorder

Paranoid is in Cluster A with schizoid and schizotypal.

Rationale: DSM-5-TR groups personality disorders into three clusters: A (paranoid, schizoid, schizotypal) for odd or eccentric patterns; B (antisocial, borderline, histrionic, narcissistic) for dramatic or erratic patterns; and C (avoidant, dependent, obsessive-compulsive) for anxious or fearful patterns.

References: DSM-5-TR — Personality Disorders, cluster groupings

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old shows a pervasive pattern of deceitfulness, impulsivity, repeated fights, and lack of remorse since age 15. To diagnose antisocial personality disorder, which additional finding does DSM-5-TR require?

A. A documented history of arrest or jail

Legal involvement is common but not required; failure to conform to social norms can be shown in other ways.

B. No co-occurring substance use disorder

Substance use disorders frequently co-occur and do not rule out antisocial personality disorder.

C. Onset of the pattern after age 18 only

The pattern must be evident since age 15; onset is not restricted to adulthood.

D. Evidence of conduct disorder before 15 ✓ KEY

DSM-5-TR requires evidence of conduct disorder with onset before age 15, and the person must be at least 18.

Rationale: Antisocial personality disorder requires a pervasive pattern of disregard for and violation of the rights of others occurring since age 15, a current age of at least 18, and evidence of conduct disorder with onset before age 15. The behavior must not occur only during schizophrenia or bipolar disorder.

References: DSM-5-TR — Antisocial Personality Disorder, Criteria B–C

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old describes episodes, about twice a week for 6 months, of eating far more than most people would in a short period while feeling unable to stop. She eats rapidly, eats alone out of embarrassment, and feels disgusted afterward. She does not vomit, fast, misuse laxatives, or exercise to compensate. Which diagnosis BEST fits?

A. Bulimia nervosa

Bulimia nervosa requires recurrent inappropriate compensatory behaviors, which she does not use.

B. Binge-eating disorder ✓ KEY

Recurrent binges with loss of control and marked distress, at least weekly for 3 months, without regular compensatory behavior, fit binge-eating disorder.

C. ARFID

ARFID involves restricted intake, not binge episodes.

D. Anorexia nervosa, binge/purge type

Anorexia nervosa requires restriction leading to significantly low body weight and fear of weight gain, none of which is described, and she does not purge.

Rationale: Binge-eating disorder requires recurrent binge episodes with a sense of loss of control, three or more associated features (such as eating rapidly, eating alone from embarrassment, or feeling disgusted afterward), marked distress, and a frequency of at least once a week for 3 months. Unlike bulimia nervosa, there are no recurrent compensatory behaviors.

References: DSM-5-TR — Binge-Eating Disorder, Criteria A–E

☐ Approve

☐ Revise

☐ Retire

Notes:

Since choking on a piece of food 4 months ago, a 10-year-old refuses most solid foods, eats only liquids and soft foods, and has lost weight. She expresses no concern about body shape or size and wants to be "back to normal." Which diagnosis is MOST likely?

A. Anorexia nervosa

Anorexia nervosa involves fear of weight gain or disturbed body image, which she does not show.

B. Rumination disorder

Rumination disorder involves repeated regurgitation of food, not avoidance after choking.

C. ARFID ✓ KEY

ARFID covers restriction driven by fear of aversive consequences such as choking, without body image disturbance.

D. Pica

Pica involves persistent eating of nonnutritive, nonfood substances.

Rationale: Avoidant/restrictive food intake disorder involves restricted eating that leads to weight loss, nutritional deficiency, or impaired functioning, motivated by lack of interest in food, sensory aversions, or fear of aversive consequences such as choking or vomiting. It is distinguished from anorexia nervosa by the absence of fear of weight gain or disturbed body image.

References: DSM-5-TR — Avoidant/Restrictive Food Intake Disorder, Criteria A and C

☐ Approve

☐ Revise

☐ Retire

Notes:

A 74-year-old retired teacher and her daughter both report a gradual decline in her memory over the past year. Testing shows a modest decline from her prior level. She still manages her own bills, medications, and shopping, although these tasks now take more effort and reminder lists. Which diagnosis BEST fits?

A. Major neurocognitive disorder

Major NCD requires substantial decline that interferes with independence in everyday activities.

B. Delirium

Delirium has an acute onset with fluctuating attention and awareness, not a gradual year-long decline.

C. Mild neurocognitive disorder ✓ KEY

Modest documented decline that does not interfere with independence, even if compensation is needed, fits mild NCD.

D. Dissociative amnesia

Dissociative amnesia involves inability to recall autobiographical information, usually traumatic, not a gradual cognitive decline.

Rationale: Mild and major neurocognitive disorder differ chiefly in functional impact. Mild NCD shows modest decline from a previous level, supported by concern and testing, without loss of independence in everyday activities, though greater effort or compensatory strategies may be needed. Major NCD involves significant decline that interferes with independence.

References: DSM-5-TR — Mild Neurocognitive Disorder, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes:

An 81-year-old hospitalized for a urinary tract infection became confused over 2 days. Staff note she is alert one hour and disoriented the next. A counselor on the unit is asked to help distinguish delirium from major neurocognitive disorder. Which feature MOST points to delirium?

A. Acute onset with fluctuating attention ✓ KEY

Delirium develops over hours to days and fluctuates in severity, with disturbed attention and awareness.

B. Memory impairment found on screening

Memory deficits occur in both delirium and major NCD, so they do not distinguish them.

C. Onset of symptoms after the age of 65

Both conditions are more common in older adults; age does not separate them.

D. Presence of vivid visual hallucinations

Visual hallucinations can occur in delirium and in major NCD, notably with Lewy bodies.

Rationale: Delirium is a disturbance in attention and awareness that develops over a short period (hours to a few days), represents a change from baseline, and tends to fluctuate during the day. It often has a medical cause, such as infection or medication effects, and requires prompt medical evaluation.

References: DSM-5-TR — Delirium, Criteria A–B

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old whose wife died 7 months ago reports intense yearning for her nearly every day, feeling that part of himself has died, and difficulty re-engaging with friends. His grief is far beyond what is expected in his community and is impairing his work. Which statement about prolonged grief disorder is MOST accurate?

A. It can be diagnosed now because symptoms have lasted over 6 months

Six months is the threshold for children and adolescents, not adults.

B. It cannot yet be diagnosed because 12 months have not yet passed ✓ **KEY**

For adults, DSM-5-TR requires at least 12 months since the death.

C. It is not in DSM-5-TR, so his grief is coded only as a Z code

Prolonged grief disorder was added to the trauma- and stressor-related disorders in DSM-5-TR; uncomplicated bereavement is the Z code.

D. It is ruled out, and major depression must be diagnosed instead

Nothing here establishes a major depressive episode; yearning and identity disruption are grief-specific.

Rationale: DSM-5-TR added prolonged grief disorder to the trauma- and stressor-related disorders. It requires that at least 12 months have passed since the death for adults (6 months for children and adolescents), with intense yearning or preoccupation nearly every day for at least the last month, plus at least three additional symptoms such as identity disruption or emotional numbness.

References: DSM-5-TR — Prolonged Grief Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

Six weeks after his mother died, a client reports sadness, poor sleep, and crying spells. Which feature is MORE typical of grief than of a major depressive episode?

A. Pervasive worthlessness and self-loathing

Pervasive worthlessness and self-loathing are more typical of a major depressive episode.

B. Persistent unhappiness unrelated to reminders

Mood that stays low regardless of reminders of the loss is more typical of depression.

C. Self-esteem that is generally preserved ✓ **KEY**

In grief, self-esteem is usually preserved, while depression often brings worthlessness.

D. Inability to anticipate any future pleasure

Pervasive anhedonia and pessimism about the future are characteristic of a depressive episode.

Rationale: DSM-5-TR notes that in grief the predominant feelings are emptiness and loss, dysphoria tends to come in waves ("pangs") linked to reminders of the deceased, humor can coexist with pain, and self-esteem is usually preserved. In a major depressive episode, mood is persistently low and is often accompanied by worthlessness and self-loathing.

References: DSM-5-TR — Major Depressive Disorder, note distinguishing grief from a major depressive episode

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor volunteers at a shelter the day after a tornado destroyed many homes. Survivors are shaken, tired, and worried about relatives. Which approach is MOST appropriate in this early phase?

A. Lead a required group debriefing on each survivor's experience

Mandatory single-session debriefing is not recommended and may increase distress for some survivors.

B. Give each survivor a structured PTSD diagnostic interview

Most survivors recover naturally; diagnostic interviews this early are premature and intrusive.

C. Start trauma-focused exposure work with the survivors who are upset

Trauma-focused therapy is for persistent symptoms after the acute phase, not as an immediate shelter intervention.

D. Offer psychological first aid for safety, comfort, and needs ✓ **KEY**

Psychological first aid promotes safety, calm, connection, and practical help, and is the recommended early response.

Rationale: In the immediate aftermath of a disaster, the recommended response is psychological first aid: ensuring safety, providing comfort, meeting practical needs, connecting people with loved ones and supports, and screening for those needing more help. Routine single-session psychological debriefing is not recommended for preventing PTSD.

References: NICE guidelines — NG116 Post-traumatic stress disorder: psychologically focused debriefing not offered

► **Check:** No PFA-specific source on the approved list; NICE NG116 supports the no-debriefing point only.

☐ Approve ☐ Revise ☐ Retire Notes:

A client calls the counselor sobbing after receiving an eviction notice. She says, "I just don't see the point of anything anymore." What should the counselor do FIRST?

A. Ask her directly about suicidal thoughts and intent ✓ **KEY**

A statement of hopelessness in crisis calls for direct assessment of suicidal ideation, intent, and plan before other steps.

B. Help her brainstorm options for finding new housing now

Problem solving is useful later, but safety must be assessed first.

C. Reflect feelings and save risk questions for later

Delaying risk assessment after a possible warning sign could miss imminent danger.

D. Give her the number of a local legal aid clinic

A referral addresses the stressor but skips assessment of her immediate safety.

Rationale: In crisis intervention, ensuring safety comes before problem solving. Hopeless statements such as "I don't see the point" are possible warning signs, and asking directly about suicidal thoughts does not increase risk. Structured tools such as the C-SSRS guide questions about ideation, intent, plan, and behavior.

References: C-SSRS — Screener: suicidal ideation and intent questions

☐ Approve ☐ Revise ☐ Retire Notes:

During the individual portion of a couples intake, one partner discloses that the other checks her phone, controls all the money, and shoved her last month. She says she is afraid of what will happen if he learns she told. What is the counselor's BEST next step?

A. Begin joint sessions on communication skills

Conjoint work is contraindicated while coercive control and fear are present, as it can increase danger.

B. Raise her disclosure in the next joint session

Revealing the disclosure to the partner could trigger retaliation.

C. Refer both partners to one anger management group

Anger management does not address coercive control, and joint referral may compromise her safety.

D. Assess danger with her alone and plan for safety ✓ KEY

Individual risk assessment and safety planning come first when violence and fear are disclosed.

Rationale: When intimate partner violence involving fear and coercive control is disclosed, conjoint counseling is generally contraindicated because sessions can provoke retaliation. The counselor should meet with the person alone, assess lethality and danger, develop a safety plan, and connect her with advocacy resources while protecting the confidentiality of the disclosure.

References: ACA Code of Ethics — A.4.a Avoiding Harm

► **Check:** IPV item cites general sources (ACA, Sue & Sue); no IPV-specific source on the approved list.

☐ Approve ☐ Revise ☐ Retire Notes:

A recently immigrated client discloses that her husband hits her and threatens that she will be deported if she leaves him, since her visa depends on his sponsorship. She speaks limited English and has no family nearby. Which counselor response is MOST helpful?

A. Advise her to stay until her immigration status is secure

This accepts the abuser's threat at face value and leaves her in danger without exploring legal protections.

B. Connect her with an immigration-informed IPV advocate ✓ KEY

Specialized advocates can explain protections available to abused immigrants and support culturally responsive safety planning.

C. Report the husband's conduct to immigration authorities

This is not the counselor's role, breaches confidentiality, and may put the client at greater risk.

D. Suggest couples counseling to ease conflict at home now

Couples counseling is contraindicated while there is ongoing violence and threats.

Rationale: Abusers of immigrant partners often use immigration status as a tool of control. Counselors should recognize this cultural and structural vulnerability, prioritize safety, provide interpreter and language access, and link clients with advocates who understand immigration-related protections rather than offering legal advice themselves.

References: Sue & Sue (Counseling the Culturally Diverse) — Immigrant and refugee clients: structural barriers and advocacy

► **Check:** IPV item cites general sources; the key deliberately avoids specific immigration-law (VAWA) claims.

☐ Approve ☐ Revise ☐ Retire Notes:

During a session at an elementary school, a 7-year-old tells the school counselor that her stepfather hit her with a belt last night, and the counselor sees welts on her arm. What should the counselor do FIRST?

A. Make a report to child protective services as the law requires of mandated reporters ✓ KEY

Reasonable suspicion of abuse triggers a mandated report; the counselor reports promptly and lets child protective services investigate.

B. Call the stepfather to hear his account of what happened before deciding to file any report

Contacting the alleged offender can put the child at risk and is not required before reporting; investigation belongs to CPS.

C. Examine and photograph all of the child's injuries so the evidence is complete before reporting

Counselors do not conduct forensic examinations; doing so can retraumatize the child and is the investigator's role.

D. Schedule a follow-up meeting for next week to see whether new marks appear before reporting

Waiting delays protection; the reporting duty arises at reasonable suspicion, which is already present.

Rationale: Counselors are mandated reporters in every U.S. state. The threshold is reasonable suspicion, not proof, and the report is made promptly to child protective services or law enforcement. The counselor's job is to report and support the child, not to investigate or confront the alleged perpetrator.

References: State law — Mandated reporting statutes for suspected child abuse and neglect · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

A 10-year-old client mentions that he is often home alone for whole weekends with little food in the house. He gives few details and changes the subject. A colleague advises the counselor not to report neglect until the boy gives a clear account. Which statement BEST reflects the reporting standard?

A. A report is required only after the child gives a clear and consistent disclosure

Children often disclose partially; the law does not require a complete or consistent disclosure before reporting.

B. A report is required only after a parent has a chance to explain the circumstances

Mandated reporters do not need to hear the caregiver's explanation first; that is part of the CPS investigation.

C. A report is required when there is reasonable suspicion; proof of neglect is not needed ✓ KEY

The legal threshold is reasonable cause to suspect abuse or neglect, which repeated lack of supervision and food can meet.

D. A report is required only once a supervisor independently confirms that neglect occurred

Consulting a supervisor is good practice, but the individual duty to report does not wait for anyone to confirm the facts.

Rationale: Mandated reporting laws use a reasonable-suspicion standard. Counselors report what they have observed or been told and leave fact-finding to child protective services. Neglect, including inadequate supervision and failure to provide food, is reportable in the same way as physical abuse.

References: State law — Child abuse and neglect reporting statutes: reasonable cause to suspect standard

☐ Approve ☐ Revise ☐ Retire Notes:

In Holland's theory, consistency describes how close the first two letters of a person's code sit on the RIASEC hexagon. Which Holland code shows the LOWEST consistency?

A. RIC (Realistic-Investigative)

Realistic and Investigative are adjacent on the hexagon, so this code is highly consistent.

B. SEA (Social-Enterprising)

Social and Enterprising are adjacent on the hexagon, so this code is highly consistent.

C. ECR (Enterprising-Conventional)

Enterprising and Conventional are adjacent on the hexagon, so this code is highly consistent.

D. RSI (Realistic-Social) ✓ KEY

Realistic and Social sit opposite each other on the hexagon, the least consistent pairing.

Rationale: Holland arranged the six types in the order R-I-A-S-E-C around a hexagon. Adjacent types share the most in common and opposite types the least, so a code whose first two letters are opposite (R-S, I-E, A-C) has low consistency. Consistency is distinct from differentiation (how sharply one type stands out) and congruence (person-environment fit).

References: Sharf (Career Development Theory) — Holland's theory of types: consistency, differentiation, congruence

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old machinist who expected to finish his career at one plant is laid off when it closes. He is now researching training programs, trying out new interests, and reconsidering what work means to him. Which concept from Super's life-span, life-space theory BEST describes this process?

A. Recycling through exploration tasks during a later life stage ✓ KEY

Super held that adults can recycle through earlier stage tasks, such as exploration, when a transition disrupts their career.

B. Circumscription of options that conflict with his social class

Circumscription belongs to Gottfredson's theory and describes early elimination of options by sex type and prestige.

C. Planned happenstance that turns the chance layoff into learning

Planned happenstance is Krumboltz's concept, not Super's, and describes using unplanned events, not a stage process.

D. Correspondence between his abilities and the reinforcers of work

Correspondence is the core idea of the theory of work adjustment (Dawis and Lofquist), not of Super's stage model.

Rationale: Super's stages (growth, exploration, establishment, maintenance, disengagement) form a maxicycle across the life span, but people can recycle through minicycles of earlier stages at transitions such as job loss. A midlife worker re-exploring options is the classic example of recycling.

References: Sharf (Career Development Theory) — Super's life-span, life-space theory: maxicycle, minicycles and recycling

☐ Approve ☐ Revise ☐ Retire Notes:

A 7-year-old girl who loves building things tells her counselor that she cannot be an engineer because "that is a job for boys." In Gottfredson's theory, what is this child doing?

A. Compromising by settling for a job that is more accessible to her

Compromise comes later, when people give up preferred options because of real-world barriers to access.

B. Circumscribing her options by ruling out jobs seen as wrong for her sex ✓ **KEY**

Ages 6 to 8 are Gottfredson's orientation to sex roles, when children eliminate jobs that seem wrong for their gender.

C. Showing low differentiation because her interests are not yet clear

Differentiation is a Holland concept about how distinct a profile is; her interest in building is actually clear.

D. Crystallizing a vocational preference as the exploration stage begins

Crystallization is a task in Super's exploration stage during adolescence, not a process of ruling options out in childhood.

Rationale: Gottfredson's theory of circumscription and compromise describes children progressively eliminating occupations: by size and power (ages 3-5), by sex type (ages 6-8), by social valuing or prestige (ages 9-13), and then by internal, unique self (14 and older). Options eliminated early are often never reconsidered, so counselors can help children revisit them.

References: Sharf (Career Development Theory) — Gottfredson's theory of circumscription and compromise: orientation to sex roles

☐ Approve ☐ Revise ☐ Retire Notes:

In couples sessions, one partner regularly rolls her eyes, mocks her spouse's tone, and calls him "pathetic." According to Gottman's research, which of the Four Horsemen is this, and the single strongest predictor of divorce?

A. Criticism

Criticism attacks the partner's character, but it lacks the superiority and disgust that mark contempt.

B. Stonewalling

Stonewalling is emotional withdrawal from the interaction, not mockery or eye-rolling.

C. Defensiveness

Defensiveness is self-protection through excuses or counterattack, not a display of disgust.

D. Contempt ✓ **KEY**

Eye-rolling, mockery and name-calling express contempt, which Gottman found to be the strongest predictor of divorce.

Rationale: Gottman's Four Horsemen are criticism, contempt, defensiveness and stonewalling. Contempt communicates superiority and disgust through sarcasm, mockery, eye-rolling and insults, and it was the single best predictor of divorce in his studies. The antidote is building a culture of appreciation and respect.

References: Gladding (Counseling: A Comprehensive Profession) — Marriage, couple and family counseling: Gottman's research on the Four Horsemen

► **Check:** Confirm the Gladding section that covers Gottman's Four Horsemen.

☐ Approve ☐ Revise ☐ Retire Notes:

A newly formed stepfamily seeks counseling because the 13-year-old daughter openly defies her new stepfather's rules, and he responds by giving stricter punishments. Which recommendation is MOST consistent with research on stepfamily adjustment?

A. Have the stepfather take over discipline so the teen accepts him as the new head of the home

Early stepparent discipline before a bond forms tends to increase conflict and resentment.

B. Have the biological parent lead discipline while the stepparent builds a warm relationship first ✓ **KEY**

Stepfamily research supports the biological parent handling discipline early while the stepparent focuses on connection.

C. Have both adults avoid setting any rules until the teen says she is ready to accept the stepfather

Removing structure leaves the teen without limits and puts her in charge of the family's functioning.

D. Have the teen spend more time with her other parent until she is willing to follow the new rules

Using contact with the other parent as leverage triangulates the teen and can deepen loyalty conflicts.

Rationale: Stepfamilies form after losses and bring loyalty binds. Stepparents gain authority gradually, after a relationship exists, so the biological parent usually remains the primary disciplinarian at first while the stepparent acts more like a supportive adult. The couple still agrees on house rules together.

References: Gladding (Counseling: A Comprehensive Profession) — Family counseling: blended families and stepparent roles

► **Check:** Confirm the Gladding section that covers stepfamily discipline roles.

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor is about to begin conjoint counseling with a married couple. Each partner may also call or meet with the counselor alone from time to time. What should the counselor establish at the outset?

A. A promise that any individual disclosure will stay sealed from the other partner

Promising to hold secrets can trap the counselor in a hidden alliance and undermine the couple work.

B. An agreement that each partner will be assigned their own separate counselor

Separate counselors are an option but do not answer how this counselor will handle the couple case.

C. A clear policy, agreed by both, on how information shared individually is handled ✓ **KEY**

Clarifying who the client is and how individual information will be handled is required at the outset of multiple-client work.

D. A rule that the counselor will refuse to speak to either partner outside sessions

Some contact outside joint sessions can be appropriate; the issue is a clear policy, not a ban.

Rationale: When counseling more than one person who share a relationship, counselors clarify at the outset who the client is and the limits of confidentiality, including whether a no-secrets or limited-secrets policy applies. Both partners agree to that policy before individual contacts happen.

References: ACA Code of Ethics — A.8 Multiple Clients; B.4.b Couples and Family Counseling

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor uses Schlossberg's transition model with a 58-year-old woman who has just retired early to care for her spouse. Which set of factors does the model use to assess her resources for coping?

A. Situation, self, support and strategies ✓ KEY

Schlossberg's 4 S's are situation, self, support and strategies, used to weigh a person's coping resources.

B. Denial, anger, bargaining and acceptance

These are stages from Kübler-Ross's grief model, not Schlossberg's factors for coping with a transition.

C. Growth, exploration, establishment and decline

These are life stages in Super's career theory, not resources for coping with a transition.

D. Shock, withdrawal, adjustment and integration

This is a generic phase sequence, not the four coping-resource factors of Schlossberg's model.

Rationale: Schlossberg defines a transition as any event or non-event that changes roles, relationships, routines and assumptions. Coping is assessed through the 4 S's: the situation (timing, control, concurrent stress), the self (personal and psychological resources), support (relationships and institutions) and strategies (ways of coping).

References: Sharf (Career Development Theory) — Adult career transitions: Schlossberg's transition model (4 S's)

► **Check:** Confirm Sharf covers Schlossberg's 4 S model (adult transitions).

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old client lives with his parents, has changed jobs and majors several times, and says he feels "not really a kid, but not quite an adult." He is still exploring identity, work and relationships. Which concept BEST fits this period?

A. Identity foreclosure

Foreclosure is commitment without exploration; this client is exploring and has not committed.

B. Midlife transition

Levinson's midlife transition occurs around age 40, not in the early twenties.

C. Generativity versus stagnation

Erikson's generativity stage concerns middle adulthood and guiding the next generation.

D. Emerging adulthood ✓ KEY

Arnett described emerging adulthood (about ages 18 to 25) as a time of identity exploration and feeling in-between.

Rationale: Arnett proposed emerging adulthood as a distinct period, roughly ages 18 to 25 in industrialized societies, marked by identity exploration, instability, self-focus, feeling in-between and a sense of possibilities. Counselors can frame this exploration as developmentally typical rather than as failure to launch.

References: Berk (Development Through the Lifespan) — Early adulthood: Arnett's emerging adulthood

☐ Approve ☐ Revise ☐ Retire Notes:

A 79-year-old widower in assisted living spends sessions reviewing his life, some proud moments and some regrets, and asks whether his life "added up to anything." Which of Erikson's psychosocial stages is he working through?

A. Generativity versus stagnation

Generativity is the middle-adulthood task of guiding the next generation and being productive.

B. Intimacy versus isolation

Intimacy is the young-adult task of forming close, committed relationships.

C. Integrity versus despair ✓ KEY

Late adulthood centers on accepting one's life as meaningful (integrity) rather than feeling regret (despair).

D. Identity versus role confusion

Identity versus role confusion is the adolescent task of forming a coherent sense of self.

Rationale: Erikson's final stage, integrity versus despair, involves looking back and accepting one's life as it was. Life review, described by Butler, is a structured way to help older adults integrate successes and regrets and move toward integrity.

References: Berk (Development Through the Lifespan) — Late adulthood: Erikson's ego integrity versus despair; life review

☐ Approve ☐ Revise ☐ Retire Notes:

An adult daughter brings her 74-year-old mother to counseling, worried about "early dementia." Over two months the mother has lost interest in her garden, sleeps poorly, and often says "I just can't remember anything anymore." She seems distressed by her memory lapses. What should the counselor do FIRST?

A. Screen for depression and refer for a medical evaluation of the memory complaints ✓ KEY

Depression in older adults can mimic cognitive decline, and a medical workup rules out treatable causes.

B. Tell the family that memory loss of this kind is a normal part of growing older

Noticeable decline with mood change is not normal aging and should not be dismissed.

C. Begin memory-training exercises in sessions to slow the progression of dementia

This assumes a dementia diagnosis that has not been made and skips assessment of depression.

D. Recommend that the daughter begin looking into memory-care residential placement

Placement planning is premature without assessment and could cause harm if the cause is depression.

Rationale: Late-life depression often presents with memory and concentration complaints, sometimes called pseudodementia. Clues include recent onset with mood symptoms and a person who is distressed by and emphasizes their lapses. The counselor screens for depression (for example with the Geriatric Depression Scale) and coordinates a medical evaluation.

References: DSM-5-TR — Major depressive disorder: diminished ability to think or concentrate; differential with major neurocognitive disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A client who uses a wheelchair after a spinal cord injury says, "My body is not the problem. The problem is buildings without ramps and managers who assume I can't do the job." Which model of disability does her statement reflect?

A. Medical model

The medical model locates disability in the individual's impairment, to be diagnosed and treated.

B. Moral model

The moral model treats disability as a sign of sin or a test of character.

C. Rehabilitation model

This model focuses on restoring individual function, still locating the problem in the person.

D. Social model ✓ KEY

The social model locates disability in environmental and attitudinal barriers, not in the person.

Rationale: The social model distinguishes impairment (a bodily difference) from disability (exclusion created by inaccessible environments and attitudes). Counselors working from it attend to barriers, advocacy and accommodation as well as the client's adjustment.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling individuals with disabilities: medical versus social models of disability

► **Check:** Confirm Sue & Sue's disability coverage; 'rehabilitation model' distractor is a looser term than medical/moral/social.

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client with type 1 diabetes often skips glucose checks and insulin because, she says, "thinking about it makes it too real." Her blood sugar control has worsened and she has had two ER visits. She reports no persistent low mood, worry about having other illnesses, or other psychiatric symptoms. Which DSM-5-TR diagnosis BEST fits?

A. Illness anxiety disorder of the care-avoidant type

Illness anxiety involves preoccupation with having or getting a serious illness, which she does not report.

B. Psychological factors affecting other medical conditions ✓ KEY

Her avoidance is a psychological factor that worsens a diagnosed medical condition, with no other mental disorder.

C. Somatic symptom disorder with a persistent course

Somatic symptom disorder requires distressing physical symptoms with excessive related thoughts, not avoidance of care.

D. Factitious disorder imposed on self, with recurrent episodes

Factitious disorder involves deliberately falsifying or inducing illness, which is not described here.

Rationale: Psychological factors affecting other medical conditions applies when psychological or behavioral factors, such as denial or poor adherence, adversely affect a medical condition and are not better explained by another mental disorder. It is listed with somatic symptom and related disorders but does not require distressing somatic symptoms.

References: DSM-5-TR — Psychological Factors Affecting Other Medical Conditions: criteria A-B

☐ Approve ☐ Revise ☐ Retire Notes:

A bisexual client has never experienced overt harassment, yet she hides her identity at work, constantly scans for signs of rejection, and says part of her believes something is wrong with her. In Meyer's minority stress model, these experiences are BEST described as which kind of stressors?

A. Distal stressors from discrimination

Distal stressors are external events such as harassment or discrimination, which she has not experienced.

B. General life stressors shared by all

These stressors are tied specifically to her stigmatized identity, not common to everyone.

C. Proximal stressors tied to stigma ✓ KEY

Concealment, expected rejection and internalized stigma are Meyer's proximal (internal) minority stressors.

D. Coping resources within the community

Community connection is a protective factor in the model, not a source of stress.

Rationale: Meyer's model separates distal stressors (external prejudice events, discrimination, violence) from proximal stressors (expectations of rejection, concealment and internalized homophobia or biphobia). Both add to the general stress everyone faces, and community connection and coping can buffer their effects on mental health.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ individuals: minority stress

☐ Approve

☐ Revise

☐ Retire

Notes:

The parents of a 16-year-old who recently came out as gay ask a counselor to "help him become straight." The teen does not want to change his orientation. How should the counselor respond?

A. Decline to attempt orientation change and offer support for the teen and family ✓ KEY

Orientation change efforts lack scientific support and carry risk of harm; family support work is appropriate.

B. Agree to the goal because parents hold legal authority over a minor's treatment

Parental consent does not make a harmful, unsupported technique ethical to use.

C. Refer the family to a provider who specializes in sexual orientation change work

Referring for a harmful practice still facilitates it and does not protect the client.

D. Explore with the teen whether his orientation might be a phase that will soon pass

Framing his identity as a phase invalidates it and reflects the parents' goal, not his own.

Rationale: The ACA Code bars techniques with substantial evidence of harm even when requested, and sexual orientation change efforts fall in that category. The counselor can decline that goal while still helping the family: providing education, supporting the parents' reactions and reducing family rejection, which is a strong risk factor for LGBTQ+ youth.

References: ACA Code of Ethics — C.7.a Scientific Basis for Treatment; C.7.c Harmful Practices

☐ Approve

☐ Revise

☐ Retire

Notes:

A man who immigrated from Vietnam 15 years ago has stopped speaking Vietnamese, no longer observes family traditions, and says he identifies only with mainstream American culture. Which of Berry's acculturation strategies does this describe?

A. Integration

Integration keeps the heritage culture while also taking part in the host culture.

B. Assimilation ✓ KEY

Assimilation means giving up heritage culture and adopting the host culture.

C. Separation

Separation keeps the heritage culture while avoiding the host culture.

D. Marginalization

Marginalization means little connection to either the heritage or host culture.

Rationale: Berry's model crosses two questions: is the heritage culture maintained, and is contact with the host society sought? Yes and yes is integration; no and yes is assimilation; yes and no is separation; no and no is marginalization.

References: Sue & Sue (Counseling the Culturally Diverse) — Acculturation: Berry's four acculturation strategies

☐ Approve ☐ Revise ☐ Retire Notes:

A refugee teen has drifted away from her parents' language and customs but also feels unwelcome and isolated among peers at her new school. In Berry's framework, which strategy does this reflect, and how does it generally compare with the others in acculturative stress?

A. Separation, generally tied to the lowest acculturative stress

Separation keeps heritage culture; she has moved away from hers, and integration, not separation, is lowest in stress.

B. Integration, generally tied to the highest acculturative stress

She does not hold both cultures, and integration is usually associated with the least stress.

C. Assimilation, generally tied to the lowest acculturative stress

She has not joined the host culture, and assimilation is not the lowest-stress strategy.

D. Marginalization, generally tied to the highest acculturative stress ✓ KEY

Losing ties to both cultures is marginalization, which Berry found carries the greatest acculturative stress.

Rationale: In Berry's research, integration is generally associated with the best adaptation and least acculturative stress, while marginalization, low engagement with both cultures, is associated with the most. Assimilation and separation fall in between. Counselors can help marginalized clients rebuild connection to one or both cultures.

References: Sue & Sue (Counseling the Culturally Diverse) — Acculturation and acculturative stress: Berry's model

☐ Approve ☐ Revise ☐ Retire Notes:

A devout client feels intense guilt about her divorce because of her faith community's teachings. The counselor is not religious and privately views the teachings as harmful. Which approach is MOST appropriate?

A. Encourage her to question the teachings that are leading her to feel guilty

Steering her away from her beliefs imposes the counselor's values on the client.

B. Explore her guilt within her own faith framework and the resources it offers ✓ **KEY**

Working within the client's belief system respects her values and can draw on her faith as a resource.

C. Refer her to clergy because religious concerns fall outside counseling scope

Spiritual concerns are within counseling scope; referral based on the topic alone is not warranted.

D. Disclose the counselor's own views so she can weigh both sides of the issue

Sharing personal views on her religion serves the counselor and risks pressuring the client.

Rationale: Counselors avoid imposing their own values, attitudes and beliefs and respect the diversity of clients' spiritual lives. Exploring guilt within the client's framework, including forgiveness or grace in her tradition, honors her worldview and can turn faith into a source of coping.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old discloses repeated nonsuicidal self-injury and says, "When everything builds up, it is the only thing that makes the feeling stop." Which function of self-injury does her statement MOST clearly reflect?

A. A wish to end her life that has not yet been spoken aloud

NSSI by definition lacks suicidal intent; her words describe emotional relief, not a wish to die.

B. A bid to get attention from peers and adults around her

NSSI is often hidden; attributing it to attention-seeking is a common, inaccurate assumption here.

C. Relief from intense negative emotion (affect regulation) ✓ **KEY**

Affect regulation, gaining relief from overwhelming feelings, is the most common reported function of NSSI.

D. Self-punishment for having broken a rule in her family

Self-punishment is a possible function, but she describes stopping a feeling, not punishing herself.

Rationale: Research consistently finds that the most common function of nonsuicidal self-injury is affect regulation: obtaining relief from a negative feeling or cognitive state. DSM-5-TR lists this expectation in its proposed criteria for nonsuicidal self-injury disorder. Treatment builds alternative regulation skills, as in DBT.

References: DSM-5-TR — Conditions for Further Study: Nonsuicidal Self-Injury Disorder, criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

A college student shows the counselor healed marks on her forearm and says she has hurt herself during stressful weeks for two years. She denies wanting to die. Which counselor response is BEST?

A. Ask her to sign a no-harm contract before agreeing to continue in counseling

No-harm contracts lack evidence of benefit and can shut down honest disclosure.

B. Arrange an inpatient admission because self-injury signals imminent danger

NSSI without suicidal intent or other acute risk does not by itself call for hospitalization.

C. Respond calmly, explore what the behavior does for her, and assess suicide risk ✓ **KEY**

A calm, curious stance keeps her engaged, and NSSI still raises suicide risk, so risk is assessed.

D. Avoid discussing the marks so that attention does not reinforce the behavior

Ignoring the disclosure misses assessment and can feel dismissive, discouraging further honesty.

Rationale: A respectful, nonjudgmental response to self-injury builds trust and allows functional assessment. NSSI is distinct from a suicide attempt but is a strong risk factor for later suicidal behavior, so counselors also assess suicidal ideation and behavior directly, for example with the C-SSRS.

References: C-SSRS — Suicidal behavior section: distinguishing nonsuicidal self-injurious behavior from suicide attempts

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor is using cognitive behavioral therapy for insomnia (CBT-I) with a client who lies awake in bed for hours most nights. Which instruction is part of the stimulus control component?

A. Avoid caffeine after noon and keep the bedroom cool, dark and quiet

These are sleep hygiene recommendations, a separate and weaker component of insomnia care.

B. Limit time in bed to the hours actually slept, then extend it slowly

This describes sleep restriction, a different CBT-I component that consolidates sleep.

C. Practice progressive muscle relaxation for twenty minutes at bedtime

Relaxation training is a separate CBT-I component aimed at lowering arousal.

D. Get out of bed if unable to sleep and return only once feeling sleepy ✓ **KEY**

Stimulus control re-links the bed with sleep by leaving it when awake and returning when sleepy.

Rationale: Stimulus control therapy aims to restore the bed and bedroom as cues for sleep: go to bed only when sleepy, use the bed only for sleep and sex, leave the bed when unable to sleep, keep a fixed wake time and avoid daytime naps. It is combined with sleep restriction, cognitive work and education in CBT-I, the recommended first-line treatment for chronic insomnia.

References: NICE guidelines — Insomnia: CBT-I as first-line treatment for long-term insomnia

► **Check:** NICE insomnia guidance is mainly the Clinical Knowledge Summary; citation placement is loose.

☐ Approve ☐ Revise ☐ Retire Notes: _____

A client reports difficulty staying asleep, with daytime fatigue that affects her work. The problem is not explained by a medical condition, substance or other sleep disorder. According to DSM-5-TR, how often and for how long must the sleep difficulty occur to meet criteria for insomnia disorder?

A. At least 3 nights a week for at least 3 months ✓ KEY

DSM-5-TR requires the difficulty at least 3 nights per week, present for at least 3 months.

B. At least 5 nights a week for at least 2 weeks

This is far shorter than required; two weeks fits a brief, situational sleep problem.

C. At least 1 night a week for at least 6 months

One night a week is below the frequency threshold for insomnia disorder.

D. At least 4 nights a week for at least 1 month

One month falls short of the 3-month duration; briefer insomnia is coded as other specified insomnia disorder.

Rationale: DSM-5-TR insomnia disorder requires dissatisfaction with sleep quantity or quality (trouble initiating sleep, maintaining sleep or early waking) occurring at least 3 nights per week for at least 3 months, despite adequate opportunity for sleep, with clinically significant distress or impairment.

References: DSM-5-TR — Insomnia Disorder: criteria C and D (frequency and duration)

☐ Approve ☐ Revise ☐ Retire Notes: _____

A client with bipolar I disorder who takes lithium had a stomach virus with vomiting for two days. In session she has a coarse hand tremor, seems confused, and walks unsteadily. What should the counselor do?

A. Reassure her that tremor is a common side effect and continue the planned session

A mild tremor can occur on lithium, but confusion and unsteady gait after dehydration suggest toxicity.

B. Suggest she skip her next lithium dose until she feels better, then call her prescriber

Advising medication changes is outside counselor scope, and the symptoms need urgent evaluation now.

C. Arrange immediate medical evaluation because the symptoms may signal lithium toxicity ✓ KEY

Dehydration can raise lithium levels; confusion, coarse tremor and ataxia warrant urgent medical care.

D. Explore whether anxiety about her illness is causing her shaking and trouble focusing

Attributing these signs to anxiety risks missing a medical emergency.

Rationale: Lithium has a narrow therapeutic range and requires regular blood-level monitoring. Vomiting, diarrhea, dehydration and some medications can raise levels into the toxic range, producing coarse tremor, confusion, ataxia and gastrointestinal symptoms. Counselors do not adjust medication but recognize warning signs and arrange urgent medical evaluation.

References: NICE guidelines — Bipolar disorder (CG185): lithium monitoring and toxicity

☐ Approve ☐ Revise ☐ Retire Notes: _____

A client with major depressive disorder started an SSRI prescribed by her physician 10 days ago. She says, "It is not doing anything, so I am going to stop taking it." What is the counselor's BEST response?

A. Agree that stopping makes sense since there has been no improvement

Ten days is too early to judge, and stopping without the prescriber is not the counselor's call to endorse.

B. Explain SSRIs often take weeks to work; urge a talk with the prescriber ✓ **KEY**

SSRI benefit usually builds over several weeks, and medication decisions belong with the prescriber.

C. Suggest she ask her physician for a benzodiazepine to work more quickly

Recommending a specific drug is outside counselor scope, and benzodiazepines carry dependence risk.

D. Advise her to take half the dose for a week to see if she feels different

Counselors do not advise dose changes; that is a prescriber decision.

Rationale: SSRIs typically take several weeks to produce a full antidepressant effect, although side effects can appear sooner. Counselors provide basic education, support adherence and coordinate with the prescriber, but they do not recommend starting, stopping or changing specific medications.

References: APA CPG — Depression across the lifespan: second-generation antidepressants · ACA Code of Ethics — C.2.a Boundaries of Competence

☐ Approve ☐ Revise ☐ Retire Notes:

104. nce-s-clf-056 · Human Growth and Development · school and academic concerns · medium · In review

A 9-year-old with ADHD performs at grade level when given extended test time and a seat near the teacher. He does not need specially designed instruction. Which school plan BEST fits his needs?

A. An Individualized Education Program under IDEA

An IEP is for students who need specially designed instruction, which he does not.

B. A gifted education plan for advanced placement

Nothing indicates a need for gifted programming.

C. A behavior intervention plan for disruptive acts

A behavior plan targets problem behaviors; the need described is for accommodations.

D. A Section 504 plan for classroom accommodations ✓ **KEY**

Section 504 provides accommodations for a disability that limits learning without special education.

Rationale: Section 504 of the Rehabilitation Act covers students with a disability that substantially limits a major life activity such as learning, and provides accommodations such as extended time or preferential seating. IDEA provides an IEP only for students in eligible categories who need specially designed instruction.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: Section 504 plans and IEPs under IDEA

☐ Approve ☐ Revise ☐ Retire Notes:

An 8-year-old has missed most of three weeks of school, crying and complaining of stomachaches each morning that ease once his mother lets him stay home. He worries something bad will happen to her while he is away. Pediatric causes have been ruled out. Which plan is MOST appropriate?

A. Arrange home-based instruction until his worry about his mother eases on its own

Staying home relieves anxiety in the short term but reinforces avoidance and lets the fear grow.

B. A gradual return to school with exposure, coping skills and school collaboration ✓ **KEY**

Graduated exposure with parent and school coordination is the evidence-based approach to anxiety-based school refusal.

C. Tell his mother to keep him home on any morning that he has physical complaints

Making symptoms the ticket to stay home rewards them and maintains the cycle.

D. Delay any school plan until weekly play therapy has resolved the underlying fear

Each week out of school makes return harder; return should begin alongside treatment.

Rationale: School refusal driven by separation anxiety is maintained by avoidance, because staying home brings immediate relief. Treatment uses CBT with graduated exposure to attending school, coping skills, parent training to reduce accommodation, and close collaboration with school staff to support a prompt, stepwise return.

References: DSM-5-TR — Separation Anxiety Disorder: reluctance or refusal to go to school; physical complaints on anticipated separation

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old Army veteran meets criteria for PTSD after combat deployments. He is willing to engage in therapy and asks what treatment has the strongest support. According to the VA/DoD clinical practice guideline, which approach is recommended as first-line?

A. Individual, manualized trauma-focused therapy such as PE or CPT ✓ **KEY**

The VA/DoD guideline recommends manualized trauma-focused psychotherapies such as PE, CPT and EMDR first.

B. A benzodiazepine to reduce his hyperarousal and improve sleep

The guideline recommends against benzodiazepines for PTSD because of harm and lack of benefit.

C. Single-session psychological debriefing about his deployments

Psychological debriefing is not supported for PTSD and may worsen outcomes.

D. Nondirective supportive counseling without trauma processing

Supportive counseling is less effective than trauma-focused therapy and is not first-line.

Rationale: The VA/DoD PTSD guideline recommends individual, manualized trauma-focused psychotherapies, chiefly prolonged exposure (PE), cognitive processing therapy (CPT) and EMDR, over pharmacotherapy. It recommends against benzodiazepines for PTSD. Counselors working with veterans should know these options and refer when they are not trained to deliver them.

References: VA/DoD CPG — Management of PTSD and Acute Stress Disorder: trauma-focused psychotherapy recommendations

► **Check:** How VA/DoD 2023 grades debriefing and supportive counseling (distractors) is from memory; verify.

☐ Approve ☐ Revise ☐ Retire Notes:

A Marine veteran says his worst memory is not being in danger himself but following an order that led to civilian deaths. He is consumed by guilt and shame, believes he is "a bad person," and has pulled away from his faith community. Which concept BEST describes his distress?

A. Moral injury from acts that violated his own deeply held moral beliefs ✓ KEY

Moral injury describes guilt, shame and loss of trust after perpetrating or witnessing acts that violate one's moral code.

B. Compassion fatigue from caring for wounded civilians and fellow troops

Compassion fatigue affects helpers worn down by caregiving; his distress centers on his own actions.

C. Survivor guilt from having lived through events in which comrades died

Survivor guilt centers on surviving when others did not; his guilt is about what he did.

D. Adjustment disorder triggered by the stress of returning to civilian life

His distress is tied to a specific morally violating combat event, not to the transition home.

Rationale: Moral injury, described by Litz and colleagues, is lasting distress after perpetrating, failing to prevent or witnessing acts that transgress deeply held moral beliefs. It shows up mainly as guilt, shame, self-condemnation and spiritual or relational withdrawal rather than fear. It overlaps with PTSD criterion D (negative beliefs about oneself, guilt, shame) but is not a DSM diagnosis.

References: DSM-5-TR — PTSD criterion D: persistent negative beliefs about oneself and persistent guilt or shame

► **Check:** Moral injury (Litz et al.) is not a DSM construct; the DSM-5-TR reference points to PTSD criterion D only.

☐ Approve ☐ Revise ☐ Retire Notes:

Treatment Planning

17 items · NCE weight 9% (14 of 160 scored items)

108. nce-s-trt-001 · Counseling and Helping Relationships · SMART goals and measurable objectives · easy · In review

A counselor is writing a treatment plan for a 34-year-old client with social anxiety. Which objective is BEST written according to SMART criteria?

A. Client will feel much more confident in social settings by the end of treatment.

Feeling "more confident" is not observable or measurable, and "end of treatment" is not a specific time frame.

B. Client will start one conversation with a coworker at least 3 days a week within 6 weeks. ✓ **KEY**

This is specific, measurable (frequency count), achievable, relevant to social anxiety, and time-bound (6 weeks).

C. Client will understand the cognitive roots of social anxiety and work through them fully.

"Understand" and "work through" are vague internal processes with no measurable criterion or deadline.

D. Client will attend weekly counseling sessions and complete homework as the counselor assigns.

Attendance and compliance describe the treatment process, not a measurable change in the presenting problem.

Rationale: SMART objectives are Specific, Measurable, Achievable, Relevant and Time-bound. A good objective names an observable behavior, a criterion for success, and a deadline, so progress can be tracked and the plan revised. Vague feeling states and process statements (attendance) do not meet the standard.

References: Wiger (Documentation) — Treatment plans: behaviorally measurable goals and objectives

► **Check:** Wiger reference detail describes the concept, not an exact chapter title.

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old client with an alcohol use disorder has relapsed twice while attending weekly outpatient sessions. There is no withdrawal risk, no acute medical or psychiatric concern, and the client has stable housing and a supportive partner. Which level of care is MOST appropriate?

A. Medically managed intensive inpatient treatment in a hospital setting

Hospital-level care is for severe, unstable withdrawal or acute medical/psychiatric needs, none of which are present.

B. Long-term residential treatment with 24-hour structure and support

Residential care is more restrictive than needed for a client with stable housing and a supportive recovery environment.

C. An intensive outpatient program with several sessions per week ✓ **KEY**

Repeated relapse despite weekly outpatient care, with no withdrawal, medical or housing barriers, points to a step up to the next more intensive outpatient level.

D. Continue weekly outpatient sessions with no change to the treatment plan

Repeated relapse at the current intensity signals that the present level is not meeting the client's needs.

Rationale: Placement criteria such as the ASAM Criteria match clients to the least restrictive level of care that safely meets their needs across multiple dimensions (withdrawal, medical, emotional, readiness, relapse potential, recovery environment). When a client is not progressing at one level and has no acute risks, the usual step is to the next more intensive level, not a jump to residential or hospital care.

References: ASAM Criteria — Multidimensional assessment and least restrictive level of care; outpatient vs intensive outpatient

► **Check:** ASAM Criteria 4th ed. (2023) renamed some levels; confirm the level-of-care wording is current.

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old veteran meets criteria for posttraumatic stress disorder after a combat deployment. According to major clinical practice guidelines, which psychotherapy approach has the strongest research support as a first-line treatment?

A. Trauma-focused therapy such as cognitive processing therapy or prolonged exposure ✓ **KEY**

APA and VA/DoD guidelines give their strongest recommendations to trauma-focused therapies such as CPT and PE.

B. A single session of psychological debriefing held soon after returning home

Single-session debriefing is not recommended; it has not been shown to prevent or treat PTSD and may be harmful.

C. Open-ended nondirective supportive counseling focused on current life stressors

Nondirective supportive counseling is often used as a control condition and is not a first-line PTSD treatment.

D. Progressive muscle relaxation training used alone as the primary treatment

Relaxation may help with arousal, but used alone it lacks the support of trauma-focused treatments for PTSD.

Rationale: Evidence-based treatment selection integrates the best research evidence with clinical expertise and client preferences. For PTSD, the APA (2017) and VA/DoD guidelines recommend trauma-focused psychotherapies such as cognitive processing therapy and prolonged exposure as first-line treatments.

References: APA CPG — Clinical Practice Guideline for the Treatment of PTSD (2017): CPT, PE, CBT and CT strongly recommended · VA/DoD CPG — Management of PTSD and Acute Stress Disorder: trauma-focused psychotherapy as first-line treatment

► **Check:** VA/DoD 2023 reference detail is paraphrased; confirm wording.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor has completed an intake with a 52-year-old Korean American client who reports grief and insomnia after her husband's death. What is the BEST way for the counselor to develop the treatment plan?

- A. Write the plan alone based on the diagnosis, then review it with the client next week
Presenting a finished plan treats the client as a recipient rather than a partner and misses her priorities.
- B. Use a standard grief protocol so the client receives the same plan as other clients
A one-size-fits-all plan ignores individual goals, cultural values and preferences.
- C. Ask the client's adult children what goals they would like their mother to work on
Family input may be welcome with consent, but the client herself is the primary partner in setting her goals.
- D. Work with the client to identify her goals and the methods she finds acceptable ✓ KEY
Collaborative planning engages the client in choosing goals and methods, respecting her values and culture.

Rationale: Treatment plans should be developed jointly with the client. Collaboration respects autonomy, incorporates cultural values and preferences, and increases engagement and follow-through. The ACA Code directs counselors and clients to work jointly in devising counseling plans that offer reasonable promise of success.

References: ACA Code of Ethics — A.1.c Counseling Plans

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor gives a 23-year-old client the PHQ-9 at every session. After eight sessions of treatment for depression, the client's scores have not meaningfully changed from intake. What should the counselor do NEXT?

- A. Stop giving the PHQ-9, since repeated testing seems to be discouraging the client
Dropping the measure removes the very data that revealed the lack of progress.
- B. Review the scores with the client and revise the treatment plan together ✓ KEY
Measurement-based care uses nonresponse as a signal to discuss progress with the client and adjust the plan.
- C. Continue the same approach unchanged, since depression often improves slowly
Ignoring eight sessions of flat scores delays a needed change in treatment.
- D. Refer the client out at once, since the lack of change shows a poor counselor fit
Flat scores call for review and adjustment first; immediate referral is premature.

Rationale: Routine outcome monitoring with a standardized measure lets the counselor detect clients who are not responding. Feedback on scores should be shared with the client and used to revisit goals, interventions or level of care. Measurement-based care has been shown to improve outcomes, especially for clients at risk of treatment failure.

References: Hays (Assessment) — Outcome assessment and progress monitoring with standardized measures

☐ Approve ☐ Revise ☐ Retire Notes: _____

During a first session, a 47-year-old client mentions that she also sees a psychiatrist who prescribes her antidepressant. What should the counselor do to coordinate care?

A. Call the psychiatrist right away to share the intake findings and diagnosis

Sharing information without the client's authorization breaches confidentiality.

B. Ask the client to stop seeing the psychiatrist to avoid conflicting treatment

Medication management and counseling are complementary; there is no basis for asking her to stop.

C. Request the client's written consent to inform and consult with the psychiatrist ✓ KEY

The ACA Code directs counselors to request a release so they can inform the other professional and collaborate.

D. Note the psychiatrist in the file and leave all contact entirely up to the client

This is passive; counselors are expected to seek permission to coordinate with other providers.

Rationale: When counselors learn that a client is in a professional relationship with another mental health professional, they request a release to inform that professional and work toward a collaborative relationship. Coordination improves safety and consistency of care, while written consent protects confidentiality.

References: ACA Code of Ethics — A.3 Clients Served by Others

☐ Approve ☐ Revise ☐ Retire Notes:

A program director is training new counselors at a community mental health agency. When should discharge and aftercare planning ideally begin for a new client?

A. At intake, revisiting the plan throughout the course of treatment ✓ KEY

Planning for discharge from the start keeps treatment goal-directed and prepares the client for transition.

B. In the final session, once the client has met every treatment goal

Waiting until the last session leaves too little time to arrange supports and aftercare.

C. When the insurer signals that authorized sessions are running out

Letting payer limits drive discharge timing is reactive and can leave gaps in care.

D. Only after the client asks to end treatment or stops attending

This leaves aftercare to chance and misses the chance to plan termination together.

Rationale: Discharge and aftercare planning should begin at admission and be updated as treatment progresses. Early planning defines what successful completion looks like, identifies community supports and step-down services, and reduces the risk of relapse or abrupt termination.

References: Wiger (Documentation) — Discharge planning and the discharge summary

► **Check:** Wiger reference detail describes the concept, not an exact chapter title.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is completing a Stanley-Brown Safety Planning Intervention with a 19-year-old college student who has recent suicidal ideation. Which step does the safety plan address FIRST?

A. Listing professionals and crisis lines to contact in an emergency

Professional and crisis contacts come later in the plan, after self-directed and social steps.

B. Making the environment safe by limiting access to lethal means

Means safety is a key part of the plan but is addressed as the final step.

C. Naming people the student can ask directly for help in a crisis

Reaching out to trusted people for help follows warning signs and internal coping.

D. Recognizing personal warning signs that a crisis may be developing ✓ KEY

The plan begins with identifying warning signs so the client knows when to use the plan.

Rationale: The Stanley-Brown Safety Plan proceeds in order: (1) warning signs, (2) internal coping strategies, (3) people and social settings that provide distraction, (4) people to ask for help, (5) professionals and agencies to contact, and (6) making the environment safe. Recognizing warning signs comes first because it tells the client when to begin using the plan.

References: Stanley-Brown SPI — Six-step safety plan: Step 1, recognizing warning signs

☐ Approve ☐ Revise ☐ Retire Notes:

116. nce-s-trt-009 · Counseling and Helping Relationships · stages of change and treatment readiness · easy · In review

A 38-year-old client says, "I know my drinking is hurting my marriage, and part of me wants to cut back, but I'm just not sure I'm ready to give it up." Which stage of change does this statement BEST reflect?

A. Precontemplation

In precontemplation the person does not see a problem or intend to change; this client recognizes the problem.

B. Contemplation ✓ KEY

Acknowledging the problem while feeling ambivalent about changing is the hallmark of contemplation.

C. Preparation

Preparation involves a commitment to act soon and small steps toward change, which are absent here.

D. Action

Action involves actively modifying the behavior, which this client has not begun.

Rationale: In the Transtheoretical Model, contemplation is marked by awareness that a problem exists and serious thought about change, combined with ambivalence and no firm commitment. Counselors often use motivational interviewing to explore and resolve that ambivalence.

References: Transtheoretical Model — Stages of change: contemplation and ambivalence

☐ Approve ☐ Revise ☐ Retire Notes:

A 60-year-old client stopped smoking four months ago. He uses nicotine gum, avoids places where he used to smoke, and calls a friend when cravings hit. According to Prochaska and DiClemente, which stage is he in?

A. Preparation

Preparation precedes the behavior change; this client has already quit.

B. Maintenance

Maintenance generally begins after about six months of sustained change; he is at four months.

C. Action ✓ KEY

He has changed the behavior within the past six months and is actively using change strategies.

D. Contemplation

Contemplation involves thinking about change without acting; he has already acted.

Rationale: The action stage covers roughly the first six months after a person has made an overt change in behavior, when effort and relapse risk are highest. Maintenance begins once the change has been sustained for more than about six months and focuses on preventing relapse.

References: Transtheoretical Model — Action vs maintenance: the six-month criterion

► **Check:** Action vs. maintenance rests on the conventional ~6-month line in the Transtheoretical Model.

☐ Approve ☐ Revise ☐ Retire Notes:

Three months into a new school-based anger management group, a counselor gathers feedback from students and teachers so she can refine the sessions while the program is still running. This is an example of which type of evaluation?

A. Formative evaluation ✓ KEY

Formative evaluation occurs during implementation to improve the program as it runs.

B. Summative evaluation

Summative evaluation judges overall effectiveness or outcomes after the program is completed.

C. Needs assessment

A needs assessment happens before a program is designed, to identify gaps and priorities.

D. Cost-benefit analysis

Cost-benefit analysis compares program costs with monetary value of outcomes, not process feedback.

Rationale: Formative evaluation is conducted while a program is under way to give feedback for improvement. Summative evaluation is conducted at the end to judge whether the program achieved its goals and should be continued, expanded or ended.

References: Erford (Research and Evaluation in Counseling) — Program evaluation: formative vs summative evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

Before creating any new services, a rural counseling center surveys residents, local physicians and school staff to find out which mental health concerns are most common and which services are missing. What is the center conducting?

A. An outcome evaluation

Outcome evaluation measures the effects of an existing program; no program exists yet.

B. A process evaluation

Process evaluation examines how an existing program is being delivered.

C. A fidelity review

A fidelity review checks whether a program is delivered as designed, which requires a program in place.

D. A needs assessment ✓ KEY

Gathering data on community problems and service gaps before planning services is a needs assessment.

Rationale: A needs assessment systematically identifies the gap between current conditions and desired conditions in a population, using surveys, key informants, focus groups or existing data. It is typically the first step in program planning and guides which services to develop.

References: Erford (Research and Evaluation in Counseling) — Needs assessment as the first phase of program evaluation

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor records a student's disruptive outbursts during baseline, introduces a token system, removes it, and then reintroduces it (an ABAB design). What is the main purpose of the withdrawal phase?

A. To let the student rest so the token system does not lose its appeal

Withdrawal is not about preventing satiation; it is a test of experimental control.

B. To show that behavior changes when and only when the treatment changes ✓ KEY

A return toward baseline when treatment is removed, and improvement when it returns, supports a causal link.

C. To compare the student's behavior with a randomly assigned control group

Single-subject designs use the person as his or her own control; there is no separate group.

D. To produce enough data points to compute a reliable correlation value

The goal of the reversal is to demonstrate experimental control, not to compute a correlation.

Rationale: In an ABAB (reversal or withdrawal) single-subject design, behavior is measured at baseline (A), during treatment (B), after treatment is withdrawn (A), and after it is reinstated (B). If behavior worsens on withdrawal and improves again on reinstatement, the counselor has stronger evidence that the intervention, not some other factor, caused the change.

References: Erford (Research and Evaluation in Counseling) — Single-subject designs: AB and ABAB (withdrawal) designs

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher delivers a new career-readiness curriculum to one intact ninth-grade class and uses a second existing class at the same school as a comparison. Both classes complete pretests and posttests. Which design is this?

A. A true experimental pretest-posttest control group design

A true experiment requires random assignment of participants to conditions, which did not occur.

B. A single-subject multiple-baseline design across settings

Single-subject designs track individuals over repeated phases, not two classes compared once.

C. A quasi-experimental nonequivalent control group design ✓ KEY

Using intact groups without random assignment, with pre- and posttests, is a nonequivalent group quasi-experiment.

D. A correlational design examining two naturally linked variables

The researcher manipulated an independent variable (the curriculum), so this is not correlational.

Rationale: The defining feature of a true experiment is random assignment to conditions. When a researcher manipulates an independent variable but uses pre-existing groups, the design is quasi-experimental. Pretests help assess initial group differences, but selection remains a threat to internal validity.

References: Erford (Research and Evaluation in Counseling) — Experimental vs quasi-experimental designs; random assignment

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor selects the 15 students with the highest scores on a test anxiety scale for a six-week group. There is no comparison group, all students complete the group, and nothing notable happens at school during that time. Posttest scores are lower. Which threat to internal validity MOST likely accounts for part of this drop?

A. Experimental mortality, or attrition of group members

All students completed the group, so loss of participants cannot explain the change.

B. History, or outside events occurring during the study period

The stem rules out notable outside events during the six weeks.

C. Diffusion of treatment between groups being compared

Diffusion requires a comparison group that is exposed to the treatment; there is none.

D. Statistical regression of extreme scores toward the mean ✓ KEY

Participants selected for extreme scores tend to score closer to the mean on retest even without treatment.

Rationale: Statistical regression is a threat to internal validity whenever participants are chosen because of extreme scores. Because no measure is perfectly reliable, extreme scorers tend to move toward the mean on retesting, which can be mistaken for a treatment effect. A comparison group selected the same way helps rule this out.

References: Erford (Research and Evaluation in Counseling) — Threats to internal validity: statistical regression

☐ Approve ☐ Revise ☐ Retire Notes:

A researcher sets alpha at .05 and finds $p = .03$ when comparing a new intervention with treatment as usual, so she rejects the null hypothesis. If the intervention in fact has no real effect, what has occurred?

A. A Type I error, a false positive finding ✓ KEY

Rejecting a null hypothesis that is actually true is a Type I error; its probability is set by alpha.

B. A Type II error, a false negative finding

A Type II error is failing to reject a false null, missing a real effect; here the null was rejected.

C. A correct decision, since p fell below alpha

A p -value below alpha does not guarantee a true effect; if there is none, rejecting the null was an error.

D. A power failure caused by too small a sample

Low power raises the risk of Type II errors, not of falsely detecting an effect that does not exist.

Rationale: A Type I error (alpha) is rejecting a true null hypothesis, concluding an effect exists when it does not. A Type II error (beta) is failing to reject a false null. The p -value is the probability of results at least this extreme if the null is true; it is not the probability that the null is true, so a significant p can still be a false positive.

References: Erford (Research and Evaluation in Counseling) — Hypothesis testing: Type I and Type II errors, alpha and p -values

☐ Approve ☐ Revise ☐ Retire Notes:

A study of 4,000 clients finds that a brief online module reduces anxiety scores compared with a waitlist, $p < .001$, with Cohen's $d = 0.08$. How should a counselor BEST interpret these results?

A. The module has a large effect, as shown by the very small p -value

The p -value reflects sample size as well as effect; it does not indicate magnitude.

B. The difference is statistically significant but practically trivial ✓ KEY

With a very large sample, tiny effects reach significance; $d = 0.08$ is well below even a small effect (0.2).

C. The result is likely a Type II error because the sample was so large

A Type II error is missing a real effect; here an effect was detected, and large samples reduce Type II risk.

D. The module causes anxiety to drop by about 8 percent for most clients

Cohen's d is a difference in standard deviation units, not a percentage reduction.

Rationale: Statistical significance tells whether an effect is unlikely under the null hypothesis; effect size tells how large it is. Cohen's guidelines for d are about 0.2 small, 0.5 medium and 0.8 large. Large samples can make trivial differences significant, so counselors should look at effect size to judge clinical or practical importance.

References: Erford (Research and Evaluation in Counseling) — Effect size (Cohen's d) vs statistical significance

☐ Approve ☐ Revise ☐ Retire Notes:

Counseling Skills and Interventions

60 items · NCE weight 30% (48 of 160 scored items)

125. nce-s-cou-001 · Counseling and Helping Relationships · psychodynamic and psychoanalytic theory · easy · In review

After a tense meeting in which she felt belittled by her supervisor, a client goes home and yells at her teenage son over a minor chore. Which defense mechanism does this BEST illustrate?

A. Projection

Projection attributes one's own unacceptable impulses to someone else; she is not accusing her son of her feelings.

B. Displacement ✓ **KEY**

Displacement redirects an impulse from a threatening target (the supervisor) to a safer one (the son).

C. Reaction formation

Reaction formation expresses the opposite of the true impulse, such as excessive friendliness toward the supervisor.

D. Sublimation

Sublimation channels an impulse into a socially valued outlet, such as vigorous exercise; yelling at her son is not that.

Rationale: In Freudian theory the ego uses defense mechanisms to reduce anxiety. Displacement shifts feelings away from the person who aroused them toward a less threatening target, as when anger at a boss is discharged at a family member.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: ego-defense mechanisms (displacement)

☐ Approve ☐ Revise ☐ Retire Notes:

126. nce-s-cou-002 · Counseling and Helping Relationships · psychodynamic and psychoanalytic theory · medium · In review

Several months into treatment, a client begins reacting to the counselor's neutral comments with hurt and anger, saying, "You sound just like my father, always judging me." From a psychoanalytic perspective, the counselor should view this reaction as:

A. Countertransference that the counselor ought to disclose to the client

Countertransference is the counselor's own emotional reaction to the client; here the reaction is the client's.

B. Resistance that the counselor should confront firmly and directly

Analysts explore resistance rather than confront it firmly, and this reaction is better understood as transference.

C. A realistic response to the counselor's own critical style

The comments were neutral, so the client is reacting to a figure from the past rather than to the counselor's actual behavior.

D. Transference that offers material for analysis and interpretation ✓ **KEY**

The client is redirecting feelings about a significant past figure onto the counselor, which analysts treat as central therapeutic material.

Rationale: Transference occurs when a client unconsciously shifts feelings and attitudes from significant early figures onto the therapist. In psychoanalytic work, analyzing and interpreting transference helps clients gain insight into unresolved conflicts and how they repeat in current relationships.

References: Corey (Theory & Practice) — Psychoanalytic Therapy: transference and its interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

An Adlerian counselor tells a client that his frequent helplessness seems to get his siblings to handle his responsibilities for him. Once the hidden payoff is named, the behavior becomes less appealing to the client. This technique is known as:

A. Spitting in the client's soup ✓ KEY

Exposing the purpose or payoff of a behavior "spoils" it, so the client can no longer use it as innocently as before.

B. Acting as if

Acting as if asks the client to behave as though already the person he wishes to be; it does not expose a payoff.

C. Catching oneself

Catching oneself has the client notice self-defeating behavior in the moment; here the counselor names its purpose.

D. The push-button technique

The push-button technique has clients alternate pleasant and unpleasant images to see that they create their feelings.

Rationale: Adlerian therapy assumes behavior is purposeful. "Spitting in the client's soup" means pointing out the hidden goal or payoff of a self-defeating behavior; the client may continue it, but it no longer tastes as good.

References: Corey (Theory & Practice) — Adlerian Therapy: techniques of the reorientation phase

☐ Approve ☐ Revise ☐ Retire Notes:

In Adler's view of psychological birth order, which child is MOST associated with the experience of being "dethroned" when a new sibling arrives?

A. The second-born child

Adler described the second child as racing to catch up with the older sibling, not as dethroned.

B. The youngest child

The youngest is described as the "baby" who may be pampered; no later sibling displaces them.

C. The oldest child ✓ KEY

The firstborn enjoys undivided attention until a sibling is born and must then adjust to losing that favored position.

D. The only child

An only child is never displaced by a sibling, although Adler noted they may struggle with sharing.

Rationale: Adler emphasized psychological (not merely ordinal) birth position within the family constellation. The oldest child is initially the center of attention and is "dethroned" by a sibling's birth, which can shape a lifestyle focused on responsibility, achievement, or regaining status.

References: Corey (Theory & Practice) — Adlerian Therapy: birth order and the family constellation

☐ Approve ☐ Revise ☐ Retire Notes:

Carl Rogers proposed that clients move toward growth when the counselor offers congruence, accurate empathic understanding, and which other core condition?

A. Interpretation of defenses

Interpreting defenses is a psychoanalytic technique, not a Rogerian core condition.

B. Structured homework assignments

Homework is typical of cognitive and behavioral approaches, not person-centered therapy.

C. Unconditional positive regard ✓ KEY

Rogers named unconditional positive regard, nonjudgmental acceptance of the client, as a core condition.

D. Confrontation of irrational beliefs

Disputing irrational beliefs is central to REBT, not to the person-centered approach.

Rationale: Rogers held that congruence (genuineness), unconditional positive regard, and accurate empathic understanding are necessary and sufficient conditions for therapeutic change. The counselor's attitudes, rather than techniques, release the client's actualizing tendency.

References: Corey (Theory & Practice) — Person-Centered Therapy: core therapeutic conditions

☐ Approve ☐ Revise ☐ Retire Notes:

A college student tells her counselor she feels worthwhile only when she earns top grades, the one achievement her parents consistently praised while growing up. From a person-centered perspective, her distress MOST likely stems from:

A. Internalized conditions of worth that distort her self-concept ✓ KEY

Rogers held that conditional acceptance from significant others leads people to value themselves only when meeting those conditions, creating incongruence.

B. An unresolved conflict between her id impulses and her superego

Id-superego conflict is a psychoanalytic formulation, not a person-centered one.

C. Irrational demands that she must perform well to be acceptable

Absolutistic "musts" are the REBT explanation of disturbance, not Rogers's.

D. A mistaken private logic formed in her early family constellation

Private logic and the family constellation are Adlerian concepts.

Rationale: Rogers described how children who receive only conditional positive regard introject conditions of worth. They then deny or distort experiences that do not fit, producing incongruence between self-concept and experience, which shows up as anxiety and vulnerability.

References: Corey (Theory & Practice) — Person-Centered Therapy: conditions of worth and incongruence

☐ Approve ☐ Revise ☐ Retire Notes:

Which theorist founded logotherapy, drawing on his experiences in Nazi concentration camps to argue that people can find meaning even in unavoidable suffering?

A. Rollo May

May was an American existential psychologist known for his work on anxiety, freedom, and love, not logotherapy.

B. Irvin Yalom

Yalom described the existential givens and group therapeutic factors but did not found logotherapy.

C. Fritz Perls

Perls founded gestalt therapy.

D. Viktor Frankl ✓ **KEY**

Frankl developed logotherapy, centered on the will to meaning, and wrote Man's Search for Meaning.

Rationale: Viktor Frankl's logotherapy holds that the primary human motivation is the will to meaning. Meaning can be found through creative work, experiences and relationships, and the attitude one takes toward unavoidable suffering.

References: Corey (Theory & Practice) — Existential Therapy: Frankl and the search for meaning

☐ Approve ☐ Revise ☐ Retire Notes:

Irvin Yalom described four "ultimate concerns," or givens of existence, that are central to existential psychotherapy. Which set lists them?

A. Anxiety, guilt, shame, and existential despair

These are emotional states that may arise from facing the givens, not the givens themselves.

B. Death, freedom, isolation, and meaninglessness ✓ **KEY**

Yalom identified death, freedom (and responsibility), existential isolation, and meaninglessness as the ultimate concerns.

C. Survival, belonging, power, freedom, and fun

These are Glasser's five basic needs from choice theory.

D. Safety, love, esteem, and self-actualization

These resemble levels of Maslow's hierarchy of needs.

Rationale: Yalom organized existential therapy around four givens: death, freedom, existential isolation, and meaninglessness. Anxiety arises when people confront these givens, and therapy helps clients face them authentically rather than through defensive avoidance.

References: Corey (Theory & Practice) — Existential Therapy: Yalom's ultimate concerns

☐ Approve ☐ Revise ☐ Retire Notes:

A client grieving her late father says she never told him how angry she was about his drinking. The counselor invites her to imagine her father sitting in an empty chair and to speak to him directly, in the present tense. This intervention is MOST characteristic of:

A. Gestalt therapy ✓ KEY

The empty chair technique is a hallmark gestalt experiment for working through unfinished business in the here and now.

B. Rational emotive behavior therapy

REBT disputes irrational beliefs; it does not rely on enactment with an empty chair.

C. Reality therapy

Reality therapy focuses on current behavior and the WDEP system, not on enacted dialogues.

D. Solution-focused brief therapy

SFBT emphasizes exceptions and preferred futures rather than expressing unfinished feelings.

Rationale: Gestalt therapy, associated with Fritz and Laura Perls, uses experiments such as the empty chair to bring unfinished business into present awareness. Speaking directly to the imagined person helps the client experience and complete blocked feelings.

References: Corey (Theory & Practice) — Gestalt Therapy: unfinished business and the empty-chair technique

☐ Approve ☐ Revise ☐ Retire Notes:

When a coworker criticizes him, a client reports that he wants to shout back but instead clenches his jaw and digs his fingernails into his own palms. In gestalt terms, this pattern of turning back onto oneself what one would like to do to others is called:

A. Introjection

Introjection is uncritically swallowing others' values or beliefs without assimilating them.

B. Confluence

Confluence is blurring the boundary between self and environment so that differences are not experienced.

C. Retroflexion ✓ KEY

Retroflexion is doing to oneself what one wants to do to someone else, such as turning anger inward.

D. Deflection

Deflection is avoiding contact through distraction, humor, or vagueness.

Rationale: Gestalt therapy describes several contact-boundary disturbances: introjection, projection, retroflexion, deflection, and confluence. Retroflexion turns an impulse meant for the environment back against the self, often seen in self-harm, muscle tension, or self-blame.

References: Corey (Theory & Practice) — Gestalt Therapy: contact and resistances to contact

☐ Approve ☐ Revise ☐ Retire Notes:

Joseph Wolpe's systematic desensitization pairs deep relaxation with gradual imagined exposure to a fear hierarchy. The technique is based on which principle?

A. Operant extinction

Operant extinction removes reinforcement for a behavior; desensitization targets a conditioned anxiety response.

B. Reciprocal inhibition ✓ KEY

Wolpe reasoned that a response incompatible with anxiety, such as relaxation, inhibits anxiety when paired with the feared stimulus.

C. Negative reinforcement

Negative reinforcement increases behavior by removing an aversive stimulus; it is not the basis of the technique.

D. Vicarious learning

Vicarious learning through modeling comes from Bandura's social learning theory.

Rationale: Systematic desensitization is a classical-conditioning procedure. The client learns relaxation, builds an anxiety hierarchy, and then imagines each step while relaxed. Because relaxation and anxiety are incompatible, the relaxed state reciprocally inhibits the fear response.

References: Corey (Theory & Practice) — Behavior Therapy: systematic desensitization and reciprocal inhibition

☐ Approve ☐ Revise ☐ Retire Notes: _____

To reduce a 7-year-old's hitting, his parents remove his tablet for the evening each time he hits his sister, and the hitting decreases. Which operant procedure are they using?

A. Positive reinforcement

Positive reinforcement adds a desirable stimulus to increase a behavior.

B. Negative reinforcement

Negative reinforcement removes an aversive stimulus to increase a behavior.

C. Positive punishment

Positive punishment adds an aversive stimulus, such as extra chores, to decrease a behavior.

D. Negative punishment ✓ KEY

Removing a desired item (the tablet) to decrease a behavior is negative punishment.

Rationale: In operant conditioning, "positive" means adding and "negative" means removing a stimulus; reinforcement increases behavior and punishment decreases it. Taking away a privilege to reduce hitting is therefore negative punishment (sometimes called response cost).

References: Corey (Theory & Practice) — Behavior Therapy: operant conditioning techniques

☐ Approve ☐ Revise ☐ Retire Notes: _____

Aaron Beck's cognitive triad of depression describes a negative view of which three areas?

A. The self, others, and the past

The triad concerns the future, not the past, and the world rather than others alone.

B. Thoughts, feelings, and behaviors

This is the general CBT model of interacting domains, not the cognitive triad.

C. The past, present, and future

The triad is not a time sequence; it includes the self and the world.

D. The self, the world, and the future ✓ KEY

Beck described depressed clients as viewing themselves, their world, and their future negatively.

Rationale: Beck proposed that depression is maintained by a negative cognitive triad: "I am inadequate" (self), "Everything goes wrong" (world), and "It will never get better" (future). Cognitive therapy helps clients test these appraisals against evidence.

References: Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's cognitive triad

☐ Approve

☐ Revise

☐ Retire

Notes:

A mother tells her counselor that her son's soccer team lost the championship because she had to miss the game for work. In Beck's cognitive therapy, this is an example of which cognitive distortion?

A. Magnification

Magnification exaggerates the importance of an event rather than wrongly taking responsibility for it.

B. Personalization ✓ KEY

She attributes an external event to herself without a basis for making the connection.

C. Dichotomous thinking

Dichotomous thinking sorts experiences into all-or-nothing categories.

D. Selective abstraction

Selective abstraction draws conclusions from one detail while ignoring the larger context.

Rationale: Personalization is the tendency to relate external events to oneself when there is no basis for doing so. Beck catalogued distortions such as arbitrary inference, selective abstraction, overgeneralization, magnification and minimization, personalization, and dichotomous thinking.

References: Corey (Theory & Practice) — Cognitive Behavior Therapy: Beck's systematic errors in reasoning

☐ Approve

☐ Revise

☐ Retire

Notes:

In Albert Ellis's ABCDE model of rational emotive behavior therapy, what does the "D" represent?

A. Describing the activating event

The activating event is the "A" in the model.

B. Developing new behaviors

New behaviors may follow, but "D" is a cognitive step, not behavior development.

C. Disputing irrational beliefs ✓ KEY

D is disputing, the active challenge of the irrational beliefs ("B") that cause disturbed consequences ("C").

D. Defining emotional consequences

Emotional and behavioral consequences are the "C" in the model.

Rationale: Ellis's model holds that an Activating event does not directly cause emotional Consequences; Beliefs about the event do. The counselor Disputes irrational beliefs, producing an Effective new philosophy (E) and healthier feelings.

References: Corey (Theory & Practice) — Rational Emotive Behavior Therapy: the A-B-C framework

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I must get this promotion. If I don't, it will be awful, I won't be able to stand it, and it will prove I'm worthless." According to Ellis, which element is the CORE of this irrational belief, from which the others derive?

A. Demandingness, or absolutistic "musts" ✓ KEY

Ellis held that rigid demands ("I must") are primary, and awfulizing, low frustration tolerance, and self-downing derive from them.

B. Catastrophic misinterpretation

Catastrophic misinterpretation of bodily sensations is a cognitive model of panic, not Ellis's core belief.

C. Internalized conditions of worth

Conditions of worth are a person-centered concept.

D. Introjected parental messages

Introjection is a gestalt contact disturbance, not the REBT core of irrationality.

Rationale: REBT identifies demandingness ("must," "should," "ought" held as absolutes) as the core of irrational thinking. Awfulizing, "I-can't-stand-it-itis" (low frustration tolerance), and global self-rating are derivatives that follow from rigid demands.

References: Corey (Theory & Practice) — Rational Emotive Behavior Therapy: musturbation and derivative beliefs

☐ Approve ☐ Revise ☐ Retire Notes:

Reality therapy is often taught through the WDEP system (Wants, Doing and direction, Evaluation, Planning). Who developed and expanded this system?

A. Robert Wubbolding ✓ **KEY**

Wubbolding developed the WDEP system as a practical framework for applying Glasser's reality therapy.

B. Albert Ellis

Ellis founded REBT and the ABCDE model.

C. Steve de Shazer

De Shazer co-developed solution-focused brief therapy.

D. Michael White

White co-developed narrative therapy.

Rationale: William Glasser founded reality therapy and choice theory; Robert Wubbolding extended it with the WDEP system. Counselors explore what clients want, what they are doing, whether it is working, and a simple, attainable plan.

References: Corey (Theory & Practice) — Reality Therapy: the WDEP system

☐ Approve ☐ Revise ☐ Retire Notes:

A high school student says he skips class because "nobody there likes me anyway." After exploring what he wants and what he is currently doing, the reality therapist asks, "Is skipping class helping you get the friendships you want?" This question reflects which part of WDEP?

A. Wants

Wants were explored earlier; this question judges whether behavior serves those wants.

B. Doing and direction

Doing describes current behavior; it does not ask the client to judge its effectiveness.

C. Evaluation ✓ **KEY**

Asking whether current behavior is getting the client what he wants is self-evaluation, the core of reality therapy.

D. Planning

Planning comes after evaluation and creates a specific, attainable course of action.

Rationale: In Wubbolding's WDEP system, Evaluation asks clients to judge whether their total behavior is meeting their needs and wants. Glasser's five basic needs are survival, love and belonging, power, freedom, and fun; here the student is pursuing belonging ineffectively.

References: Corey (Theory & Practice) — Reality Therapy: self-evaluation in the WDEP system

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor asks a client, "Suppose that tonight, while you sleep, the problem that brought you here is solved. When you wake up, what will be the first thing you notice that tells you things are different?" This is an example of:

A. An exception question

Exception questions ask about past times the problem was absent or less severe.

B. The miracle question ✓ KEY

Imagining the problem gone overnight and describing the difference is the classic miracle question.

C. A scaling question

Scaling questions ask clients to rate progress or confidence on a numerical scale.

D. A coping question

Coping questions ask how the client has managed to keep going despite difficulties.

Rationale: The miracle question, developed within solution-focused brief therapy by Steve de Shazer and Insoo Kim Berg, invites clients to describe a preferred future in concrete, observable terms. Those details then become the basis for goals and small next steps.

References: Corey (Theory & Practice) — Postmodern Approaches: solution-focused brief therapy, the miracle question

☐ Approve

☐ Revise

☐ Retire

Notes:

A counselor says, "On a scale from 0 to 10, where 10 means the problem is fully resolved, you said you are at a 4. What would tell you that you had moved to a 5?" This technique and the approach it comes from are MOST closely associated with:

A. Michael White and David Epston

White and Epston developed narrative therapy, known for externalizing conversations.

B. Jay Haley and Cloé Madanes

Haley and Madanes are associated with strategic family therapy and directives.

C. William Miller and Stephen Rollnick

Miller and Rollnick developed motivational interviewing; MI uses importance rulers, but this framing is SFBT scaling.

D. Steve de Shazer and Insoo Kim Berg ✓ KEY

Scaling questions focused on small next steps are a signature solution-focused technique from de Shazer and Berg.

Rationale: Solution-focused brief therapy, developed by Steve de Shazer and Insoo Kim Berg, uses scaling questions to make progress concrete and to identify the next small, observable step. The focus is on what is already working rather than on the problem's history.

References: Corey (Theory & Practice) — Postmodern Approaches: solution-focused brief therapy, scaling questions

☐ Approve

☐ Revise

☐ Retire

Notes:

A counselor working with a 10-year-old who refuses to go to school asks, "When does the Worry show up, and how does it try to convince you to stay home?" The counselor is using which technique?

A. Reframing

Reframing offers a new meaning for a behavior; it does not separate the problem from the person.

B. Paradoxical intention

Paradoxical intention asks the client to deliberately do or wish for the feared behavior.

C. Externalizing the problem ✓ KEY

Naming the Worry as a separate entity that acts on the child externalizes the problem.

D. Cognitive restructuring

Cognitive restructuring identifies and replaces distorted thoughts rather than personifying the problem.

Rationale: Narrative therapy, developed by Michael White and David Epston, holds that "the person is not the problem; the problem is the problem." Externalizing conversations separate clients from problem-saturated identities so they can examine and resist the problem's influence.

References: Corey (Theory & Practice) — Postmodern Approaches: narrative therapy, externalizing conversations

☐ Approve ☐ Revise ☐ Retire Notes:

A narrative counselor learns that a client who describes himself as "a hopeless addict" turned down a drink at a family wedding last month. The counselor explores this event in detail to begin building an alternative story. In narrative therapy, such events are called:

A. Unique outcomes ✓ KEY

White used "unique outcomes" for moments that contradict the dominant problem story and seed re-authoring.

B. Exceptions

"Exceptions" is the solution-focused term for times the problem is absent; the question asks for narrative terminology.

C. Peak experiences

Peak experiences are Maslow's term for moments of intense fulfillment.

D. Corrective emotional experiences

This psychodynamic term refers to re-experiencing old conflicts with a new, healthier outcome in therapy.

Rationale: Narrative therapists listen for unique outcomes: times when the client acted outside the problem-saturated story. Thickening these accounts helps re-author an alternative, preferred identity story.

References: Corey (Theory & Practice) — Postmodern Approaches: narrative therapy, unique outcomes and re-authoring

► **Check:** 'Exceptions' (SFBT) sits beside the key 'unique outcomes' (narrative); confirm one best answer reads clearly.

☐ Approve ☐ Revise ☐ Retire Notes:

Which statement BEST reflects a central principle of feminist therapy?

A. Distress mainly reflects unresolved intrapsychic conflict

Feminist therapy looks beyond intrapsychic causes to social, political, and cultural context.

B. The personal is political, so context shapes distress ✓ **KEY**

"The personal is political" holds that individual problems are linked to social structures and oppression.

C. The counselor holds expert authority to direct the client

Feminist therapy emphasizes an egalitarian relationship that minimizes power differences.

D. A diagnosis is required before exploring social context

Feminist therapists are cautious about diagnostic labels and prioritize the client's context.

Rationale: Feminist therapy rests on principles such as "the personal is political," an egalitarian counseling relationship, valuing women's experiences, and a commitment to social change. Symptoms are often reframed as understandable responses to oppressive conditions.

References: Corey (Theory & Practice) — Feminist Therapy: key principles

☐ Approve

☐ Revise

☐ Retire

Notes:

A counselor helps a client identify the messages she received growing up about what women "should" be, where those messages came from, and how they influence her guilt about setting limits with family today. This feminist intervention is called:

A. Lifestyle assessment

Lifestyle assessment is an Adlerian technique exploring family constellation and early recollections.

B. Genogram construction

A genogram maps multigenerational family patterns, a Bowenian tool.

C. Cognitive restructuring

Cognitive restructuring targets distorted thoughts without centering gender socialization.

D. Gender-role analysis ✓ **KEY**

Gender-role analysis examines the impact of gender-role expectations on a client's well-being.

Rationale: Gender-role analysis helps clients recognize gender-role messages, their sources, and their consequences, and then decide which to keep or change. Along with power analysis, it is a signature feminist therapy intervention.

References: Corey (Theory & Practice) — Feminist Therapy: gender-role analysis and intervention

☐ Approve

☐ Revise

☐ Retire

Notes:

In motivational interviewing, the core skills are summarized by the acronym OARS: open questions, affirmations, reflections, and what?

A. Solutions

MI avoids prematurely supplying solutions; the client voices reasons for change.

B. Summaries ✓ KEY

Summaries collect and link what the client has said, often highlighting change talk.

C. Silence

Silence can be useful, but it is not part of the OARS acronym.

D. Self-disclosure

Self-disclosure is not one of the OARS core skills.

Rationale: Miller and Rollnick describe OARS (open questions, affirming, reflecting, summarizing) as the foundational communication skills of MI. These skills support engaging, focusing, evoking, and planning.

References: Miller & Rollnick (MI) — Core interviewing skills: OARS

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I know I should cut back on drinking, but it's the only thing that helps me relax after work." Which counselor response is MOST consistent with motivational interviewing?

A. "You need to understand that drinking is harming your health."

Unsolicited warnings reflect the righting reflex and tend to elicit sustain talk.

B. "Why haven't you been able to cut back on your drinking yet?"

Asking why the client has not changed invites defense of the status quo.

C. "It sounds like you are in denial about how serious this is."

Labeling and confronting denial increases discord, which MI avoids.

D. "It helps you unwind, and you also see reasons to cut back." ✓ KEY

A double-sided reflection acknowledges both sides of ambivalence and ends on the change side.

Rationale: MI treats ambivalence as normal. A double-sided reflection captures both sustain talk and change talk, typically ending with the change side, so the client hears his own reasons for change without feeling pushed. Confrontation and advice-giving tend to strengthen sustain talk.

References: Miller & Rollnick (MI) — Responding to sustain talk and discord: double-sided reflections

☐ Approve ☐ Revise ☐ Retire Notes:

A DBT skills group is teaching TIPP skills for crisis survival and practicing radical acceptance of painful realities that cannot be changed right now. These skills belong to which DBT module?

A. Distress tolerance ✓ KEY

Crisis survival skills such as TIPP and reality-acceptance skills such as radical acceptance are taught in distress tolerance.

B. Emotion regulation

Emotion regulation focuses on understanding emotions, reducing vulnerability, and changing unwanted emotions.

C. Interpersonal effectiveness

Interpersonal effectiveness teaches asking for what one needs and saying no (for example, DEAR MAN).

D. Core mindfulness

Mindfulness teaches observing, describing, and participating nonjudgmentally (wise mind).

Rationale: Marsha Linehan's DBT skills training has four modules: mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness. Distress tolerance helps clients survive crises without making things worse and accept reality as it is.

References: Corey (Theory & Practice) — Behavior Therapy: dialectical behavior therapy skills modules

► **Check:** Corey's placement of DBT (third-wave section of Behavior Therapy) may differ by edition.

☐ Approve ☐ Revise ☐ Retire Notes:

Marsha Linehan developed dialectical behavior therapy initially for chronically suicidal clients. The central dialectic of the approach is the balance between:

A. Insight and catharsis

Insight and catharsis are associated with psychodynamic and experiential approaches.

B. Structure and spontaneity

This is not the organizing dialectic of DBT.

C. Acceptance and change ✓ KEY

DBT balances validation and acceptance of the client with behavioral change strategies.

D. Thinking and feeling

DBT integrates reason and emotion in "wise mind," but its central dialectic is acceptance and change.

Rationale: DBT integrates cognitive-behavioral change strategies with acceptance practices drawn from Zen and mindfulness. Linehan developed it for chronically suicidal clients, many meeting criteria for borderline personality disorder, who experienced pure change-focused therapy as invalidating.

References: Corey (Theory & Practice) — Behavior Therapy: dialectical behavior therapy, acceptance and change

► **Check:** Corey's placement of DBT (third-wave section of Behavior Therapy) may differ by edition.

☐ Approve ☐ Revise ☐ Retire Notes:

Two parents in a tense marriage each confide in their 15-year-old daughter about their frustrations with the other, and the daughter now feels responsible for keeping the peace. A Bowenian family counselor would MOST likely describe this pattern as:

A. Triangulation ✓ KEY

Bowen described how anxious two-person systems draw in a third person to stabilize tension, forming a triangle.

B. Emotional cutoff

Emotional cutoff is managing unresolved fusion by distancing from family, the opposite of being drawn in.

C. Sibling position

Sibling position concerns how birth order shapes functioning, not parental conflict.

D. Societal regression

Societal regression applies Bowen's concepts to society as a whole.

Rationale: In Bowen family systems theory, the triangle is the smallest stable relationship unit; dyads under stress pull in a third person. Bowenian counselors help members increase differentiation of self, often using genograms to trace multigenerational patterns, so they can stay connected without being triangled.

References: Corey (Theory & Practice) — Family Systems Therapy: Bowen's triangulation and differentiation of self

☐ Approve ☐ Revise ☐ Retire Notes:

Parents tell a family counselor they always agree on rules. The counselor asks them to decide, right now in session, whether their son may stay out late Saturday, then watches as the son interrupts and his mother sides with him. In Minuchin's structural family therapy, this technique is called:

A. Prescribing the symptom

Prescribing the symptom is a paradoxical directive from strategic therapy (Haley, Madanes).

B. Enactment ✓ KEY

Enactment has the family act out a transaction in session so the counselor can observe and modify its structure.

C. Detriangulation

Detriangulation is a Bowenian goal of removing a third person from a two-person conflict.

D. Unbalancing

Unbalancing is a structural technique in which the counselor temporarily sides with one member to shift power; that is not what is described.

Rationale: Minuchin's structural family therapy focuses on subsystems, boundaries, and hierarchy. In an enactment, the counselor has family members interact in session rather than report on their interactions, revealing patterns such as a cross-generational coalition that the counselor can then help restructure.

References: Corey (Theory & Practice) — Family Systems Therapy: structural family therapy, enactment

► **Check:** 'Unbalancing' is also a Minuchin technique; confirm the scenario reads clearly as an enactment.

☐ Approve ☐ Revise ☐ Retire Notes:

In the third session of a community anger-management group, members begin arguing about the attendance rule, and two members openly question whether the leader is qualified to run the group. According to Tuckman's model, the group is MOST likely in which stage?

A. Forming

Forming is the orientation stage of politeness, dependence on the leader and tentative participation, before open conflict emerges.

B. Storming ✓ KEY

Storming is marked by conflict, challenges to the leader and struggles over control and rules as members test the group.

C. Norming

Norming follows storming; members have settled conflicts, agreed on norms and developed cohesion.

D. Performing

Performing is the productive stage in which the group works on its goals with little energy spent on conflict.

Rationale: Tuckman (1965; adjourning added with Jensen in 1977) described forming, storming, norming, performing and adjourning. Conflict over rules and direct challenges to the leader's authority are the hallmark of storming, which leaders should treat as a normal developmental step rather than a sign the group is failing.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: Tuckman stages of group development

☐ Approve ☐ Revise ☐ Retire Notes:

During the transition stage of a counseling group, a member crosses his arms and says to the leader, 'Honestly, this group is a waste of my time.' Other members fall silent. Which leader response BEST fits Corey's guidance for this stage?

A. Remind him that the group rules ask members to stay positive and to avoid discouraging the others in the room.

Invoking a rule to suppress the comment shuts down a challenge the group needs to work through and models avoidance of conflict.

B. Move to a structured exercise so the silence ends quickly and the group can return to the planned topic of the day.

Changing the subject bypasses the conflict; unaddressed resistance in the transition stage tends to go underground and erode trust.

C. Ask the other members whether they agree with him so that the group can vote on whether to keep meeting each week.

Polling members turns a personal reaction into a referendum, can isolate the member and puts the group itself on trial.

D. Respond without defensiveness and invite him to say more about what isn't working for him in the group so far.

✓ KEY

Corey advises leaders to meet challenges openly and non-defensively, treating resistance as useful information and modeling direct, respectful handling of conflict.

Rationale: In Corey's model the transition stage is characterized by anxiety, defensiveness, resistance and struggles for control, including challenges to the leader. The leader's task is to recognize and explore these reactions directly and non-defensively, which builds the trust needed to move into the working stage.

References: Corey (Theory & Practice) — Group stages: transition stage, dealing with resistance and challenges to the leader

☐ Approve ☐ Revise ☐ Retire Notes:

A high school counselor plans a six-week group in which students learn and practice specific study and test-anxiety management skills, with brief lessons and structured activities each week. Under the ASGW classification of group work, this is BEST described as which type of group?

A. Psychoeducation group ✓ KEY

Psychoeducation groups emphasize teaching information and skills, often to prevent problems, using structured content and activities.

B. Task group

Task and work groups focus on accomplishing an organizational goal, such as a committee or planning team, rather than member learning.

C. Counseling group

Counseling groups address interpersonal and developmental concerns through member interaction, with less didactic structure.

D. Psychotherapy group

Psychotherapy groups address more severe or chronic problems and personality reconstruction, which this skills group does not target.

Rationale: The ASGW identifies four group types: task/work, psychoeducation, counseling and psychotherapy. A time-limited, structured group whose main aim is teaching skills to a nonclinical population is a psychoeducation group.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: ASGW types of groups

☐ Approve ☐ Revise ☐ Retire Notes:

After a member of a grief group shares that she still sets a place at the table for her late husband, three other members say they have done something similar. She replies, 'I thought I was the only one.' Which of Yalom's therapeutic factors is MOST evident?

A. Instillation of hope

Instillation of hope comes from seeing others improve or cope well, not from discovering that others share the same experience.

B. Altruism

Altruism is the benefit of helping others; the member here is the one receiving relief rather than giving help.

C. Universality ✓ KEY

Universality is the relief of learning that one is not alone or unique in one's problems, thoughts or feelings.

D. Catharsis

Catharsis is the release of strong emotion; the key experience described is recognizing shared experience.

Rationale: Yalom identified 11 therapeutic factors. Universality, the discovery that others share one's 'secret' struggles, often appears early in groups and reduces shame and isolation, which is why the member's statement that she thought she was the only one points to it.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 1: The therapeutic factors, universality

☐ Approve ☐ Revise ☐ Retire Notes:

In a young adult counseling group, two members repeatedly press a quieter member to 'finally tell us what really happened' with her family, even after she says she is not ready. Which leadership skill is MOST needed at this moment?

A. Linking, by connecting her reluctance to similar feelings that others in the group have described

Linking builds connection among members but does not stop the pressure being placed on her right now.

B. Drawing out, by inviting the quiet member to share a little more so the others feel heard

Drawing her out would join the pressure; she has already stated a boundary about disclosure.

C. Blocking, by stopping the pressure and affirming her right to decide what and when to share ✓ **KEY**

Blocking protects members from counterproductive behavior such as coercive questioning while respecting their autonomy over disclosure.

D. Summarizing, by restating the main points of the discussion before moving to another member

Summarizing may move the group along but does not address the coercion or protect the member.

Rationale: Group leaders have a duty to protect members from undue pressure and from behaviors that threaten safety. Blocking intervenes in harmful patterns, such as pressuring, gossiping or breaking confidences, without attacking the members doing it.

References: Corey (Theory & Practice) — Group leadership skills: blocking and protecting members · ACA Code of Ethics — A.9.b Protecting Clients (group work)

☐ Approve ☐ Revise ☐ Retire Notes:

In a long-term interpersonal group, a man reacts to the male co-leader with the same deference and hidden resentment he felt toward his father, and competes with an older member as he did with his brother. Over months he recognizes the pattern and responds to them differently. Which of Yalom's therapeutic factors does this process BEST illustrate?

A. Corrective recapitulation of the primary family group ✓ **KEY**

The group re-creates early family dynamics, and the member relives and then reworks those patterns in a new, corrective way.

B. Imitative behavior modeled on the co-leader and members

Imitative behavior means adopting behaviors seen in others; the core change here is reworking old family-based roles.

C. Socializing techniques learned through group role-plays

Socializing techniques are basic social skills learned in the group, not the reworking of family-of-origin patterns.

D. Existential factors, such as accepting final responsibility

Existential factors involve accepting responsibility, mortality and life's unfairness, not reliving family roles.

Rationale: Yalom describes groups as resembling a family in many ways, so members often relate to leaders and peers as they did to parents and siblings. When those patterns are not merely relived but explored and corrected, the factor is corrective recapitulation of the primary family group.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 1: Corrective recapitulation of the primary family group

☐ Approve ☐ Revise ☐ Retire Notes:

In Egan's SOLER guideline for nonverbal attending, what does the letter 'L' stand for?

A. Listen without interrupting

Uninterrupted listening matters, but SOLER describes physical posture, and L is not listening.

B. Look at the client's face

Eye contact is the E in SOLER, not the L.

C. Limit note-taking in session

Note-taking is not one of the SOLER behaviors.

D. Lean slightly toward the client ✓ KEY

L stands for leaning toward the client, a posture that signals involvement and interest.

Rationale: SOLER stands for Squarely face the client, Open posture, Lean toward the client, Eye contact and Relaxed manner. Egan presents these as flexible guidelines; the counselor adapts them to the client's culture and comfort, since direct eye contact or close posture is not welcome in every context.

References: Gladding (Counseling: A Comprehensive Profession) — Building the counseling relationship: attending behavior (SOLER)

☐ Approve

☐ Revise

☐ Retire

Notes:

162. nce-s-cou-038 · Counseling and Helping Relationships · basic attending and listening skills · medium · In review

A practicum student is eager to start using confrontation and interpretation with clients. According to Ivey's microskills hierarchy, which skills should the student master FIRST?

A. Reflection of meaning and the interpretation of core themes

Reflection of meaning and interpretation sit higher in the hierarchy and rely on the listening foundation.

B. Attending behavior and the basic listening sequence ✓ KEY

Attending and the basic listening sequence are the foundation on which higher-level influencing skills are built.

C. Influencing skills such as directives and feedback

Influencing skills are later in the hierarchy and are effective only after the client feels heard.

D. Skill integration and a personal style of counseling

Skill integration and personal style form the top of the hierarchy, reached after the other skills are mastered.

Rationale: Ivey's hierarchy rests on ethics, multicultural competence and wellness, then attending behavior, then the basic listening sequence (questions, observation, encouraging, paraphrasing, summarizing, reflecting feeling). Focusing, empathic confrontation, reflection of meaning, influencing skills and skill integration build on that base.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: Ivey microskills hierarchy

► **Check:** Ivey's editions differ slightly on the hierarchy's base; the key holds across editions.

☐ Approve

☐ Revise

☐ Retire

Notes:

A client says, 'My supervisor criticized my report in front of the whole team again, and I just sat there and said nothing.' Which counselor response is the BEST example of a reflection of feeling?

A. You felt embarrassed, and maybe angry with yourself for not speaking up. ✓ KEY

This names the likely emotions behind the statement, which is the defining feature of reflecting feeling.

B. Your supervisor criticized your report in front of the team once again.

This restates the content of the message, which makes it a paraphrase rather than a reflection of feeling.

C. What do you think stopped you from saying something to your supervisor?

This is an open question that explores reasons; it does not reflect the emotion being expressed.

D. Next time, it might help to ask your supervisor to talk with you privately.

This is advice-giving, an influencing response that skips past the client's feelings.

Rationale: A reflection of feeling identifies and names the emotion in the client's message, often tentatively, so the client feels understood and can explore it further. A paraphrase captures content; questions and advice are different skills.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: reflection of feeling versus paraphrasing

☐ Approve

☐ Revise

☐ Retire

Notes:

Midway through a session, a client has moved quickly among worries about money, conflict with her sister and trouble sleeping. The counselor wants to pull these threads together, check accuracy and help the client choose a focus. Which skill is MOST suited to this purpose?

A. Paraphrasing

Paraphrasing restates the content of a single statement rather than tying together several themes across the session.

B. Reflection of meaning

Reflection of meaning explores values and deeper significance, not the organization of several topics into a focus.

C. Summarizing ✓ KEY

Summarizing condenses several statements or themes, checks understanding and helps set direction or transition.

D. Encouraging

Minimal encouragers keep the client talking; they do not integrate content across topics.

Rationale: Summarizing draws together the main content and feelings from a longer stretch of talk. It is used to begin or end sessions, to make transitions and to help a client who has raised many concerns decide what to work on.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: summarizing

☐ Approve

☐ Revise

☐ Retire

Notes:

A client who is tearful and twisting a tissue says, 'I'm completely fine with the divorce. It's no big deal.' Which response BEST illustrates empathic confrontation?

A. You aren't being honest with me or yourself, because you are clearly upset right now.

This accuses and judges the client, which is likely to raise defensiveness rather than invite exploration.

B. Divorce is one of the most stressful life events, so being upset would be quite normal.

This normalizes and reassures, but it does not name the discrepancy between the words and the behavior.

C. Why do you think you are telling me it is no big deal when it seems to matter a lot?

"Why" questions tend to put clients on the defensive and ask for justification rather than reflection.

D. You're saying the divorce is no big deal, and you're also in tears as we talk about it. ✓ **KEY**

This nonjudgmentally places the verbal message beside the nonverbal one and lets the client explore the mixed message.

Rationale: Empathic confrontation identifies discrepancies, mixed messages or conflicts (often with an "on the one hand ... and on the other" format) in a supportive, nonjudgmental way. Its aim is to help the client notice and resolve the incongruity, not to win an argument.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: confrontation of discrepancies

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor says, 'I notice that whenever we get close to talking about your father, you change the subject and glance at me as if checking my reaction. I'm wondering what is happening between us right now.' This statement is BEST described as:

A. Self-disclosure

Self-disclosure shares information about the counselor's own life; here the focus is the current relationship.

B. Immediacy ✓ **KEY**

Immediacy addresses what is occurring in the here-and-now between counselor and client.

C. Interpretation

Interpretation offers a new frame for the client's behavior; this response invites discussion of the present interaction instead.

D. Reframing

Reframing recasts a situation in a different light; it does not focus on the counselor-client relationship.

Rationale: Immediacy is the counselor's direct, present-tense comment on the counseling relationship or on a pattern unfolding in the session. It is useful when trust, tension, dependence or avoidance in the relationship is blocking progress.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: immediacy

☐ Approve ☐ Revise ☐ Retire Notes:

In a child-centered play therapy session, a 6-year-old grows frustrated and raises a wooden block as if to throw it at the counselor. Using Landreth's ACT model of limit setting, which response is BEST?

A. If you throw that block, our playtime is over for today and we will go back out.

Leading with a consequence skips acknowledging the feeling and offering an alternative; consequences come only if limits are repeatedly broken.

B. We don't throw blocks in here. Can you please put it back on the shelf for me?

This states a limit but ignores the child's feeling and gives no acceptable outlet for it.

C. You're really mad. I'm not for throwing. You can throw the soft ball at the wall. ✓ KEY

This acknowledges the feeling, communicates the limit and targets an acceptable alternative, the three steps of ACT.

D. It sounds like you are feeling frustrated with how the game is going for you today.

Reflecting the feeling alone leaves the counselor and child unprotected; a limit is needed when safety is at stake.

Rationale: Landreth's ACT model: Acknowledge the child's feeling, Communicate the limit and Target an acceptable alternative. Limits in child-centered play therapy are few, are stated calmly and briefly, and protect physical safety and the relationship while leaving the feeling itself accepted.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: child-centered play therapy and limit setting

► **Check:** The key's wording is in Landreth's style, not a direct quote.

☐ Approve ☐ Revise ☐ Retire Notes:

A play therapist spends early sessions building an egalitarian relationship with a 9-year-old, then gathers information about the child's lifestyle and mistaken beliefs, helps the child gain insight, and finally uses reorientation to practice new ways of belonging. This sequence is characteristic of which approach?

A. Adlerian play therapy ✓ KEY

Kottman's Adlerian play therapy proceeds through relationship building, exploring lifestyle, gaining insight and reorientation/reeducation.

B. Child-centered play therapy

Child-centered play therapy is nondirective and does not assess lifestyle or work toward planned reeducation phases.

C. Theraplay

Theraplay is a directive, attachment-based approach involving caregivers in structured engagement and nurture activities.

D. Filial therapy

Filial therapy trains parents to conduct child-centered play sessions; it is not organized around lifestyle analysis.

Rationale: Terry Kottman's Adlerian play therapy integrates Adlerian concepts (lifestyle, mistaken beliefs, the Crucial Cs, encouragement) with play. Its four phases are building an egalitarian relationship, exploring the child's lifestyle, helping the child gain insight and reorientation/reeducation.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: approaches to play therapy (Adlerian, child-centered, filial)

► **Check:** Kottman's Adlerian play therapy phases are cited via Gladding, which may not cover Kottman in depth.

☐ Approve ☐ Revise ☐ Retire Notes:

A college student arrives at the counseling center hours after learning his scholarship was revoked, saying he 'can't see a way forward.' Following Roberts' seven-stage crisis intervention model, what should the counselor do FIRST?

A. Help him generate options for funding and plan his next steps with financial aid

Generating and exploring alternatives is stage 5, after safety, rapport, problems and feelings have been addressed.

B. Conduct a thorough psychosocial and lethality assessment, including suicide risk ✓ **KEY**

Stage 1 of Roberts' model is a biopsychosocial and lethality assessment; his hopeless statement makes this urgent.

C. Explore and validate his feelings of shock, anger and shame about the revocation

Dealing with feelings and emotions is stage 4 of the model, not the first step.

D. Identify the main problems and the precipitating event that led him to come in today

Identifying major problems and precipitants is stage 3, after assessment and rapport building.

Rationale: Roberts' seven stages are: (1) plan and conduct a thorough biopsychosocial and lethality assessment, (2) establish rapport, (3) identify major problems and precipitants, (4) deal with feelings and emotions, (5) generate and explore alternatives, (6) implement an action plan and (7) follow up. Stages can overlap, but safety assessment comes first.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling: Roberts' seven-stage crisis intervention model

☐ Approve ☐ Revise ☐ Retire Notes:

A family is seen by a crisis counselor two days after their apartment was destroyed in a fire. Which goal BEST reflects the primary aim of crisis intervention?

A. Uncover early losses that make the family vulnerable to grief and resolve them

Exploring early history is a goal of longer-term therapy and can overwhelm people in acute crisis.

B. Restructure family roles and boundaries that may have contributed to stress

Structural change in family organization is a family therapy goal, not the immediate aim of crisis work.

C. Complete a diagnostic evaluation of each member to begin long-term treatment

Crisis intervention is brief and focused on coping; a crisis reaction is not by itself a disorder requiring diagnosis.

D. Restore the family to at least its pre-crisis level of coping and functioning ✓ **KEY**

Crisis intervention seeks to stabilize people and restore equilibrium, at least to the pre-crisis level of functioning.

Rationale: A crisis is a time-limited state in which usual coping is overwhelmed. Crisis intervention is brief, active and present-focused, aiming to ensure safety, mobilize resources and coping and return people to at least their pre-crisis level of functioning; deeper personality or system change is left for later work if needed.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling: goals of crisis intervention

☐ Approve ☐ Revise ☐ Retire Notes:

On Holland's RIASEC hexagon, which pair of personality types is MOST consistent, meaning the two types share the most characteristics?

A. Realistic and Investigative ✓ KEY

Realistic and Investigative are adjacent on the hexagon, so they share the most traits and form a consistent pair.

B. Realistic and Social

Realistic and Social sit opposite each other on the hexagon and are the least consistent pairing.

C. Artistic and Conventional

Artistic and Conventional are opposite types, one valuing self-expression and the other order and structure.

D. Investigative and Enterprising

Investigative and Enterprising are opposite each other on the hexagon, so they are inconsistent.

Rationale: Holland arranged the six types in the order R-I-A-S-E-C around a hexagon. Adjacent types are most consistent, alternate types are intermediate and opposite types (R-S, I-E, A-C) are least consistent. Holland's related ideas include congruence (fit between person and environment) and differentiation.

References: Sharf (Career Development Theory) — Holland's theory of types: the hexagon and consistency

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old nurse manager tells a counselor that her main concerns are holding on to the position she has built, keeping her clinical skills current and innovating within her role. In Super's life-span, life-space theory, these concerns BEST fit which career stage?

A. Establishment

Establishment (about ages 25 to 44) centers on stabilizing in and advancing within a chosen field.

B. Exploration

Exploration (about ages 15 to 24) involves crystallizing, specifying and implementing an occupational choice.

C. Maintenance ✓ KEY

Maintenance (about ages 45 to 64) involves holding, updating and innovating to preserve one's position.

D. Disengagement

Disengagement (about 65 and older) involves decelerating, retirement planning and retirement living.

Rationale: Super's stages are growth, exploration, establishment, maintenance and disengagement, each with developmental tasks; the maintenance tasks are holding, updating and innovating. Super also stressed that people recycle through stages and that work is one of several life roles shown in the life-career rainbow.

References: Sharf (Career Development Theory) — Super's life-span, life-space theory: maintenance stage tasks

► **Check:** Super's stage ages are approximate; the rationale notes recycling.

☐ Approve ☐ Revise ☐ Retire Notes:

A client is anxious because she has no clear five-year career plan. The counselor helps her see that past opportunities arose from unexpected events, and encourages curiosity, persistence, flexibility, optimism and risk taking to create and use future chance events. This approach reflects:

A. Theory of work adjustment

The theory of work adjustment (Dawis and Lofquist) addresses correspondence between worker needs and job reinforcers.

B. Planned happenstance theory ✓ KEY

Planned happenstance (Mitchell, Levin and Krumboltz) teaches clients to create and capitalize on unplanned events.

C. Circumscription and compromise

Gottfredson's theory explains how children narrow options by sex type and prestige, not how to use chance events.

D. Cognitive information processing

CIP (Sampson, Peterson and colleagues) focuses on the pyramid of information processing and the CASVE decision cycle.

Rationale: Planned happenstance grew out of Krumboltz's social learning theory of career decision making. It reframes indecision as open-mindedness and builds five skills (curiosity, persistence, flexibility, optimism and risk taking) so that clients can turn unplanned events into career opportunities.

References: Sharf (Career Development Theory) — Krumboltz's social learning theory and planned happenstance

☐ Approve ☐ Revise ☐ Retire Notes:

A 4-year-old eagerly plans games and starts projects, but his parents often scold him for 'making messes' when he tries new activities. In Erikson's theory, this child is working on which psychosocial crisis?

A. Autonomy vs. shame and doubt

Autonomy vs. shame and doubt is the toddler stage (about 1 to 3 years), centered on self-control and independence.

B. Industry vs. inferiority

Industry vs. inferiority is the school-age stage (about 6 to 12 years), centered on competence and productivity.

C. Trust vs. mistrust

Trust vs. mistrust is the infancy stage (birth to about 1 year), centered on reliable caregiving.

D. Initiative vs. guilt ✓ KEY

Initiative vs. guilt (about 3 to 6 years) involves planning and starting activities; harsh criticism can foster guilt.

Rationale: Erikson's eight stages are trust vs. mistrust (infancy), autonomy vs. shame and doubt (toddlerhood), initiative vs. guilt (preschool), industry vs. inferiority (school age), identity vs. role confusion (adolescence), intimacy vs. isolation (young adulthood), generativity vs. stagnation (middle adulthood) and integrity vs. despair (late adulthood).

References: Berk (Development Through the Lifespan) — Erikson's psychosocial theory: initiative vs. guilt in early childhood

☐ Approve ☐ Revise ☐ Retire Notes:

175. nce-s-cou-051 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · easy · In review

An 8-year-old correctly explains that a ball of clay has the same amount of clay after it is rolled into a long snake, but she struggles with a purely hypothetical question about what would happen if people had no need for sleep. In Piaget's theory, she is MOST likely in which stage?

A. Concrete operational ✓ KEY

The concrete operational child (about 7 to 11) masters conservation and logical operations on concrete objects but not abstract hypotheticals.

B. Preoperational

Preoperational children (about 2 to 7) typically fail conservation tasks because of centration.

C. Formal operational

Formal operational thinkers (about 11 and older) can reason about abstract and hypothetical propositions, which she cannot yet do.

D. Sensorimotor

The sensorimotor stage (birth to about 2) involves learning through senses and actions, before symbolic thought.

Rationale: Piaget's four stages are sensorimotor (birth to 2), preoperational (2 to 7), concrete operational (7 to 11) and formal operational (11 and up). Conservation marks concrete operations; abstract, hypothetico-deductive reasoning marks formal operations.

References: Berk (Development Through the Lifespan) — Piaget's cognitive-developmental theory: concrete operational stage

☐ Approve ☐ Revise ☐ Retire Notes:

176. nce-s-cou-052 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · medium · In review

Asked whether a man should steal an unaffordable medicine to save his wife, a 16-year-old answers, 'No, because if everyone broke the law when they felt like it, society would fall apart.' This reasoning BEST fits which Kohlberg stage?

A. Stage 2: Instrumental exchange

Stage 2 (preconventional) reasoning focuses on self-interest and fair deals, not on maintaining social order.

B. Stage 3: Interpersonal conformity

Stage 3 reasoning centers on being seen as good and winning the approval of others.

C. Stage 4: Law and order orientation ✓ KEY

Stage 4 (conventional) reasoning upholds laws and duties because they maintain the social system.

D. Stage 5: Social contract

Stage 5 (postconventional) sees laws as changeable agreements that can be set aside to protect basic rights such as life.

Rationale: Kohlberg described three levels with two stages each: preconventional (1 punishment and obedience, 2 instrumental purpose), conventional (3 interpersonal conformity, 4 social-order maintenance or law and order) and postconventional (5 social contract, 6 universal ethical principles). Stage is judged by the reasoning given, not by the yes-or-no answer.

References: Berk (Development Through the Lifespan) — Kohlberg's theory of moral development: conventional level

☐ Approve ☐ Revise ☐ Retire Notes:

In the Strange Situation, a 14-month-old stays close to her mother and explores little. She becomes very distressed when her mother leaves, and on reunion she clings to her mother while also pushing away and crying angrily. Which attachment classification BEST fits?

A. Secure

Securely attached infants explore using the parent as a base and are readily comforted on reunion.

B. Anxious-ambivalent (resistant) ✓ KEY

Resistant infants explore little, are highly distressed at separation and both seek and resist contact on reunion.

C. Avoidant

Avoidant infants show little distress at separation and ignore or avoid the parent on reunion.

D. Disorganized/disoriented

Disorganized infants show contradictory, confused behaviors such as freezing or approaching with the head averted.

Rationale: Ainsworth's Strange Situation identified secure, avoidant and anxious-ambivalent (resistant) patterns; Main and Solomon later added disorganized/disoriented. Mixing angry resistance with contact seeking on reunion is the signature of the resistant pattern, often linked to inconsistent caregiving.

References: Berk (Development Through the Lifespan) — Attachment: Ainsworth Strange Situation classifications

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor reviewing an infant assessment reads that, on reunion with a parent, the 12-month-old approached with his face turned away, then froze with a dazed expression. Which statement about this pattern is MOST accurate?

A. It is Ainsworth's original avoidant category, linked mainly to warm but busy caregivers

Avoidant infants show organized avoidance, and the pattern is linked to rejecting, not warm-but-busy, caregiving.

B. It reflects a secure pattern in a temperamentally shy infant who needs time to warm up

Freezing and approach-avoidance are not features of secure attachment, regardless of temperament.

C. It is the resistant pattern, defined by clinging mixed with anger toward the caregiver

Resistant infants show organized ambivalence; the freezing and averted approach described here is different.

D. It fits the disorganized category added by Main and Solomon, tied to frightening care ✓ KEY

Disorganized attachment involves contradictory or dazed behavior and is associated with frightened or frightening caregiving.

Rationale: Main and Solomon identified disorganized/disoriented attachment in infants whose behavior did not fit Ainsworth's three organized patterns. The infant lacks a coherent strategy, often because the caregiver is a source of both comfort and fear, as in maltreatment or unresolved parental trauma.

References: Berk (Development Through the Lifespan) — Attachment: disorganized/disoriented attachment (Main & Solomon)

☐ Approve ☐ Revise ☐ Retire Notes:

The original Sue, Arredondo and McDavis multicultural competencies were organized around attitudes and beliefs, knowledge and skills. Which additional aspirational competency did the Multicultural and Social Justice Counseling Competencies (MSJCC) add?

A. Action ✓ KEY

The MSJCC framework adds action (AKSA), calling counselors to engage in advocacy and intervention at multiple levels.

B. Awareness

Awareness of one's own cultural values and biases was already central to the original competencies.

C. Assessment

Culturally appropriate assessment is part of skills, not a separate aspirational competency in the MSJCC.

D. Accreditation

Accreditation is a program-level standard, not one of the aspirational competencies.

Rationale: The MSJCC (Ratts and colleagues, endorsed by ACA in 2015) uses quadrants of privileged and marginalized counselor and client identities and four developmental domains: counselor self-awareness, client worldview, the counseling relationship, and counseling and advocacy interventions. Each domain includes attitudes and beliefs, knowledge, skills and action.

References: Sue & Sue (Counseling the Culturally Diverse) — Multicultural and Social Justice Counseling Competencies (MSJCC)

► **Check:** MSJCC cited via Sue & Sue; consider the primary source (Ratts et al., 2016) when one is added.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices she feels impatient with clients who defer major decisions to their extended families, and she begins examining how her own upbringing shapes that reaction. In the Sue, Arredondo and McDavis framework, this work MOST directly reflects which competency area?

A. Understanding the worldview of the culturally different client

Understanding the client's worldview is a separate area; her focus here is on her own conditioning.

B. Developing culturally appropriate intervention strategies

Intervention strategies concern what the counselor does in session, not examining her own reactions.

C. Awareness of her own cultural values, assumptions and biases ✓ KEY

Examining how her own background shapes reactions to clients is counselor self-awareness, the first competency area.

D. Knowledge of family structures common among collectivist cultures

Knowledge of client cultures matters, but the activity described is reflection on her own biases.

Rationale: Sue, Arredondo and McDavis (1992) described three characteristics of the culturally skilled counselor: awareness of one's own assumptions, values and biases; understanding the worldview of culturally different clients; and developing appropriate intervention strategies. Each is assessed across beliefs and attitudes, knowledge and skills.

References: Sue & Sue (Counseling the Culturally Diverse) — Multicultural counseling competencies: counselor awareness of own assumptions, values and biases

☐ Approve ☐ Revise ☐ Retire Notes:

Months after a racist incident on campus shook her belief that race did not matter, a Black college student now surrounds herself with Black-centered organizations, studies Black history intensely and expresses anger toward White people. In Cross's Nigrescence model, she is MOST likely in which stage?

A. Pre-encounter

Pre-encounter involves low race salience or pro-White attitudes, which she has moved beyond.

B. Immersion-emersion ✓ KEY

Immersion-emersion involves intense involvement in Black culture and often anger toward White people.

C. Encounter

The encounter is the event, here the incident, that challenged her prior worldview; she has moved past it.

D. Internalization

Internalization brings a secure, less reactive Black identity and openness to other groups.

Rationale: Cross's model describes pre-encounter, encounter, immersion-emersion and internalization (with internalization-commitment for sustained action). After an encounter disrupts the old identity, the person often immerses in Black culture and withdraws from or rejects White culture before reaching a more secure, internalized identity.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial/cultural identity development: Cross Nigrescence model

☐ Approve ☐ Revise ☐ Retire Notes:

After a multicultural course, a White counseling student feels guilt and confusion. She now sees racism around her, yet feels torn between loyalty to family members who make racist jokes and her new beliefs. In Helms' White racial identity model, this experience BEST fits which status?

A. Contact

Contact involves obliviousness to racism and a naive, color-blind view, which she has moved past.

B. Reintegration

Reintegration resolves the conflict by retreating to beliefs in White superiority and blaming people of color.

C. Pseudo-independence

Pseudo-independence is an intellectual commitment to change that comes after the conflict of disintegration.

D. Disintegration ✓ KEY

Disintegration involves guilt, confusion and conflict between new awareness of racism and loyalty to one's own group.

Rationale: Helms' White racial identity statuses are contact, disintegration, reintegration, pseudo-independence, immersion/emersion and autonomy. Disintegration is the uncomfortable phase in which new awareness of racism collides with prior beliefs and group loyalties, producing guilt, shame and moral dilemmas.

References: Sue & Sue (Counseling the Culturally Diverse) — White racial identity development: Helms' model

☐ Approve ☐ Revise ☐ Retire Notes:

At an intake, a client with a history of assault becomes visibly tense when the counselor closes the office door and moves her chair to sit between the client and the exit. Which response BEST reflects trauma-informed practice?

A. Name what she noticed, explain the reason for privacy and ask how the client wants the room set up ✓ **KEY**

This offers transparency, voice and choice, restoring a sense of safety and control, which are core trauma-informed principles.

B. Continue the intake as planned so the client can see the office is safe and her anxiety will fade

Pressing on ignores a cue of distress and puts the counselor's agenda ahead of the client's sense of safety.

C. Begin a detailed account of the assault now so that the tension can be processed early in treatment

Pushing for trauma details at intake, before safety and stabilization, risks retraumatization.

D. Reassure her that the building is secure and that no one can come in while the door is fully closed

Reassurance alone does not give the client any control and may increase her sense of being trapped.

Rationale: SAMHSA's trauma-informed approach rests on safety; trustworthiness and transparency; peer support; collaboration and mutuality; empowerment, voice and choice; and attention to cultural, historical and gender issues. Small environmental choices, explained openly, help clients regain control and prevent retraumatization.

References: Gladding (Counseling: A Comprehensive Profession) — Trauma and crisis counseling: principles of trauma-informed care

► **Check:** SAMHSA's six trauma-informed principles are cited via Gladding (SAMHSA TIP 57 is not on the approved list).

☐ Approve ☐ Revise ☐ Retire Notes:

A client has met her goals for managing panic after 14 sessions, and she and the counselor agree to end counseling. Which approach to the final sessions is BEST?

A. End at the next session with a brief goodbye to avoid fostering dependence on counseling

An abrupt ending skips consolidation and relapse planning and gives no room to process feelings about ending.

B. Review her gains, anticipate setbacks, plan to keep progress and discuss feelings about ending ✓ **KEY**

Effective termination consolidates learning, prepares for relapse, addresses feelings and clarifies how to return if needed.

C. Begin work on new goals so that she stays engaged and does not lose momentum after her progress

Adding goals once the agreed goals are met and the client no longer needs services is not in her interest.

D. Reduce session length gradually while avoiding talk about the ending so she is not made anxious

Avoiding the topic of ending denies the client a chance to process it and to plan for maintaining gains.

Rationale: Termination is a planned phase, not an event. Counselors review progress, help clients attribute change to their own efforts, anticipate challenges and relapse, explore feelings about ending and leave the door open, consistent with ACA guidance to terminate when services are no longer needed and to provide pretermination counseling.

References: ACA Code of Ethics — A.11.c Appropriate Termination · Gladding (Counseling: A Comprehensive Profession) — Termination of counseling relationships

☐ Approve ☐ Revise ☐ Retire Notes:

Core Counseling Attributes

16 items · NCE weight 8% (13 of 160 scored items)

185. nce-s-cor-001 · Counseling and Helping Relationships · empathy and its levels · medium · In review

A 34-year-old client says, "My boss praised my project in front of everyone, but then gave the promotion to someone else. I don't know what to think." Which counselor response is an interchangeable (Level 3) response on Carkhuff's empathy scale?

A. You should ask your boss directly why you were passed over this time.

Advice-giving ignores the client's feelings entirely; it is subtractive (Level 1 or 2), not interchangeable.

B. You feel confused because the praise and the promotion decision don't match. ✓ **KEY**

This captures the feeling (confusion) and its stated cause at the same depth the client expressed, which is the definition of a Level 3 interchangeable response.

C. Underneath the confusion, you may fear that your hard work goes unrecognized.

This goes beyond what the client expressed to an underlying feeling and meaning, making it additive (Level 4), not interchangeable.

D. Lots of people get passed over at work, and it usually works out fine in the end.

Reassurance minimizes the experience and shifts away from the client's feelings, a subtractive response.

Rationale: Carkhuff's scale rates counselor responses from 1 to 5. Level 3 is the minimally facilitative, interchangeable level: the counselor reflects essentially the same feeling and meaning the client expressed, adding nothing and taking nothing away. Levels 1 and 2 subtract from the client's message, and Levels 4 and 5 add depth beyond it.

References: Gladding (Counseling: A Comprehensive Profession) — Carkhuff's scale of empathic understanding, Levels 1-5

☐ Approve ☐ Revise ☐ Retire Notes:

In a sixth session, a graduate student says, "I keep telling everyone I'm fine with moving home after graduation. My parents are happy, so I guess I should be too." Which counselor response BEST illustrates additive (Level 4) empathy?

A. So you're moving home after graduation, and your parents are pleased with it.

This paraphrases the content but omits feeling; it is at best Level 2, slightly subtractive.

B. You feel fine about moving home, since your parents are happy with the plan.

This repeats the client's surface statement and misses the doubt in "I guess I should be"; it does not add depth.

C. Why do you think your parents' happiness matters so much in this decision?

A why-question invites intellectualizing and does not communicate understanding of the feeling, so it is not an empathic response.

D. You sound unsure, as if your own wishes got set aside to keep your parents happy. ✓ KEY

This names the implicit ambivalence and the meaning beneath it (setting aside one's own wishes), adding accurately to what the client said: Level 4.

Rationale: Additive empathy (Carkhuff Levels 4-5) goes beyond the client's explicit words to reflect feelings and meanings the client implied but did not fully state, which helps the client explore at a deeper level. To be additive rather than inaccurate, the reflection must stay grounded in cues the client gave, such as the hedged "I guess I should be."

References: Gladding (Counseling: A Comprehensive Profession) — Advanced (additive) empathy, Carkhuff Levels 4-5

► **Check:** Option A is rated Level 2 in its rationale; raters may disagree on the exact level (it is clearly not additive).

☐ Approve ☐ Revise ☐ Retire Notes:

On Carkhuff's five-level scale of empathic understanding, a counselor response rated at Level 1 or Level 2 is described as which of the following?

A. Subtractive, detracting from what the client expressed ✓ KEY

Levels 1 and 2 are subtractive: they ignore or only partly capture the client's feelings and meaning.

B. Interchangeable, matching the client's expressed feeling

Interchangeable, minimally facilitative responses are Level 3.

C. Additive, reflecting feelings below the client's words

Additive responses that reach underlying feelings are Levels 4 and 5.

D. Facilitative, the baseline goal for beginning counselors

Level 3 is the minimally facilitative baseline counselors aim to reach consistently; Levels 1-2 fall below it.

Rationale: Carkhuff's scale runs from Level 1 (the response ignores or contradicts the client's feelings) to Level 5 (the response adds significantly to the client's awareness). Levels 1-2 are subtractive, Level 3 is interchangeable and minimally facilitative, and Levels 4-5 are additive.

References: Gladding (Counseling: A Comprehensive Profession) — Carkhuff's scale of empathic understanding

☐ Approve ☐ Revise ☐ Retire Notes:

A client tearfully describes the recent death of her dog. Which counselor response reflects empathy rather than sympathy?

A. I'm so sorry; I lost my own dog last year and I still cry about it sometimes too.

This shifts the focus to the counselor's own loss and expresses pity or shared distress, which is sympathy and self-disclosure, not empathy.

B. That is such a sad thing to go through, and I feel terrible for you today.

Feeling sorry for the client is sympathy; it conveys the counselor's feeling rather than understanding of the client's.

C. Losing her has left a real emptiness in your days, and the grief feels heavy. ✓ **KEY**

This communicates accurate understanding of the client's own experience from her frame of reference, which is empathy.

D. At least she had a long life with you, and you gave her a very good home.

"At least" reassurance minimizes the loss and moves away from the client's feelings.

Rationale: Empathy is understanding the client's world as if it were one's own, without losing the "as if" quality, and communicating that understanding back. Sympathy centers on the helper's own feelings of sorrow or pity for the client, which can shift the focus away from the client.

References: Corey (Theory & Practice) — Person-centered therapy: accurate empathic understanding

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 52-year-old client says, "I yelled at my teenage son last night and called him useless. I'm ashamed to even tell you." Which counselor response BEST demonstrates unconditional positive regard?

A. That was harsh, but I'm sure you are a good parent most of the time anyway.

Reassurance about being a "good parent" is conditional praise; it evaluates the client rather than accepting the client.

B. Calling him useless can do real damage, so let's plan what to say instead.

This jumps to evaluation and correction before the client feels accepted; it is not an expression of UPR.

C. It took courage to share that, and I'm glad you trusted me with it here. ✓ **KEY**

This accepts the client as a person, without approving or condemning the behavior, and values the client's openness, which is UPR.

D. Don't be ashamed, since all parents lose their temper and say things they regret.

Telling the client not to feel shame dismisses the feeling and normalizes too quickly; it is reassurance, not acceptance.

Rationale: Rogers described unconditional positive regard as a warm acceptance of the client as a person, without conditions of worth. UPR does not mean approving of every behavior; it means the client's worth is not contingent on what is disclosed, which lowers defensiveness and supports self-exploration.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance

☐ Approve ☐ Revise ☐ Retire Notes: _____

Midway through a session, a client asks, "You look distracted. Are you even listening to me?" The counselor realizes her attention had drifted. Which response BEST demonstrates congruence?

A. You're right, my mind wandered for a moment. I'm with you now; please go on. ✓ KEY

The counselor's words match her inner experience, she owns it briefly, and she returns the focus to the client, which is congruence.

B. Of course I'm listening, and I have been following everything you've said.

Denying a lapse the counselor knows occurred is incongruent; outward words do not match inner experience.

C. It sounds like you worry people don't listen to you. Tell me more about that.

Reflecting the client's concern here deflects from the counselor's actual lapse and avoids genuineness.

D. I've had a long, stressful day with other clients, so I'm tired and distracted.

This is honest but over-discloses and shifts the focus to the counselor's needs rather than the relationship.

Rationale: Congruence (genuineness) means the counselor's outward behavior is consistent with inner experience, without a professional facade. Rogers viewed it as the most basic of the core conditions. Being genuine does not require disclosing every feeling; the counselor shares what is relevant to the relationship and keeps the focus on the client.

References: Corey (Theory & Practice) — Person-centered therapy: congruence or genuineness

☐ Approve ☐ Revise ☐ Retire Notes:

Bordin's pantheoretical model of the working alliance consists of which three components?

A. Empathy, congruence, and positive regard

These are Rogers' core conditions, not Bordin's alliance components.

B. Rapport, insight, and corrective feedback

These are general therapy processes, not the components Bordin named.

C. Trust, transference, and client motivation

Transference is a psychodynamic concept; Bordin's model was explicitly pantheoretical.

D. Agreement on goals, tasks, and the bond ✓ KEY

Bordin defined the working alliance as agreement on goals, agreement on tasks, and the emotional bond between counselor and client.

Rationale: Bordin (1979) proposed that the working alliance has three parts: agreement on the goals of counseling, agreement on the tasks used to reach them, and the affective bond between counselor and client. Because it is pantheoretical, the model applies across approaches, and alliance strength is one of the most consistent predictors of outcome.

References: Gladding (Counseling: A Comprehensive Profession) — The working alliance: goals, tasks, and bond

☐ Approve ☐ Revise ☐ Retire Notes:

In the fourth session, a client who has been completing CBT thought records says flatly, "I did the homework again, I guess. Whatever helps you." The client has arrived late twice. What should the counselor do FIRST?

A. Review the thought record in detail and praise the client for completing it

Proceeding with the task ignores the withdrawal cues and leaves a possible rupture unaddressed.

B. Invite the client to talk about how the work together is feeling so far ✓ **KEY**

Openly exploring the relationship addresses a likely withdrawal rupture and checks agreement on tasks and goals before continuing.

C. Set a lateness policy so that attendance does not continue to slip further

Focusing on rules treats the lateness as a compliance issue and can deepen the rupture.

D. Switch to a different intervention, since thought records are not working

Changing tasks unilaterally skips collaboration; the counselor does not yet know what the client wants or is reacting to.

Rationale: Flat compliance ("whatever helps you") and lateness suggest a withdrawal rupture in the alliance, often reflecting disagreement about tasks or goals or strain in the bond. Research on rupture and repair supports raising the relationship directly and nondefensively, which lets the counselor renegotiate tasks and goals collaboratively. Successfully repaired ruptures are associated with better outcomes.

References: Gladding (Counseling: A Comprehensive Profession) — Working alliance: goals, tasks, bond; addressing alliance ruptures

► **Check:** Rupture-repair evidence (Safran et al.) is cited to a general Gladding alliance reference.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who has completed many multicultural trainings begins work with a Hmong American client. According to Hook and colleagues' model of cultural humility, which stance BEST reflects cultural humility?

A. Ask the client what their culture means to them and stay open to being corrected ✓ **KEY**

Cultural humility is an other-oriented, curious stance that treats the client as the expert on their own cultural identity.

B. Draw on prior training about Hmong customs to anticipate the client's values

Applying general group knowledge as a template risks stereotyping; it reflects a knowledge-based, not humble, stance.

C. Set cultural issues aside unless the client raises them first in the session

Waiting for the client to raise culture places the burden on the client and can signal that culture is unwelcome.

D. Treat the training already completed as sufficient competence for this client

Viewing competence as a finished achievement contradicts the ongoing, lifelong nature of humility.

Rationale: Hook et al. (2013) described cultural humility as an interpersonal stance that is other-oriented rather than self-focused, marked by respect, openness, and lack of superiority regarding the client's cultural background. It is an ongoing process of self-reflection rather than a fixed body of knowledge, and clients' perceptions of it are associated with a stronger alliance.

References: Sue & Sue (Counseling the Culturally Diverse) — Cultural humility as an other-oriented stance

► **Check:** Confirm the cited Sue & Sue edition discusses Hook et al.'s cultural humility model.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices that she schedules a client who lives in public housing for shorter sessions and assigns fewer homework tasks, assuming the client "won't follow through." What is the MOST appropriate next step?

A. Refer the client to a counselor who has more experience with low-income clients

Referral based on the counselor's bias, rather than competence limits, abandons the client and avoids the counselor's own work.

B. Continue as planned, since shorter sessions may reduce the client's burden

This rationalizes the bias instead of examining it and keeps treating the client differently.

C. Examine the assumption in supervision and restore an equitable treatment plan ✓ **KEY**

Recognizing an implicit bias, exploring it in supervision, and correcting its effect on care reflects counselor self-awareness.

D. Tell the client about the bias so the client can decide whether to continue

Disclosing the bias burdens the client with the counselor's issue; the counselor should first address it through supervision.

Rationale: Counselor self-awareness of attitudes, beliefs, and biases is the first dimension of multicultural competence. Biases about social class can shape clinical decisions without conscious intent. When a counselor notices differential treatment, the ethical response is to examine it, often through supervision or consultation, and correct its effect on services.

References: Sue & Sue (Counseling the Culturally Diverse) — Counselor awareness of own assumptions, values, and biases · ACA Code of Ethics — C.5 Nondiscrimination

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices he feels unusually irritated and impatient with an older male client who reminds him of his own critical father. This reaction is BEST described as

A. transference by the client

Transference is the client's projection of past relationships onto the counselor; here the reaction is the counselor's.

B. countertransference ✓ **KEY**

The counselor's emotional reaction to the client, shaped by his own unresolved history, is countertransference.

C. resistance in the client

Resistance refers to client behavior that impedes change, not the counselor's feelings.

D. compassion fatigue

Compassion fatigue is cumulative exhaustion from trauma exposure, not a reaction tied to a specific personal history.

Rationale: Countertransference refers to the counselor's emotional reactions to a client, particularly those rooted in the counselor's own unresolved conflicts or past relationships. It is not inherently harmful, but unexamined countertransference can distort the counselor's perceptions and responses, so recognizing it is the first step to managing it.

References: Corey (Theory & Practice) — Psychoanalytic therapy: transference and countertransference

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor whose sister died by overdose finds herself giving extra session time and her personal cell number to a young adult client in early recovery. What is the BEST course of action?

A. Keep the extra contact, since it clearly helps the client stay engaged

This keeps a boundary crossing driven by the counselor's needs rather than the client's welfare.

B. Transfer the client right away to avoid any further personal reactions

Abrupt transfer is premature; countertransference can often be managed and abrupt referral may harm the client.

C. Disclose the sister's death so the client understands her strong concern

This self-disclosure serves the counselor's needs and burdens the client.

D. Seek supervision on the reaction and restore usual session boundaries ✓ **KEY**

Recognizing the countertransference, processing it in supervision, and restoring boundaries protects the client and the work.

Rationale: Countertransference can show up as overinvolvement, rescuing, or boundary crossings that meet the counselor's needs. Effective management involves self-awareness, supervision or personal counseling, and returning to boundaries that serve the client. Referral is reserved for cases where the counselor cannot remain objective after these steps.

References: Corey (Theory & Practice) — Countertransference and the counselor as a person · ACA Code of Ethics — A.6.b Extending Conventional Boundaries

☐ Approve ☐ Revise ☐ Retire Notes:

A supervisor tells a trainee, "Every feeling you have toward a client, not just the conflicts from your own past, can be useful information about the client and the relationship." This statement reflects which view of countertransference?

A. Classical view, treating it as an obstacle to be eliminated

The classical (Freudian) view limits countertransference to the counselor's unresolved conflicts and treats it as interference.

B. Behavioral view, treating it as a learned response to extinguish

Countertransference is not framed as a conditioned response in behavioral theory; this distractor misattributes the concept.

C. Complementary view, limited to the client's projected roles

The complementary conception frames countertransference as the counselor's reaction to the interpersonal pull of the client's style, a narrower idea than all feelings toward the client.

D. Totalistic view, including all counselor reactions to clients ✓ **KEY**

The totalistic view defines countertransference as all of the counselor's emotional reactions and sees them as potentially useful data.

Rationale: The classical view, rooted in Freud, defines countertransference narrowly as the counselor's unconscious, conflict-based reactions to the client's transference and treats it as an impediment. The totalistic view broadens it to all of the counselor's emotional responses and treats them as a source of understanding. Contemporary integrative views keep the focus on conflict-based reactions while stressing that, once understood, they can inform the work.

References: Corey (Theory & Practice) — Psychoanalytic therapy: countertransference and its therapeutic use

► **Check:** Classical/totalistic/complementary views come from Gelso & Hayes; confirm Corey covers the totalistic view or swap the source.

☐ Approve ☐ Revise ☐ Retire Notes:

A college student says, "I've been hooking up with different people most weekends, and my roommate thinks I'm a mess." Which counselor response BEST reflects a nonjudgmental stance?

A. Your roommate may have a point, so let's look at the risks you're taking.

Siding with the roommate conveys the counselor's judgment before understanding the client's view.

B. Your roommate sees it one way. I'm curious how you feel about these weekends. ✓ **KEY**

This neither condemns nor endorses the behavior and invites the client's own perspective, which is nonjudgmental.

C. There's nothing wrong with that at all, and your roommate should not judge you.

Approval is also an evaluation; it imposes the counselor's values in the opposite direction.

D. What do you think is missing in your life that leads you to keep doing this?

This presumes the behavior signals a deficit, a judgment embedded in the question.

Rationale: A nonjudgmental stance means the counselor suspends both disapproval and approval and seeks to understand the client's experience from the client's frame of reference. Counselors avoid imposing their own values on clients, which allows clients to explore their choices without becoming defensive.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A 70-year-old widower arrives early for his first session and seems nervous. Which counselor behavior BEST conveys warmth and respect?

A. Greet him by name, ask how he prefers to be addressed, and offer him a seat ✓ **KEY**

This shows genuine interest and respects his preferences and dignity, communicating warmth and respect.

B. Call him "sweetie" and speak slowly and loudly so he can follow along easily

Endearments and exaggerated speech are elderspeak; they are patronizing and disrespectful.

C. Hand him the intake forms and ask him to wait until his appointment time

This is procedural and distant, and it ignores his visible nervousness.

D. Tell him there is no need to be nervous because most clients feel fine later

This dismisses his feelings and offers reassurance in place of attunement.

Rationale: Warmth is conveyed through friendly, caring verbal and nonverbal behavior, and respect is shown by treating the client as a capable person with preferences and dignity. Asking how a client wishes to be addressed is a simple, culturally responsive way to communicate both, and avoiding ageist communication such as elderspeak is part of respect.

References: Gladding (Counseling: A Comprehensive Profession) — Building a counseling relationship: warmth and respect

☐ Approve ☐ Revise ☐ Retire Notes:

A White counselor working with a Black client hears the client say, "I wasn't sure I could talk about racism at work with you." Which counselor response BEST shows awareness of power and privilege in the relationship?

A. I don't really see color, so you can talk to me about anything you need to here.

Color-blind statements deny the relevance of race and the client's concern, which is a microinvalidation.

B. I've had many Black clients, so I understand a lot about what you face at work.

Claiming understanding based on experience with other clients overstates the counselor's knowledge and dismisses the concern.

C. As a White counselor I may miss things, and I'd like to hear it if you're willing. ✓ **KEY**

This names the racial difference and the limits of the counselor's perspective, and invites the client's experience without pressure.

D. Let's focus on what you can control at work, since we can't change the people there.

Redirecting away from racism minimizes a systemic experience and avoids the topic the client raised.

Rationale: Counselors from privileged groups hold both professional and social power. Broaching racial and cultural differences directly and nondefensively, and acknowledging the limits of one's own perspective, signals that the client's experiences of oppression are welcome. Color-blind or minimizing responses are common microaggressions that weaken trust.

References: Sue & Sue (Counseling the Culturally Diverse) — Power, privilege, and microaggressions in counseling; color-blindness

☐ Approve

☐ Revise

☐ Retire

Notes: